

Psychological self-help interventions

Delivering self-help for individuals, featuring Step-by-Step and Doing What Matters in Times of Stress



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What Matters in Times of Stress

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Psychological self-help interventions: delivering self-help for individuals, featuring Step-by-Step and Doing What Matters in Times of Stress

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Preface

Millions of people worldwide who experience mental health conditions do not have access to effective care. Self-help can help increase access to care in all countries. By providing structured psychological techniques that people can use in their own time, these interventions can empower individuals to manage stress and improve their mental health.

Evidence shows that self-help interventions are effective, particularly for depression and anxiety, and these interventions are recommended by the World Health Organization (WHO). These interventions are likely to play a key role in expanding access to evidence-based mental health care in numerous countries. Because they can be delivered in person or remotely, with limited support from trained non-specialists, they have the potential to reach large numbers of people across diverse settings.

This manual is designed to support decision-makers and practitioners in planning and implementing both unguided and guided self-help. It includes information on two WHO-developed programmes – Step-by-Step and Doing What Matters in Times of Stress – which are delivered individually, both of which have been rigorously tested in randomized controlled trials (RCTs). Typically, these programmes are offered with brief weekly remote support (around 15 minutes of time) from a trained helper, though different models of delivery are also possible, such as no support or face-to-face support.

Step-by-Step and Doing What Matters in Times of Stress have already been integrated into national services: both interventions have been scaled up nationwide in Thailand and Step-by-Step is available across Lebanon. These experiences, alongside strong research evidence, demonstrate the powerful contribution that self-help interventions can make to any mental health system. They offer a practical and effective way to bring care to more people, wherever they live.



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Declarations of interest

In accordance with United Nations procedures, all external reviewers completed and signed a standard WHO Declaration of Interest form. These were reviewed by the coordinating team. No conflicts of interest were identified.

Acronyms and abbreviations

DWM	Doing What Matters in Times of Stress
EASE	Early Adolescent Skills for Emotions
EQUIP	Ensuring Quality in Psychosocial and Mental Health Care
mhGAP	Mental Health Gap Action Programme
RCT	Randomized controlled trial
SbS	Step-by-Step
SH+	Self-Help Plus
UNICEF	United Nations Children's Fund
WeACT	Working with children – Assessment of Competencies Tool
WHO	World Health Organization

Background

1. Introduction

Psychological self-help interventions¹ are a valuable addition to mental health services in countries of all income levels, especially to help scale up care for common mental health conditions. They are recommended by the World Health Organization (WHO) to support people, or carers of people, experiencing a number of mental health, brain health and substance use conditions (1).² Some guided self-help interventions delivered individually (for example, for depression and anxiety) demonstrate a similar level of effectiveness as more intensive interventions (2, 3). A key benefit is that they can increase access to evidence-based support for large numbers of people.

Psychological self-help interventions provide exercises and techniques that a person can use to help manage psychological distress and symptoms of mental health and other conditions. They can be delivered through different media (such as mobile application, website, book, video or audio recording), individually or in groups. They are structured so that a person can work through them completely on their own (unguided) or with the addition of some support, such as a brief introduction and follow-ups from a health professional or weekly contact (for example, via email, instant message, telephone, video or in person) from a trained and supervised non-specialist (guided).³

Psychological self-help interventions require less provider time to deliver than many mental health interventions, which may reduce costs and increase access. Digital versions can be delivered across a whole country or geographical region. This feature of digital interventions may be critical in areas where access to supervision and training is limited, as helpers and supervisors can be centralized in a few locations with the intervention provided remotely to a population. Unguided self-help is cheaper and easier to implement, but it may have a smaller impact on improving mental health than guided self-help. Guided self-help may have increased effects, especially for people with more severe problems. Self-help interventions can be integrated into existing mental health services and can complement any other type of care.

1.1 About this manual

1.1.1 Scope

This manual provides information to support decision-makers and workers in planning and implementing individually delivered, evidence-based, psychological self-help interventions. It describes the added value of these interventions and considers how, where and when they can be of most use. Other forms of self-help, such as self-help groups organized by people experiencing

1 In this document we use the term “self-help intervention” or “self-help” to mean “psychological self-help intervention”.

2 The terms “mental health symptoms” and “mental health conditions” are used in this manual, yet all information is relevant for the wider set of mental health, brain health and substance conditions.

3 In this document we use the term “helper” to refer to someone providing support with self-help interventions.

similar problems, or WHO's Self-Help Plus (SH+) intervention (4), are not explicitly covered, yet much of Chapter 1 is relevant for all evidence-based self-help interventions.

The manual is primarily based on evidence from research trials conducted by WHO with adults with common mental health conditions, but much of the information may be relevant for other conditions where there are similar evidence-based interventions, such as iSupport for carers of people with dementia and the web-based programme Alcohol E-health (5, 6), or for adolescents and youths where similar self-help interventions are also shown to be effective (7, 8).

This manual also provides detailed information on implementing two individually delivered, WHO psychological self-help interventions: Step-by-Step (SbS) (SbS content can be found in the [Web Annex](#) to this manual) and Doing What Matters in Times of Stress (DWM) (9), which have been shown to be effective in several randomized controlled trials (RCTs) (see [Table 2.1 Overview of SbS and DWM](#) for more information on the trials). Both can be provided unguided: as part of routine care, where the intervention is provided by primary health care staff with minimal support; or using a guided self-help model developed and tested by WHO and scaled up nationally in Lebanon and Thailand. In this model, people completing the self-help intervention receive brief support (15 minutes per week) from a trained non-specialist helper (typically a lay community provider), either by telephone or message (10, 11).

This model (including example scripts and templates) can be adapted for use with other evidence-based individually delivered self-help interventions.

1.1.2 Audience and structure

This manual is written for:

- programme managers in organizations who want to implement a self-help intervention; and
- the helpers, supervisors and other workers who will be involved in its delivery.

The document is divided into four main parts, including the annexes ([Figure 1.1 Content overview of this document](#)). These are as follows: relevant information giving a general introduction to self-help interventions (Chapter [1. Introduction](#)); practical information for setting up and providing individually delivered self-help interventions, which covers both guided and unguided interventions broadly (Chapter [2. For organizations: how to set up individually delivered self-help](#)) and provides specific information that is used in running guided self-help interventions (Chapter [3. Introduction to providing support in guided self-help](#) and Chapter [4. How to provide support to service users](#)); information specific to setting up SbS and DWM (Chapter [5. Information for helpers delivering Step-by-Step](#) and Chapter [6. Information for helpers delivering Doing What Matters in Times of Stress \(DWM\)](#)); and annexes with scripts and other key tools for helpers.

If you are interested in setting up DWM or SbS as a guided intervention and want to know specifically how helpers deliver support in 15 minutes, you may want to start by reviewing Chapter [4. How to provide support to service users](#) and then come back to the rest of the manual.

Figure 1.1 Content overview of this document

Background	
Chapter 1 Introduction	• Introduction to self-help interventions, including what they are and when and where to use them.
Practical information for general delivery	
Chapter 2 For organizations: how to set up individually delivered self-help	• The steps to follow when establishing a programme of individually delivered selfhelp, most useful for programme managers and implementing organizations.
Chapter 3 Introduction to providing support in guided self-help	• An introduction to the role of a helper, including what the role is and skills needed, aimed at helpers.
Chapter 4 How to provide support to service users	• Overview and detailed steps for delivering the WHO guided self-help model, mainly aimed at non-specialist helpers and their trainers and supervisors.
Tips and tools for specific interventions	
Chapter 5 Information for helpers delivering Step-by-Step	• An in-depth consideration of two WHO interventions, including an overview, a description of each module, and tips and tools for helpers to use when supporting participants.
Chapter 6 Information for helpers delivering Doing What Matters in Times of Stress	
General tools and templates	
Annexes	• General templates, scripts and other practical tools that can be used (or adapted for use) when delivering self-help interventions.

1.1.3 Approach to development

This document manual builds upon prior work involving testing and implementing the WHO psychological interventions Step-by-Step and Doing What Matters in Times of Stress. It originated from the need to develop resources for stakeholders to support implementation of WHO recommendations of self-help interventions for common mental health conditions. The development of both interventions and this manual was informed by an extensive literature review and consultation with experts. The process included focus group discussions, adaptation workshops, review of content by people with lived experience of mental health conditions, formative research and RCTs.

The guided self-help model presented in this manual has been rigorously tested through pilot trials, feasibility RCTs and definitive RCTs. The content draws from unpublished implementation manuals used in the various research trials. These findings have been updated to reflect further insights gained from WHO recommendations and from practical experience with delivering the interventions.

The Step-by-Step intervention material, included as web annex, was developed through an iterative process. Following an extensive literature review and expert consultations, an initial draft was created and subsequently reviewed by both experts and service users. Based on their feedback, revisions were made, and a first version of the intervention was piloted in Lebanon. Insights and feedback from this pilot and all other research phases were then incorporated into the final materials.

1.2 About self-help

1.2.1 What is self-help?

Self-help is a diverse category of approaches whereby people use information to help themselves, with limited or no support from professionals or other helpers. This includes individually accessed self-help books or mobile applications, attending psychoeducation groups, mutual self-help groups run by peers experiencing similar problems, and other diverse approaches. This manual focuses on individual, evidence-based, psychological self-help interventions.

These self-help interventions:

- provide information, psychological exercises and techniques (such as slow breathing) that a person learns and completes in their own time to help manage stress and symptoms of common mental health conditions;
- use the types of information and techniques commonly found in other psychological interventions;
- deliver content through different media (such as mobile application, website, book, video or audio recording); and
- can be offered with or without guidance.

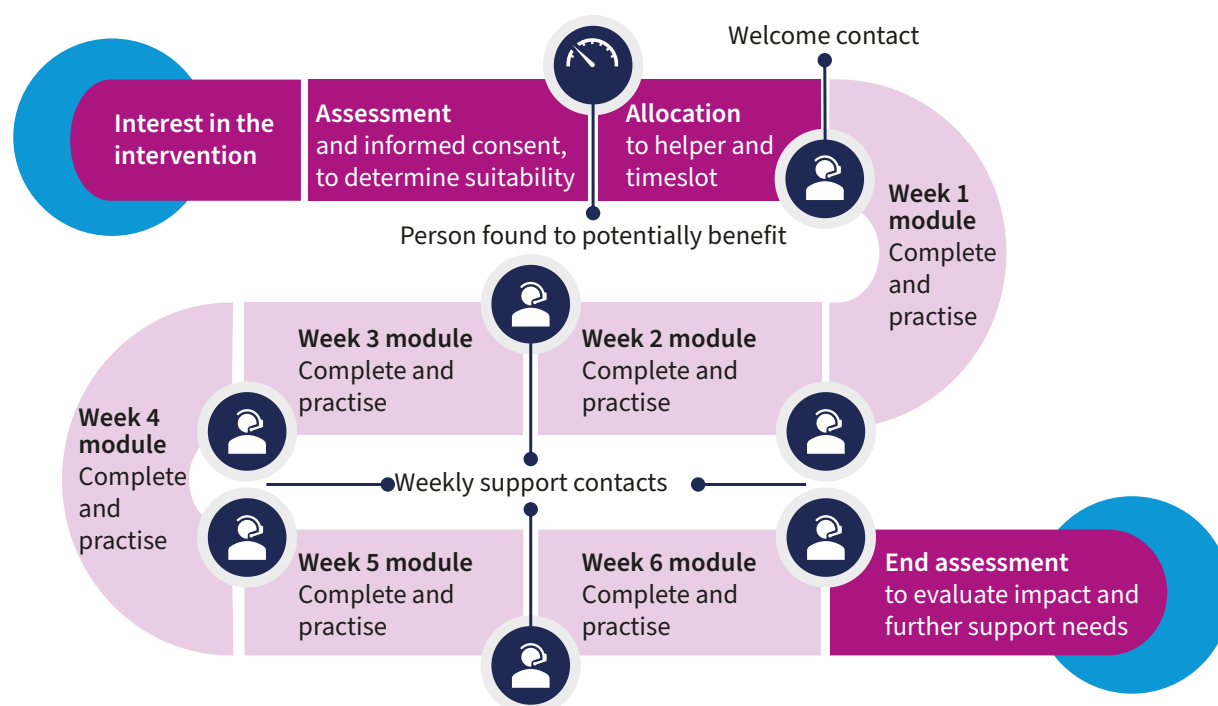
Guided self-help interventions give people some type of support in using the self-help content. Support is minimal and may include introducing the intervention, answering questions and motivating the person. It may vary in duration and intensity. Providing support has been shown to increase retention and engagement (see section [2.2.3 Engagement](#)).

The guided self-help model described in this manual uses trained and supervised helpers to provide weekly support contacts of approximately 15 minutes as service users complete the intervention. The helper does not act as a counsellor or therapist but is trained to provide structured support

that covers recapping key content, answering intervention-related questions and reviewing and encouraging use of the skills. Helpers are supervised and are trained to ensure people's safety and refer them for extra support when needed. Annexes [1A. Summary of tasks for welcome contact](#), [1C. Summary of tasks for regular support contact](#) and [1E. Summary of tasks for end contact](#) outline the content covered in a helper contact.

There are many ways to deliver self-help, but they all begin with providing information to a person that is interested in using the intervention. In unguided self-help this may be as simple as a health professional giving the person a book or access to a website, with a brief introduction to the content. In other settings, an informed consent procedure and assessment may be completed to determine if the person is suitable for the self-help intervention and presents with relevant symptoms. If suitable and interested, the person's journey then continues with access to the self-help materials. Completion of the self-help intervention ideally includes a final assessment to evaluate the intervention's outcome and, if needed, referral to further support. For guided self-help, the person receives brief but regular support contacts to help them on their journey ([Figure 1.2 Example of a person's journey through individual guided self-help](#)).

Figure 1.2 Example of a person's journey through individual guided self-help (for a six-week intervention)



1.2.2 Why use self-help and what are some limitations?

Many practitioners may have understandable concerns about whether self-help can be offered safely and effectively. Self-help is not new; it has been used for decades, both in commercial forms (such as self-help books, videos and smartphone apps) and in health services. While self-help may not be suitable for everyone, it can play a valuable role in providing effective mental health care as part of a comprehensive service approach. This section aims to provide information to address some common concerns.

The evidence

- The WHO *World mental health report* highlights the evidence for self-help interventions and the important role they can play in delivering evidence-based interventions to people living with mental health problems (12).
- Systematic reviews examining self-help programmes for depression and anxiety show that they are effective in reducing symptoms (13, 14). Guided self-help interventions probably have a greater effect than unguided interventions for most mental health conditions and have been shown to be as effective as in-person treatments (3, 13, 15). Additionally, self-help interventions have proven effective in various contexts, including in low- and middle-income countries (16, 17) and nationally at scale as an integral part of the health service.
- Recent WHO guidelines recommend psychological self-help interventions for depression, anxiety, bodily distress complaints, and alcohol and substance use disorders, and for carers of persons with dementia, psychosis or bipolar disorder (1).
- A guided version of SbS has been tested through two large RCTs in Lebanon and found to effectively reduce symptoms of common mental health conditions ([Box 1.1 Step-by-Step in Lebanon](#)). It has also been part of research trials for Filipino overseas workers in China (18) and Chinese young adults in Macao Special Administrative Region, China (19).
- SbS without guidance (unguided) was evaluated in Syrian refugees in Egypt, showing small positive effects in reducing psychological distress and improving overall functioning (20). However, the same version of unguided SbS tested in Germany and Sweden did not find a significant effect, suggesting the importance of considering the needs of communities when deciding on implementing interventions with or without guidance (21).
- DWM has demonstrated improvements in common mental health symptoms at immediate post-test in a pilot RCT and as part of RCTs testing a stepped-care model (22-24).
- Other studies show the effectiveness of other self-help interventions. For example, in Indonesia an RCT of a digital guided self-help intervention reduced symptoms of depression and improved chances of remission at 10 weeks (25). Studies of the internationally used digital self-help intervention MoodGYM have similarly shown improvement in the mental health of users (26).

Box 1.1 Step-by-Step in Lebanon

In 2020, amid multiple economic, humanitarian, political and health crises in Lebanon, a culturally adapted version of SbS was tested with people living in Lebanon, including displaced Syrians, through two large RCTs. The trials showed that the intervention was effective in reducing symptoms of depression and anxiety and improving functioning and well-being, including at three-month follow-up. The trials also showed that this intervention could be delivered safely, with procedures developed to ensure support to people at imminent risk of suicide.

Step-by-Step was later scaled up as a national service in Lebanon and made available to anyone in the country. People who completed the programme said it was relevant, acceptable and beneficial.

“I encouraged one lady I know who is going through a lot in her life to enter the app. [A] few weeks later she called me and said ‘I was dead and now I feel alive ... if I didn’t have SbS I would have definitely caused harm to myself by now.’ She said the exercises helped her be socially active again and meet new friends.” (Syrian outreach volunteer)

“During the Beirut explosion, when blood was running on my head, I started breathing slowly. I understand now the importance of these exercises.” (Syrian woman)

“Participating in Step-by-Step was a source of strength and trust for me. I learnt how to help myself when I’m feeling sad. I learnt as well to live each day by day. The most important thing is to be myself and love myself regardless.” (Lebanese woman)

Sources: Cuijpers et al. (10); Cuijpers et al. (27); Abi Ramia et al. (28).

In addition to having a strong evidence base, self-help interventions share many of the benefits of other psychological interventions delivered by non-specialists (for example, Problem Management Plus). For example, delivery by non-specialists may increase coverage and access, and free up the time of mental health specialists to provide training, supervision and interventions to people most in need. Self-help interventions can also be:

- **Practical.** They give people, including those experiencing severe distress, tools and information to help self-manage mental health conditions.
- **Accessible.** They can be provided remotely (including for persons with mobility constraints) to large numbers of people, across large geographical areas, including hard-to-reach places (such as conflict zones). Their flexibility can reduce physical, economic, and time- or stigma-related access barriers. They have been found to be feasible during health crises and pandemic lockdowns.

- **Less resource intensive.** Self-help interventions typically take less helper time to deliver than other non-specialist interventions. By delivering guided self-help remotely, helpers and supervisors can be centralized in a limited number of areas, which may help implementation or reduce costs.
- **Versatile.** They can be delivered locally through self-help books; or nationally, for example through a website, app or online video, with or without in-person or remote support. They can also be combined with more intensive interventions and delivered as a first step in stepped care or as a prevention intervention.
- **Scalable.** They can be adapted to local needs, languages and cultural contexts and reach more people than conventional in-person treatments.

Self-help interventions do have some limitations:

- Commercially available self-help interventions may not be evidence based. It is important to check the background and evidence behind any self-help intervention before providing it to populations in need (see section [2.1.1 Intervention selection](#)).
- They may not be right for all people (see section [1.2.3 Who is self-help for?](#)) and should be part of a range of services, rather than a stand-alone approach.
- They usually have higher drop-out rates than face-to-face interventions (see section [2.2.3 Engagement](#)).

The above limitations all highlight the importance of incorporating self-help interventions as part of a comprehensive system.

1.2.3 Who is self-help for?

Self-help interventions are often developed and tested to benefit people with specific mental health conditions (such as depression) or symptoms that may cross a range of mental health conditions (“transdiagnostic” interventions). Some self-help interventions do not focus on a specific mental health condition but rather aim to increase psychological well-being.

Self-help interventions can be used by different populations, including people:

- of different beliefs, religions and cultural backgrounds;
- with varying levels of education (although ability to read, use a digital device or access the internet may be required, depending on format); and
- from different age groups (preferably interventions for specific age groups are co-designed and tested with people from these age groups; however, in some cases interventions have been adapted for other age groups; see section [2.2 Adapt for context and culture](#)).

Most self-help interventions are not designed for people with severe cognitive impairments, such as those related to a mental, neurological or substance use disorder. Most are not designed for people

in high-risk situations, such as imminent risk of suicide, or harm through intimate partner violence or abuse. People facing these risks may benefit from self-help but only if the risk situation is adequately addressed. Similarly, people with additional needs, such as comorbid health conditions, protection concerns or housing problems, may benefit from self-help but should also be referred for extra support, for example from other health, social service or protection agencies.

This manual focuses on interventions that are developed to treat mental health conditions. When selecting an intervention it is important to choose one that targets the population you want to help (see section [2.1.1 Intervention selection](#)).

1.2.4. Where can self-help be used?

Self-help interventions can be offered at local, regional or national levels, and can be provided using in-person or remote support, including call centres or helplines. They suit various settings but are particularly useful for places where geographical, financial or resource constraints limit access to mental health services. Intervention use may have specific requirements, such as the ability to read or access the internet. When planning to implement self-help, it is important to consider any potential barriers to access (including literacy, digital literacy, access to digital devices and internet connectivity).

It is also important to consider how self-help interventions can be embedded in local health care and protection systems. As with all psychological interventions, appropriate referral and support options for both mental health concerns (for example, access to a clinician trained under the Mental Health Gap Action Programme (MhGAP) or referral to a local specialized service) and protection concerns (for example, in the case of gender-based violence or violence against children) should be available. If you are implementing self-help interventions, it is critical that funding is available not only for the self-help interventions, but also to ensure that access to these and other services is possible. Self-help interventions can complement other forms of care, such as more intensive psychological interventions or medicines, by providing additional tools and strategies for managing symptoms.

Interventions such as SbS and DWM generally use community providers specifically recruited to serve as helpers, for example undergraduates, community members, lay volunteers and peers. Yet other providers, including primary health care providers or trained clinicians, can also support people with self-help interventions. They may not have the time to deliver the 15 minutes of weekly support outlined in the DWM and SbS model, but they could be trained to provide a self-help book and check in on the person's use of its strategies as part of routine follow-up appointments. The guidance in this manual can be adapted for such settings.

Note. For more detailed information on how to set up psychological interventions, including self-help interventions, see the WHO *Psychological interventions implementation manual* ([29](#)).

1.2.5 When to use guided or unguided self-help

Guided self-help is likely to be more effective than unguided self-help (3, 15). Yet implementing guided self-help may not always be feasible because of the programme's aims or resource constraints. Unguided self-help can still benefit people, especially when delivered across a large population. The impact of an intervention can be considered as a combination of the effectiveness in reducing symptoms and the number of people reached. A guided self-help intervention may be more effective for individuals but will probably reach fewer people. The opposite is true for unguided self-help, although a 2021 systematic review of self-help interventions for depression suggested that guided and unguided self-help interventions may be similarly effective for people with mild symptoms (30). It is always important to consider the intervention that is planned, and how it matches the needs of populations.

Delivering guided self-help requires more financial and human resources than unguided self-help, specifically to set up and run the team of helpers. But regular contact with a helper has many benefits. As well as supporting people to adhere to and complete the intervention and so increase its effectiveness, it gives people regular opportunities for human contact, assessment and referral (see section [2.1.3 Support models for guided self-help](#) for more information on support models and what is needed to provide guidance).

Choosing guided or unguided self-help is not necessarily an either/or decision. There are many options. For example, a programme or service may:

- start by providing unguided self-help and add a guided component when resources become available;
- offer guided self-help where needs are high and offer unguided self-help elsewhere; or
- offer unguided self-help to people with less severe symptoms and offer guided self-help to people with more severe symptoms.

Practical information for general delivery

2. For organizations: how to set up individually delivered self-help

This chapter is primarily written for people responsible for developing or implementing individually delivered self-help interventions. If you want information on being a helper, supervisor or trainer you may want to skip to Chapter [3. Introduction to providing support in guided self-help](#).

Note. In the sections that follow, we set out the key activities to conduct when implementing individually delivered self-help, in line with WHO's *Psychological interventions implementation manual* (29). The implementation manual provides the information needed for planning and implementing psychological interventions delivered by non-specialists and should be read alongside this document.

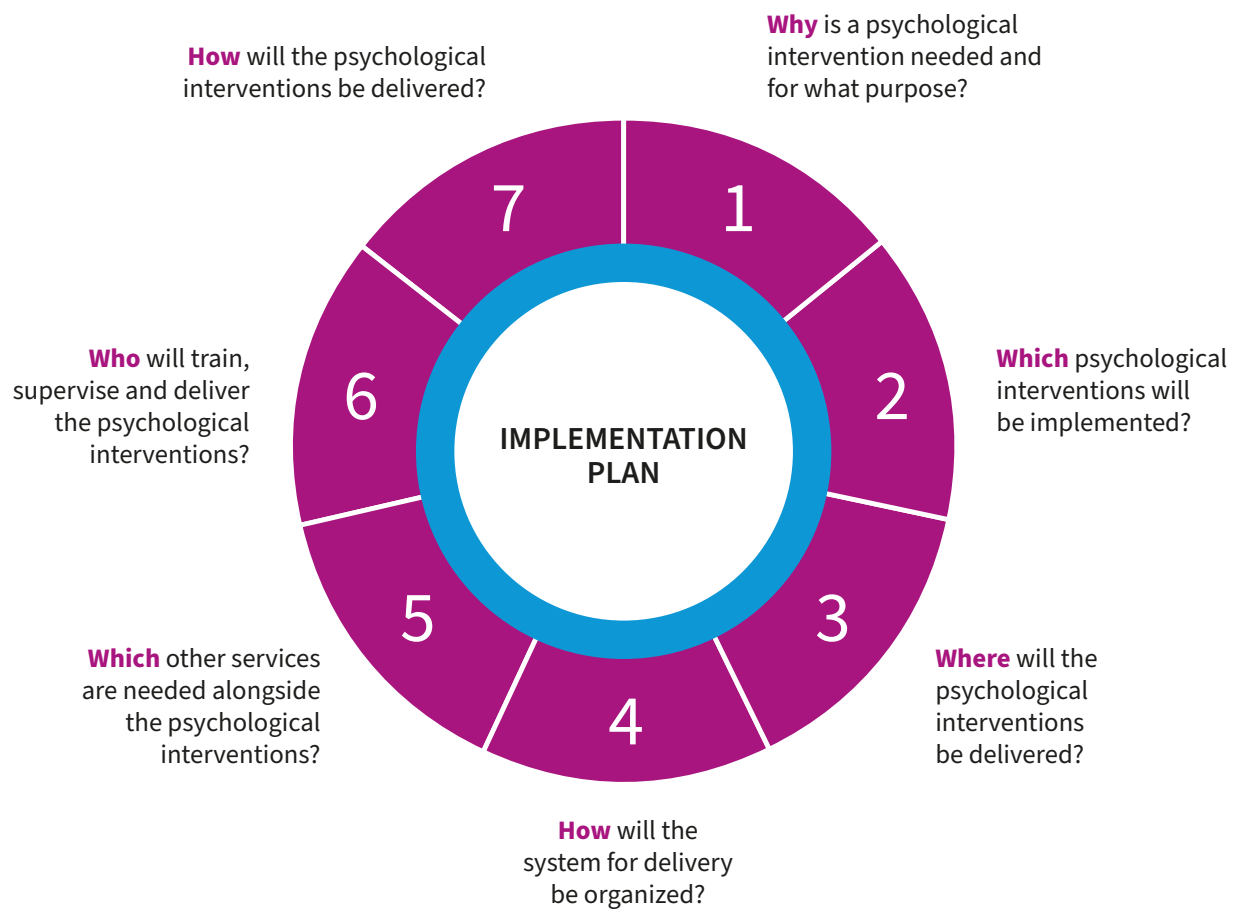
While this manual focuses mainly on individually delivered guided self-help and providing remote support, information covered in this chapter is also relevant for providing individually delivered unguided self-help or in-person support.

Planning for self-help involves making some key decisions before working out an implementation plan. Information needed to make some of these decisions was discussed in Chapter 1, and other key points will be covered here. A checklist for planning and implementing psychological interventions can be found in Annex 1 of WHO's *Psychological interventions implementation manual*.

2.1 Plan for individually delivered self-help

WHO's *Psychological interventions implementation manual* identifies seven key activities for planning psychological interventions ([Figure 2.1 Seven key questions to address in an implementation plan](#)).

Figure 2.1 Seven key questions to address in an implementation plan



Source: WHO (29).

In this section we offer additional information on four of the activities, as these have specific considerations for self-help interventions. It is important to include multiple stakeholders, including community representatives (such as community leaders and people with mental health conditions) in the planning and implementation team.

It is recommended that an implementation plan be developed in the planning phase to ensure that all key questions are addressed (see Annex 1 of WHO's *Psychological interventions implementation manual* for a checklist of key activities in planning for the implementation of a psychological intervention).

2.1.1 Intervention selection

The choice of intervention is informed by a first step of identifying local needs, capacities and priorities. The intervention content should target the mental health conditions identified as local priorities, but always as part of a comprehensive system.

It is important to review the evidence for the intervention to ensure its likely effectiveness. Gathering service user feedback is also critical to ensure that the intervention aligns with people's needs and preferences, for example with regard to literacy, access to digital devices, preferred medium of communication, or use of individual or group interventions. This feedback will guide the choice of intervention and whether an individual guided or unguided intervention is selected, or another form of self-help such as Self-Help Plus (SH+) (31).

The following information relates to individually delivered self-help interventions.

Some digital self-help interventions (both online and through mobile apps) may not have been tested for effectiveness, so it is important to check what evidence supports them. Interventions may be proven effective only for a specific population group, yet this evidence can often be generalized across genders and some age groups (32). Most evidence-based self-help interventions have been developed and tested for people aged over 18 years, but there is a growing number of interventions for younger people. While using an intervention designed for and tested with young people is preferred, in some cases interventions have been adapted for and implemented in other age groups. See WHO's *Psychological interventions implementation manual* for more information on adapting interventions (29).

WHO has developed two individually delivered self-help interventions, SbS and DWM, that have been tested, evaluated and proven to have a positive effect on mental health. Table 2.1 gives an overview of their characteristics. Chapters 5 and 6 give more information on SbS and DWM. The interventions use a similar format (illustrated self-help), but have different content that may require differing levels of adaptation. A first step in deciding whether to use DWM or SbS is reading through the content in full.

Table 2.1 Overview of SbS and DWM

	SbS (see Chapter 5)	DWM (see Chapter 6)
Target population	Adults with moderate to severe depressive symptoms.	Adults experiencing psychological distress of various levels of severity.
Main therapeutic components	Behavioural activation, stress management and other techniques.	Acceptance and commitment therapy. DWM is the book component of Self-Help Plus (SH+) (31) and includes all of the same self-help techniques and content. It has subsequently been used and tested as a stand-alone individually delivered intervention.
Format	Digital, but could be delivered in different formats, such as a book or video.	Digital and book, but could be delivered in different formats, such as a video.
Length	Five modules.	Five modules.
Guidance	One welcome contact and five weekly support contacts of 15 minutes per week.	Tested with both five weekly support contacts and three support contacts over five weeks of 15 minutes.
Adaptation	SbS follows individual storylines developed for different genders and age groups, including a young person story. This helps it feel more personalized but increases the amount of content to adapt.	DWM follows a group of narrators explaining key concepts. Images are already adapted for multiple cultures and text is short, so the amount of adaptation required may be more limited.
Current strength of evidence	Two RCTs in Lebanon showed moderate effects on mental health outcomes that were maintained at three-month follow-up (10, 27). An RCT with an adapted version of SbS with Chinese students showed it effectively reduced depressive symptoms in the short term and improved psychological well-being in the longer term (19). One trial with an unguided version of SbS in Syrian refugees in Egypt showed small effects that remained at three-month follow-up (20), while another trial with the same population in Germany and Sweden did not find an effect (21).	One published pilot trial showing small to moderate pre–post effects on symptoms (22). Two published trials showing moderate pre–post effects on symptoms, as part of a stepped care trial (11, 23). One published pre–post (not RCT) paper on unguided DWM showed significant effects in reducing symptoms of depression, anxiety and distress and improved well-being in over 1400 students in Italy (33).
Not suitable for people who:	... have a severe cognitive impairment, or are at imminent risk of suicide (for example, people with a plan to end their life).	... have a severe cognitive impairment, or are at imminent risk of suicide (for example, people with a plan to end their life).

2.1.2 Delivery options

Several planning activities in WHO's *Psychological interventions implementation manual* are about deciding how to deliver a psychological intervention. For self-help, this includes deciding whether to offer the intervention with or without guidance (or both depending on symptom severity); and deciding what format and media to use (for both content and guidance).

Self-help can be delivered using different models with different levels of guidance. For example, it may be:

- made available to all adults in a community affected by adversity, with no guidance (for example, a self-help book made available in public spaces);
- provided by primary health care workers and others as part of routine care for people with mental health conditions (including significant psychological distress), with brief check-ins during regular follow-up appointments;
- open to all adults in a community affected by adversity, with guidance offered to those who score above the cut-off value on a measure of depression or psychological distress;⁴
- combined with other forms of psychological treatment, for example as part of stepped care, whereby people are offered progressively intensive treatments based on clinical need.

The content in both guided and unguided self-help can be delivered through different media, including a physical book, a mobile application or website, a PDF document, or a series of videos or audio recordings (such as podcasts). Feedback from service users on what they can use or would prefer is very important here (see section [2.2.1 Adapting the intervention](#) on adapting the intervention).

If you choose to deliver a digital intervention or deliver self-help remotely, you will need to ensure that:

- potential service users have access to the internet, a suitable device (smartphone, tablet or computer), and, depending on the registration process, an email address or phone number; and
- the intervention is accessible for people with older mobile devices and slower or unstable internet connections. Making the application usable offline and giving choices on what to download can help make the intervention more inclusive.

For digital interventions, you will also need to ensure that sufficient resources and skills are available to implement, host and maintain a digital delivery platform that secures any personal information and is compliant with any local regulations. Digital resources may be delivered on their own platform or potentially using existing and widely used platforms (such as YouTube or WhatsApp). Key requirements for many digital interventions include:

- a technical strategy based on user needs and preferences to inform the technical design and architecture;

⁴ Contact psych_interventions@who.int for more information.

- a secure mobile application or website (such as an e-learning platform);
- technical capacity to host, maintain and update a digital platform;
- a system to register and support service users with technical issues (such as resetting passwords); and
- policies and practices to ensure compliance with local rules and regulations on data protection, security, use of digital health devices and any other relevant areas.

If the digital self-help intervention includes guidance, additional requirements may include:

- telephones or access to a secure digital video or messaging platform to contact service users; and
- a case record system to record information about the support contacts (such as duration and what was discussed).

There are numerous ways in which SbS, DWM and other evidence-based, individually delivered, psychological self-help interventions can be implemented as part of a comprehensive system of care. For example, you may choose to:

- implement one or the other intervention based on priority problems in your area;
- implement both interventions and direct people to each one depending on their symptoms; or
- provide self-help as a first step in stepped care.

This decision will in large part depend on the needs of the population and the resources available.

2.1.3 Support models for guided self-help

In guided self-help, support and motivation is provided by a person trained to give this guidance. This may be as part of routine appointments, but often it is provided by trained and supervised helpers recruited for this purpose. They provide encouragement, answer questions about the intervention and its content, and make referrals when required.

The following information relates primarily to situations where non-specialist helpers are recruited for this role.

Support can be delivered remotely or in person, either in real time (live support) or with a time delay using pre-recorded voice or written messages (delayed support).

Live support (also called “synchronous” support) happens through a live conversation, typically during a pre-booked appointment slot. Contact can be by telephone, video, instant messaging or in person. Live support is useful because:

- the service user can ask follow-up questions or get immediate clarifications, which can help them progress through the intervention;

- the regular appointment may increase motivation to continue with the intervention; and
- the communication may feel more natural and supportive.

Potential challenges include helpers needing practice to keep support contacts brief (a problem addressed in training and supervision). Remote live support is also only possible with stable internet or phone connections.

Delayed support (also called “asynchronous” support) occurs when the conversation between the helper and the service user is not completed in real time (that is, they leave messages for a response later). The helper will still allocate time to sending their support messages, but this will not be a live conversation. The advantages of this approach are that:

- it is convenient, as service users can choose when and where to engage with the support;
- it may give people time to think about their message and response;
- it is time efficient and less burdensome for helpers as they can work with (adapted) templates; and
- it creates a record (either written or audio, if voice messages are used).

The main drawback of delayed support is that it may feel less natural or engaging than a live call. Yet communication preferences vary across cultures, so for some service users (such as young people) it may be preferred. Delayed support also requires clear communication to ensure service users do not confuse the contact with a live chat and expect to get a direct response. An automatic response reminding people that helpers will reply within a given time frame will help manage communication expectations. When using delayed support, people should be aware of what to do in a crisis situation (for example, imminent risk of suicide or other protection concerns) and there should be a system in place for responding urgently, and with live support (such as a telephone call), to any messages that indicate such risks.

In some of the research for guided versions of both SbS and DWM, service users were offered the choice of live support (talking to the same helper on the telephone for 15 minutes each week) or delayed support (receiving a text or voice message). Most people chose telephone-based support – but having the choice was seen as important. While the evidence clearly shows that guided interventions are effective, there is little research on the different effects of different support types, such as live or delayed support, or telephone or message.

To increase the chance that implementation results reflect trial results, you should use a similar support model as used in the research. For SbS and DWM, this would mean using the support models described in Chapters 3–6 of this manual. Yet this can be adapted to context. Example adaptations include reducing or increasing the number of contacts, using a different technology to provide support (for example, live voice notes rather than telephone calls) or offering support on demand.

Ideally, any adaptations to the support model should be piloted before full implementation to determine whether the intervention remains engaging and effective.

Before deciding what support model to use, consider the technology available in your region, preferred methods of communication within your target population, human resources available and amount of people you want to reach, as well as user feedback on preferences. See [Annex 1. Summary of tasks for different contacts, and example scripts and texts](#) for a summary of tasks for the different support contacts and example templates for live and delayed support that you can adapt for use in your context.

2.1.4 Human resource needs

Within all delivery and support models you will need to consider your human resource needs. The simplest form of guided self-help (not explicitly covered in this manual) is for a service provider to give people access to the self-help material and then check in during routine appointments (covering some of the topics listed in section [4.2 Regular support contacts](#)).

To implement SbS or DWM, you will need a team of people to carry out various tasks ([Table 2.2](#)). This team could comprise people already employed by the organization or those specifically hired to deliver individually delivered self-help.

Table 2.2 Example tasks needed to implement a programme of guided self-help interventions using the WHO model described in this manual

Task	Example roles and responsibilities
Coordination	• Manage the programme of self-help • Assess the quality of the programme's implementation • Oversee the programme's administrative and operational elements (including monitoring and evaluation) • Ensure helpers are recruited and trained • Allocate helpers to service users at sign-up
Providing support	• Give service users brief support and encouragement as they progress through the intervention • Maintain privacy • Respond to any safety or protection concerns, such as imminent risk of suicide • (See section 3.1 for more information on what it means to be a helper)
Supervision	• Provide training and support to helpers • Ensure that support is delivered competently and safely • Follow up on any safety or protection concerns
IT support (for digital intervention)	• Ensure hosting and maintenance of the intervention • Provide helpers and service users with technical support

When delivering interventions remotely, it is important to develop clear rules and practices (also called standard operating procedures) for how helpers contact service users and make case notes. For example, helpers should only contact service users using work devices, during working hours and in ways that ensure the safety and privacy both of themselves and of the people they are helping (that is, not making calls from home).

Even when delivering self-help interventions remotely, it can be useful to have the team of helpers working in a centralized location to create a supportive atmosphere.

For information on how to prepare the team through recruitment, training and supervision, see section [2.3 Prepare the workforce](#).

2.1.5 Monitoring and evaluation

As with implementing all psychological interventions, it is essential to make a monitoring and evaluation plan to effectively measure the impact of implementing self-help interventions.

For information on making a plan and important indicators for psychological interventions, see WHO's *Psychological interventions implementation manual* (29).

2.2 Adapt for context and culture

Before implementing your chosen self-help intervention, you should consider how well it fits the local context and culture of your target population, and adapt it where necessary to maximize engagement and effectiveness. Chapter 3 of WHO's *Psychological interventions implementation manual* sets out the goals of and strategies for adaptation of diverse psychological interventions (29).

2.2.1 Adapting the intervention

The adaptation of self-help interventions may involve changes to both their content and format. For example, if the intervention is being delivered for a specific population or situation, such as health care workers during a pandemic, the content (such as illustrations) may need adapting to reflect those experiences (34). If the intervention follows a story, as in the case of SbS, then characters and parts of the story may be changed to make them more culturally appropriate or to address context-specific stressors and experiences (Box 2.1). For example, depending on the context, a doctor character that provides psychoeducational content can be replaced with another trusted, culturally appropriate character, for example a health care professional, a community helper, a community elder or a peer. Language translation and updates to audio or visual content will be necessary to ensure cultural appropriateness.

When adapting self-help interventions that use substantial illustrations, audio scripts or videos, adopting an iterative approach may help reduce errors that would be costly to fix later on. Digital interventions are particularly difficult and expensive to change in later stages. Rather than redrawing all illustrations or re-recording an entire audio intervention, try adapting one section at a time,

testing each one with the local community before moving on to the next section. This will help get the language, tone and visual presentation of the intervention right. It is also important to make self-help materials fun and easy to use, as there is limited human contact to ensure adherence. Looking at television adverts or radio shows can give an idea of the warmth or tone needed for self-help materials.

Piloting the adapted intervention with a small number of service users first is advised. This will give information about how well the adaptations work and if there is a need to make more adaptations.

Box 2.1 Examples of SbS adapted for local context

SbS uses narrative stories with illustrations. It has been adapted for various target populations using a cultural adaptation process involving substantial engagement with local community members and other stakeholders.

Lebanese, Syrian refugees and other populations in Lebanon.^a SbS, in collaboration with the National Mental Health Programme at Lebanon’s Ministry of Public Health, was adapted following interviews with the target population to reflect gender, language and cultural nuances among different populations. Two male and female versions were developed, which share the same content but feature different illustrated characters representing Lebanon’s main cultural groups. Exercise examples were revised to reflect local experiences. To increase acceptability, service users choose their preferred method, day and time of contact with a helper.

Filipino migrant workers.^b Filipino overseas workers in Macao Special Administrative Region, China, reported several issues with the generic version of SbS. Key adaptations to address these included changing the narrative to match Filipino experiences and values; changing some activities so they were more familiar to overseas Filipino workers; changing the main character from a doctor to a wise elder; and simplifying and translating the text, including by using a mix of English and Filipino, where appropriate.

Chinese young adults.^c Based on feedback from focus group discussions, changes were made to the language, narratives and characters used, as well as some of the concepts and exercises. This included adding scenarios related to university life, changing the main character from a doctor to a peer, changing expressions to fit young adults’ experiences and translating the intervention into both traditional and simplified Chinese.

Thailand: integration into the Tor-Tuem-Jai Programme.^d Thailand has integrated two WHO tools – Step-by-Step (SbS) and Doing What Matters in Times of Stress (DWM) – into a unified digital platform called Tor-Tuem-Jai. The content was carefully adapted through a four-stage process to ensure cultural relevance, including redesigning characters and illustrations, consulting experts and refining materials based on community feedback.

The programme is available for all Thais, which reduces stigma and broadens access, and uses a stepped-care model for efficiency, whereby trained “e-helpers” support users reporting

moderate-to-high levels of distress. Sustainability is ensured through a memorandum of understanding with the Praboromarajchanok Institute under Thailand's Ministry of Public Health, which has embedded e-helper training into the national health sciences curriculum, helping to ensure a sustainable workforce. Endorsed by a Thai Cabinet resolution as a national pilot project, Tor-Tuem-Jai now has both systemic partnerships and national policy support for long-term expansion.

Adaptation for WhatsApp in India.^e SbS was substantially adapted by Kaya Guides for delivery through WhatsApp to Indian youths. WhatsApp was chosen due to its familiarity and wide use in India. The adaptation process prioritized retaining core therapeutic elements while reimagining storylines and characters to reflect Indian experiences across gender, religious and socioeconomic contexts. Key adaptations included rewriting narratives to centre on challenges faced by young people in India, translating into Hindi using Latin script with some additional English words, and restructuring delivery as short, engaging videos similar to social media content.

Sources: a: Abi Ramia et al. (35); b: Garabiles, Harper-Shehadeh and Hall (36); c: Sit et al. (37); d: WHO (38); e: Kaya Guides (39).

2.2.2 Adapting protocols and guidance materials

Besides adapting the intervention content and format, it is important to adapt all the protocols and guidance given to helpers and supervisors to the local context.

The information provided in Chapter [3. Introduction to providing support in guided self-help](#) (on how to provide support in individually delivered guided self-help) is based on the approach used to run SbS and DWM. Many sections state that further information will be provided by the implementing organization. As the programme manager or implementing organization, it is important for you to read through Chapter 3 and adapt the approach to your own organization and setting before sharing the information with helpers.

It is especially important to adapt (or develop) protocols or guidance for helpers on how to:

- manage safety and protection concerns;
- refer people in need of other support or services;
- deal with unsuccessful contacts;
- deal with a friend or family member who calls to ask about the intervention; and
- remind service users to complete any final questionnaires and inform them about any further follow-ups or assessments.

It is also important to ensure you have protocols or guidance for coordinators, for example on how to monitor and evaluate the service (including which measures to use and when).

If you are using any of the tools and templates provided in the annexes to this manual (for example, the list of supporting organizations for referral, contact templates and supervision materials), these will also need adapting to your context before being shared with the self-help intervention team.

2.2.3 Engagement

The intervention will also need to be adapted to ensure it is as engaging as possible to the population you want to reach. Self-help interventions often have lower adherence and engagement rates than interventions delivered fully by a person. Dropping out – that is, not finishing the intervention – is common in mental health interventions, but may be higher in self-help interventions (40). For example, a meta-analysis of digital interventions for depression reported that on average 25% of participants completed the intervention when delivered in routine clinical settings (41).

There is limited evidence on what exactly increases adherence (42). In general, it seems that adding guidance to the self-help intervention increases adherence (43). Also, ensuring that the intervention is adapted to the needs and context of the population will increase the chances that people are more engaged.

In the design of digital self-help interventions, adding features such as nudges, reminders and mood tracking may increase adherence. For example, adding a mood monitor to smartphone interventions has been shown to lower drop-out (44).

2.3 Prepare the workforce

Chapter 4 of WHO's *Psychological interventions implementation manual* provides information on preparing the workforce, including tips and tools for recruiting, training and supervising helpers (29). In addition, WHO's Ensuring Quality in Psychological Support (EQUIP) initiative gives information on how competency-based training and assessment can be integrated (45).

The sections that follow focus on extra information that is particularly relevant to guided self-help and are based on the SbS and DWM model tested in research trials. In this manual, a “helper” refers to a non-specialist.

2.3.1 Recruitment

To implement guided self-help interventions using the support model described for SbS and DWM, you will need to recruit helpers, supervisors and trainers. Because the helper's role is to support people through the intervention, not to provide specialized mental health support, they do not need to be highly educated or experienced mental health workers. They could for example be non-specialist community workers, educators, students or graduates. These helpers will need training in the guided self-help intervention they are supporting.

When recruiting non-specialist helpers, you will want to engage people who:

- have a genuine motivation to help others;
- can work or have previously worked with people who may be in distress;
- have an adequate educational background to manage programme materials (for example, have completed at least high school);
- speak the same language and ideally have a similar cultural background to service users;
- are interested in learning how to provide support as a helper;
- can access ongoing support and supervision from a trained supervisor; and
- have enough time available to deliver the self-help.

All these characteristics support non-specialist helpers to excel at their role. For information on other attitudes, skills and experiences to look for when recruiting helpers, see section 4.1 of WHO's *Psychological interventions implementation manual* (29).

Health professionals such as doctors and nurses in primary health care can also provide guided self-help, although the support model they follow may need adjusting, for example by integrating support into routine follow-up appointments.

Helpers experienced in providing psychological interventions (for example, mental health professionals or people who have delivered Problem Management Plus) should still complete all training activities. However, they can probably complete the training more quickly than inexperienced helpers, as they already understand basic helping skills and know how to identify and respond to imminent risk of suicide and other safety or protection issues. Ongoing supervision is still required.

Supervisors and trainers are ideally mental health professionals who are competent and experienced in delivering psychological interventions or guided self-help. In some contexts this will not be feasible, and people who do not have a mental health background but are trained and experienced in providing guided self-help interventions can be trainers or supervisors under the supervision of a mental health professional.

When planning recruitment, think about how many helpers, supervisors and trainers you will need, what their working hours will be and what a manageable caseload for helpers might be. Helpers typically need at least 30 minutes per service user per contact, which includes preparation time, the 15-minute contact and writing up case notes. Time should also be available for attending supervision sessions. Manageable caseloads vary by context, so you will need to consider what is appropriate in your setting.

2.3.2 Training

Training to support guided self-help can be quicker than for some other psychological interventions. This is because in guided self-help, helpers do not have to explain concepts in depth or engage with people for extended periods of time. Training is however still essential. WHO recommends taking a competency-based training approach. For more information on competency-based training see WHO's Ensuring Quality in Psychological Support (EQUIP) initiative (45) and section 4.2 of WHO's *Psychological interventions implementation manual* (29).

Training generally happens in three stages:

- **Preparation:** becoming familiar with the intervention's content and materials.
- **Workshop:** attending an interactive training workshop, which includes hands-on practice of helper skills. The duration of the workshop will depend on the intervention. Training for SbS and DWM each take four to five days.
- **Practice and ongoing supervision:** doing practice cases and delivering the intervention to real service users, while receiving ongoing supervision and support.

Training manuals for SbS and DWM will be published by WHO.⁵

2.3.3 Supportive supervision

Supervisors are ultimately responsible for ensuring that the intervention – including the support given by helpers – is delivered competently and safely. Supportive supervision ensures that helpers provide quality interventions and continue developing and refining their helping skills.

Section 4.3 of WHO's *Psychological interventions implementation manual* describes the components of supportive supervision and sets out different options for how, where and how often to provide it (29).

The level of supervision required by helpers will vary according to their level of expertise and experience. For example, lay volunteers who are new to providing help for mental health problems may need a lot of supervision, while a doctor in primary care who is used to talking to people with mental health problems may need limited supervision when providing self-help.

In the research trials of SbS and DWM, supervisors met their helpers (non-specialists) regularly, typically in a group, for around 60–90 minutes once a week or fortnightly. Those experiences suggested that supervision was especially valuable to support helpers in keeping their contacts brief and learning how to manage people who were expecting a therapy session or who wanted or needed more support than could be provided.

⁵ Contact psych_interventions@who.int for more information.

2.4 Helping people to access self-help

Before, during and after participating in self-help, it is important for people to receive the support they need. Key questions to consider include:

- How and where will the intervention be advertised or promoted?
- Where will referrals come from, and how does this fit into existing services?
- How will people access the self-help intervention? For example, will they need a referral from an existing service? Or will they fill out an online questionnaire and be automatically referred based on their results?
- How and where will people with extra support needs be referred to or informed about other services and supports?

As with all psychological interventions, appropriate referral and support options for both mental health concerns (for example, access to a trained mhGAP clinician or referral to a local specialized service) and protection concerns (for example, gender-based violence and violence against children) should be available. When implementing self-help interventions, it is important to map out available services (such as health, rehabilitation, education, protection and social services) and build or strengthen multisectoral links for referral. Referral options for people at imminent risk of suicide and for people with severe mental, neurological or substance use conditions must be in place. When referral options are limited, they can be created by training and supervising general health care providers to assess and manage mental, neurological and substance use conditions in the local health system, for example through mhGAP.

Chapter 5 of WHO's *Psychological interventions implementation manual* describes how to identify, treat and follow up potential users of a psychological intervention – including how to refer people to other services if the intervention is not suitable – to ensure that people get the support they need (29). When implementing guided self-help there are some other points to consider, as described below.

2.4.1 Identifying, assessing and engaging people for self-help

For some self-help interventions, including SbS, it is important to identify potential beneficiaries by some form of case detection through initial assessment or screening. This ensures:

- a match between what the person needs and what the intervention can provide;
- that the intervention is offered to those most likely to benefit from it; and
- that people who may need extra support – for example, because they are at imminent risk of suicide, self-harm or harm from others – are identified and referred appropriately.

Screening and assessment will vary by context but aims to identify people who may be in need of, and benefit from, the intervention. For example, in self-help this may be done as part of routine clinical care, by screening people in the community who are identified as being at risk of the particular problem (such as depression), or online, by people accessing the screening tool after seeing a message or post advertising the intervention. Screening and assessment may be done in person or online. They may be done together or as part of a multistep process in which initial case detection is followed by further assessment where indicated ([Box 2.2 Example process for online screening and assessment based on SbS and DWM](#)). For more information on case detection methods see WHO's *Psychological interventions implementation manual* (29).

Box 2.2 Example process for online screening and assessment based on SbS and DWM

Depending on the specifics of the screening and assessment process, basic demographic and contact information, including emergency contact, and an understanding of a person's ability to engage with the materials (for example, basic literacy and, for digital interventions, ability to use a mobile app) need to be collected at step 1 or step 2.

Step 1. Screen for suitability based on a validated measure

Screening can be done using a locally validated measure of psychological distress or symptoms of common mental health conditions, such as depression and anxiety.^a If the intervention is being delivered through a digital platform, screening may be done as part of the sign-up process.

You should choose your own screening criteria and outcome based on your local context and intended use of the intervention. You might decide to:

- only provide the intervention to people who score above the cut-off value; or
- provide the intervention as guided to people who score above a certain cut-off value but still offer the intervention, in an unguided format, to everyone else.

Screening should include questions to assess whether the person has other support needs or whether there is any reason to exclude them from the intervention. This includes questions to assess for imminent risk of suicide, self-harm or any other protection concerns or needs that may require a higher level of care (such as substance dependence or psychosis). These screening questionnaires can be delivered in many ways, for example by a trained and supervised non-specialist as with other WHO psychological interventions, or online, potentially with a follow-up call to discuss the results of the screening. Decisions on how to respond to these should be based on local procedures and policies. For example, people indicating a possible need for additional services should receive a referral for these. Similarly, in online screening, people who indicate imminent risk of suicide or other protection concerns may receive a follow-up contact from a programme manager. Once the imminent risk has been managed, they can still be invited to join the intervention.

Step 2. Further assessment

Individuals who are suitable for the intervention and want to take part in it can be asked further assessment questions, for example to identify:

- level of well-being or functioning: brief weekly measures to assess symptoms can also be used by helpers to assess ongoing risk, help tailor support and determine if the intervention is helping;
- day and time preferences for support contact (if offered guided self-help);
- brief weekly measures to assess symptoms can also be integrated into the self-help programme to guide helpers in tailoring support (if offered guided self-help).

Make sure that any further assessment is not too long, so that people remain engaged in the intervention.

Step 3. End of programme assessment

At the end of the self-help programme, the assessment measures should be repeated. Depending on local resources, their results can be used to determine whether the person needs further support and for monitoring and evaluation purposes. The outcomes across a range of people might also suggest the need for further adaptation of the intervention.

^a Examples of locally validated measures include the Patient Health Questionnaire (PHQ-9), Generalized Anxiety Disorder-7 (GAD-7) and the Kessler 6 (K-6).

In guided self-help, after screening and assessment, anyone suitable for the guided self-help intervention must be allocated to a helper for support, through an agreed method of communication (which may be based on personal preference).

Providing clear information about the intervention is an important part of the screening and assessment process. Individuals need to understand what the programme involves and what to expect. This information can be delivered through the intervention itself (for example, in a mobile app or website) or by the person introducing it (for example, a helper or health professional). Because self-help interventions are easy to access, people may sign up without a full understanding of the commitment required. Experience with SbS and DWM suggests that a call or contact with everyone who registers to use the programme, even if completed without guidance, may increase engagement and reduce drop-out by allowing people to ask questions and decide if the programme is right for them. Similar to other psychological interventions, an informed consent procedure should take place after providing information about the intervention.

For more information on recruitment and identification of people for self-help see WHO's *Psychological interventions implementation manual* (29).

2.4.2 Addressing safety and protection concerns

The system for responding to safety and protection concerns (such as imminent risk of suicide or harm from others) and for connecting people to external services and support will depend on local policies and practices, as well as how the intervention is being provided (online, remote or in person). If an intervention is being delivered remotely, extra care may be needed to ensure adequate response to safety and protection concerns. To support helpers manage risk remotely, together with a mental health specialist, consider developing policies, protocols or training on:

- how to create a safety plan in the first remote support contact, where required;
- how to establish where the service user is during a support contact and whether they are safe; and
- how to respond to emergency situations such as imminent risk of suicide (for example, having contact information for referral services, such as emergency health services or a health clinic with staff trained in mhGAP).

See Annexes [2A. Service user experiences distress during a contact](#) and [2B. Service user needs more support for other problems in their life than can be provided in a brief support contact](#) for example protocols (that can be adapted to your context) for assessing and responding to cases of imminent risk of suicide or self-harm, and cases of sexual assault or intimate partner violence.

2.4.3 Managing caseloads and follow-up

One challenge with self-help interventions is that drop-out can be very high, especially at the start of online interventions. This is because the threshold and effort to sign up are very low.

Consider what will happen if someone stops engaging in the intervention, and decide:

- how many missed appointments or contact attempts will be allowed before helpers can stop contacting the person;
- what will be done with people who want to pause the intervention for some time; and
- how much time people can take to complete the intervention and receive support from a helper.

These questions are important to avoid helpers developing full caseloads of people who are not engaging with the intervention. It may be helpful to have someone in the service whose role it is to contact people before they are assigned to a helper and added to the caseload. Establishing early contact and providing an opportunity for people to ask questions about the intervention may help reduce the number of early drop-outs, as it improves informed consent and helps manage expectations.

Also consider what will happen when people finish the intervention, particularly if they are still experiencing high levels of distress (see section [4.3 End contact](#)). Have helpers or the intervention platform repeat the screening measure at discharge to monitor change and identify people who need further support. A system is needed to track participants, record case notes and manage appointments.

3. Introduction to providing support in guided self-help

This chapter is primarily written for helpers who support people through a guided self-help intervention. Programme managers may also find it useful to get a thorough understanding of how helper guidance should work in practice.

In the sections that follow, we outline the general support model used in RCTs for SbS and DWM (10, 23). It assumes that, as in the trials, helpers will be non-specialists with limited or no previous experience of working in mental health care. Much of the information here may also be helpful to health care providers delivering guided self-help as part of routine appointments.

As with other parts of this manual, the advice given below can and should be adapted to specific organizational needs. For example, it assumes you are providing remote support, but can easily be adapted for in-person support. Similarly, it recommends weekly contacts with the helper, but implementing organizations may have good reason to change the frequency of contacts. For example, there is early evidence that DWM is effective with only three contacts (at the start, after two weeks and at the end) (22). Even if you are not planning to use SbS or DWM, you can use the advice in this manual to help plan and implement other evidence-based guided self-help.

Chapters 5 and 6 of this manual offer specific tips and tools on how to support people through SbS and DWM. A training manual for SbS and DWM helpers will be published by WHO and will give further information on how helpers should provide support for these interventions.⁶

Note for programme managers. Many sections in this chapter suggest that further information will be provided by the implementing organization. Please read through this chapter and adapt the approach to your setting before sharing any information with helpers.

3.1 What does it mean to be a helper?

Helpers are trained to support people through a psychological intervention (including guided self-help). Helpers are not like professional therapists who give people counselling or advice. Instead, they work alongside a self-help intervention. The intervention provides the therapeutic content; helpers are there to offer brief, regular support to keep people on track, help them with any problems they encounter, and steer them through the intervention. They communicate with service users through real-time conversations or pre-recorded voice or written messages (see section [2.1.3 Support models for guided self-help](#)).

⁶ Contact psych_interventions@who.int for more information.

Your implementing organization will tell you which type of support to provide, when and for how long. In [Annex 1. Summary of tasks for different contacts, and example scripts and texts](#) you will find a series of templates to help you provide support (although you should always personalize these to the person you are helping).

3.1.1 Roles and responsibilities

As a helper you will:

- welcome each service user to the intervention;
- guide people through the intervention, helping them to engage, learn and use the self-help skills and exercises in their everyday life (including by answering questions or clarifying techniques and listening to other problems they are experiencing);
- make regular contact with all service users (typically 15 minutes each week for DWM and SbS) by phone, video or message, as determined by your organization;
- follow the intervention;
- maintain service users' privacy and safely keep case notes and contact records, as requested by your organization (see [Annex 5. Case note examples](#) for example case note templates);
- listen to service users and manage safety concerns according to established protocols (with supervision and support) ([Box 3.1](#));
- attend and actively participate in regular supervision;
- adhere to the principles and processes outlined in this manual;
- manage your time and schedule for contacting service users; and
- maintain your own well-being, by accepting support from your supervisor and peers and by prioritizing self-care activities.

If you are helping with a digital intervention, as well as the list above, you may also be expected to give service users technical support on, for example, obtaining a user name, resetting a password and navigating the intervention (for example, how to access exercises or move on to the next module). Your organization will tell you the process for this.

You do not need to be an expert in the theory behind guided self-help and should not get into long discussions about the therapeutic techniques and activities, since this is provided by the intervention itself.

Box 3.1 Addressing safety concerns

Helpers are not solely responsible for managing safety concerns, such as imminent risk of suicide.

Your role in these situations is to identify the safety concern, follow the protocol and inform your supervisor. Your supervisor will have sufficient skills and training to manage the situation. They are ultimately responsible for ensuring people's safety as they complete the guided self-help.

See section [4.8 How do I manage imminent risk of suicide and other immediate safety concerns?](#) for more information.

3.1.2 Ethics and principles

Helpers must have a good understanding of the local culture. Cultures can vary enormously within countries, regions and communities, and it is important to be aware of potential differences. This is something you can talk to your supervisor about ([46](#)).

Some helpers (for example, health professionals) may belong to a regulated health profession. If this is the case, you need to follow the ethical standards and guidelines of that profession. All helpers need to treat all service users with respect, regardless of their characteristics, including age, sex, gender identity, sexual orientation, culture, religion, political beliefs, race, ethnicity, physical or mental abilities, or economic or social background. Discrimination in any form is not acceptable. [Table 3.1 Some dos and don'ts of helping people through guided self-help](#) lists some dos and don'ts of helping people through guided self-help.

Table 3.1 Some dos and don'ts of helping people through guided self-help

Do ...	Don't ...
• be honest and trustworthy; • be sensitive and empathetic; • respect people's rights to make their own decisions; • be aware of and set aside personal biases, judgements and prejudices; • make it clear to people that even if they refuse help now, they can still access help later; • respect privacy and keep people's stories confidential; • know protocols and referral pathways for people who need more or other support; • behave appropriately regardless of a person's culture, religion, age or gender.	• exploit your relationship as a helper; • ask people for money or favours; • make false promises or give false information; • give the impression you are a specialized therapist or providing therapy; • provide advice beyond your role; • force help on people, or be intrusive or pushy; • pressure people to tell their story; • judge a person's actions or feelings.

3.1.3 Training and supervision

Learning to be a helper happens in three stages. It starts as soon as you sign up to be a helper and continues well after any classroom training, for as long as you continue to support people through guided self-help.

Stage I: Before classroom training

In this stage, you will:

- familiarize yourself with the intervention (for example, by reading through the self-help content and understanding the rationale for the intervention);
- complete the intervention yourself, working through every module – ideally, a knowledgeable helper can help you to experience what it is like to receive the support; and
- practise the exercises in the intervention.

Stage II: Classroom training

During classroom training, through teaching and role play you will:

- learn basic helping skills (see section [3.2 What helping skills do I need to be a helper?](#)); and
- learn about mental health and presentations of distress, and how they affect people's well-being and functioning (the type and extent of this learning will be decided by your organization).

You will also learn how to:

- support people through the intervention;
- identify and manage safety concerns;
- build up trust and confidence within a short time;
- manage the limitations of your role;
- use the digital platform (if relevant); and
- keep a record of contacts with users.

If you are providing support remotely, you should receive extra training in how to do this. During training, you will develop the competencies you need through role play and other learning activities.

Stage III: After classroom training

Even after any classroom training ends, you will continue to develop your helping skills under supportive supervision and strengthen your competency to deliver support.

- You will do practice cases, in which you will provide support to two people with mild symptoms under close supervision. If this is not possible, for example because the intervention is delivered remotely, mock cases (in which people play a role of someone in distress) can be used as an alternative.
- You will continue to use role play to help your learning.
- You will repeat the intervention yourself, and practise the exercises at home, as many times as you need to know it well.
- You will attend regular supervision and any further training that is required. Supervision is typically in a group, for around 60–90 minutes once a week or fortnightly.

Supervision is a critical part of any psychological intervention. Throughout your role as a helper, your supervisor will be there to help you build your skills and offer support and guidance, especially for difficult cases. You will not be alone.

Supervision forms can be used to prepare for supervision and think about what issues you want to raise with your supervisor (see [Annex 4. Supervision templates](#) for example supervision forms). Remember that there is no such thing as a silly question and that if you have thought it, other helpers are likely to have thought it too.

In emergency cases – such as imminent risk of suicide, self-harm or harm to or from others – you can approach your supervisor outside supervision sessions for some one-to-one support.

3.2 What helping skills do I need to be a helper?

Helpers use basic helping skills (also known as foundational helping skills) to support people through guided self-help. These skills build a sense of trust, confidence and security. They help service users feel listened to, accepted and valued.

In this section we outline the basic helping skills you will need as a helper. Note that these are different from the skills that service users learn when they complete the self-help interventions, which are described in Chapters [5. Information for helpers delivering Step-by-Step](#) and [6. Information for helpers delivering Doing What Matters in Times of Stress \(DWM\)](#) of this document.

Remember the limitations of your role: it is the self-help intervention, not you, that teaches people the skills they need to manage their mental health conditions. You are not there to be a therapist or to give people advice.

3.2.1 Communicate verbally and non-verbally

The way you communicate with someone in distress is very important; see [Table 3.2 Some dos and don'ts of communicating with service users of guided self-help](#) for some dos and don'ts of communicating with service users of guided self-help. To communicate well, you should be aware of both your words and your tone of voice; your body language, including facial expressions, eye contact and gestures; and the way you sit or stand in relation to other people.

Communicating well can be more challenging if you deliver support by telephone or written messages. But it is still possible. You should always give people your full attention and consider the tone, volume and speed of your speech and how this may be interpreted. It can help to imagine the person sitting in front of you and use your body language (such as nodding or hand gestures) as you would if they were there. This can help you keep focused and improve your verbal communication.

Remember that each culture has its own ways of behaving that are appropriate and respectful. Always speak and behave in ways that consider important characteristics, including age, sex, gender identity, sexual orientation, culture, religion, political beliefs, race, ethnicity, physical or mental abilities, or economic or social background.

Table 3.2 Some dos and don'ts of communicating with service users of guided self-help

Do ...	Don't ...
• respect the person's privacy and confidentiality; • let the person know you are listening (if using a telephone say "hmm ..." or "I see"; • if using messaging, acknowledge what the person said in their last message and always personalize replies); • be patient and calm; • give accurate information; • be honest about what you know and don't know (for example, "I don't know, but I will try to find out about that for you"); • give information in a way the person can understand – keep it simple; • acknowledge how the person is feeling and any losses or important events they tell you about; • acknowledge the person's strengths; • respect the person's beliefs or spirituality; • politely steer the conversation away if the person asks you personal questions; • if using a telephone, call from a quiet place to talk and minimize outside distractions, and allow for some silence in the conversation.	• pressure the person into telling you anything; • interrupt or rush the person when they are speaking; • judge what the person has or hasn't done or how they feel; • make up things you don't know; • use terms that are too technical or stigmatizing; • tell the person someone else's story; • talk about your own troubles; • give false promises; • try to solve the person's problems for them; • if using written messages, don't use emojis or abbreviations (for example, write "you" instead of "u").

3.2.2 Maintain confidentiality

People completing guided self-help need to know that the information they give you will remain confidential or private and will not be shared with anyone outside your team. They also need to be aware of limits to confidentiality. For example, you can share information about a person without their consent if there is a serious risk of harm to themselves or others. This includes imminent risk of suicide or self-harm, threat to life, risk of sexual violence or intimate partner violence, and child protection concerns (see section [4.8 How do I manage imminent risk of suicide and other immediate safety concerns?](#)).

Service users also need to be informed that you attend supervision, and that information about them might be shared with your supervisor and members of your team to ensure that they receive the best service. They may need to be reminded that everyone in your team is bound by the same confidentiality rules. Only share information with your team when you have to.

As part of maintaining confidentiality, you should keep all information about service users (such as assessment results and personal details) in a safe place. Make sure you follow your organization's rules about data security (including access, passwords, copying and forwarding information, and using company email accounts).

If delivering help remotely, you may need additional protections to ensure confidentiality (see section [4.6 How to protect privacy during contacts](#)).

If someone from outside your intervention team calls asking about a service user, tell them that as part of a health programme, you cannot give out information (see section [4.6 How to protect privacy during contacts](#)).

3.2.3 Show empathy and concern

It is important that service users feel heard (validated and acknowledged), so be sure to show empathy and concern. At the same time, remember that it is not your role to solve people's problems. You are there to listen to them, help them learn and apply self-help skills, and to connect them to other sources of support when needed.

Examples of statements you might use to show empathy and concern include:

- *"That sounds like it was very upsetting / challenging / frightening."*
- *"You have experienced many difficulties."*
- *"You have been through a lot."*
- *"Thank you for telling me that. It sounds like you have a lot of problems in your life right now and it's not surprising that you feel this way."*

These statements do not need to be used word for word. Rather, they show the general type of thing that can be said, and probably need adapting to your context.

3.2.4 Acknowledge emotions

When working with people experiencing high levels of distress, it is important to acknowledge their feelings. You can do this by suggesting the emotion they might be experiencing. For example:

- *"It is very sad that this has happened to you."*
- *"I can see that this is probably frustrating for you."*

- *“I can see your situation is very painful / stressful / upsetting.”*
- *“You have many worries, but you are trying your best.”*

As much as possible, try not to say, *“I understand how you feel / I know how you feel.”* This might feel dismissive and could cause a negative reaction, such as, *“How can you possibly know how I feel?!”*

Instead, you can use phrases such as *“I can see this is probably (upsetting / difficult) for you,”* or *“This seems really hard for you.”* These statements show empathy but do not suggest that you know how the person is feeling.

3.2.5 Put aside personal values

You must respect the personal values and beliefs of the people you are helping, even if you do not agree with them. Refrain from judging people who may appear to be doing little to help themselves, as they may not have the skills or resources. Working to put aside personal beliefs or values is important because it helps you interact with people in a neutral way.

3.2.6 Avoid giving advice

As a helper, you should not give advice to service users or tell them what to do if they speak to you about a problem. Giving advice means telling someone what they should or should not do. This is different from offering several suggestions to choose from, sharing options that have been useful for other people, or connecting someone with an organization for extra support.

All helpers will feel tempted to give advice sometimes. This is very common. For example, it might be very tempting when a person is reporting signs of stress or depression to tell them what you think they should do (for example, *“You should talk to your partner about this”*).

It is important to avoid giving advice. Instead, follow the steps in our troubleshooting guide, which outlines how to help the person use their own coping strategies (see [Annex 2. Troubleshooting templates](#)).

3.2.7 Practise self-care and attend supervision

All helpers should take care of themselves, so they remain able to take care of others. Supporting people who are distressed or unwell can increase your own stress. You may hear about traumatic events – such as injury, violence or loss – which can be distressing. Other common causes of helper stress include life problems, the emotional nature of the work, feeling a sense of responsibility for service users’ well-being, feeling unable to help someone, or work-related frustrations (such as people dropping out, low engagement, people not responding).

Here are some suggestions to manage your own stress.

- Regularly practise exercises from the self-help intervention (these are designed to help people manage stress).
- Think about what helped you cope with difficulties in the past.
- Take time to eat, rest and relax, even for short periods.
- Get adequate sleep and exercise.
- Keep reasonable working hours.
- Minimize your intake of alcohol, caffeine and nicotine, and avoid non-prescription drugs.
- Use supervision to discuss difficult issues that come up.
- Ask for support from, and give support to, your fellow helpers.
- Connect with friends, loved ones or other trusted people.
- Remember that not everyone will improve or be able or motivated to engage fully. Raise issues in supervision to find out if there is anything more you can do to help the person.
- Reflect on the positive experiences you had in your helper role that week and give yourself credit for your efforts and the good job you are doing. Remember that by doing your work, you are helping many people in distress.

4. How to provide support to service users

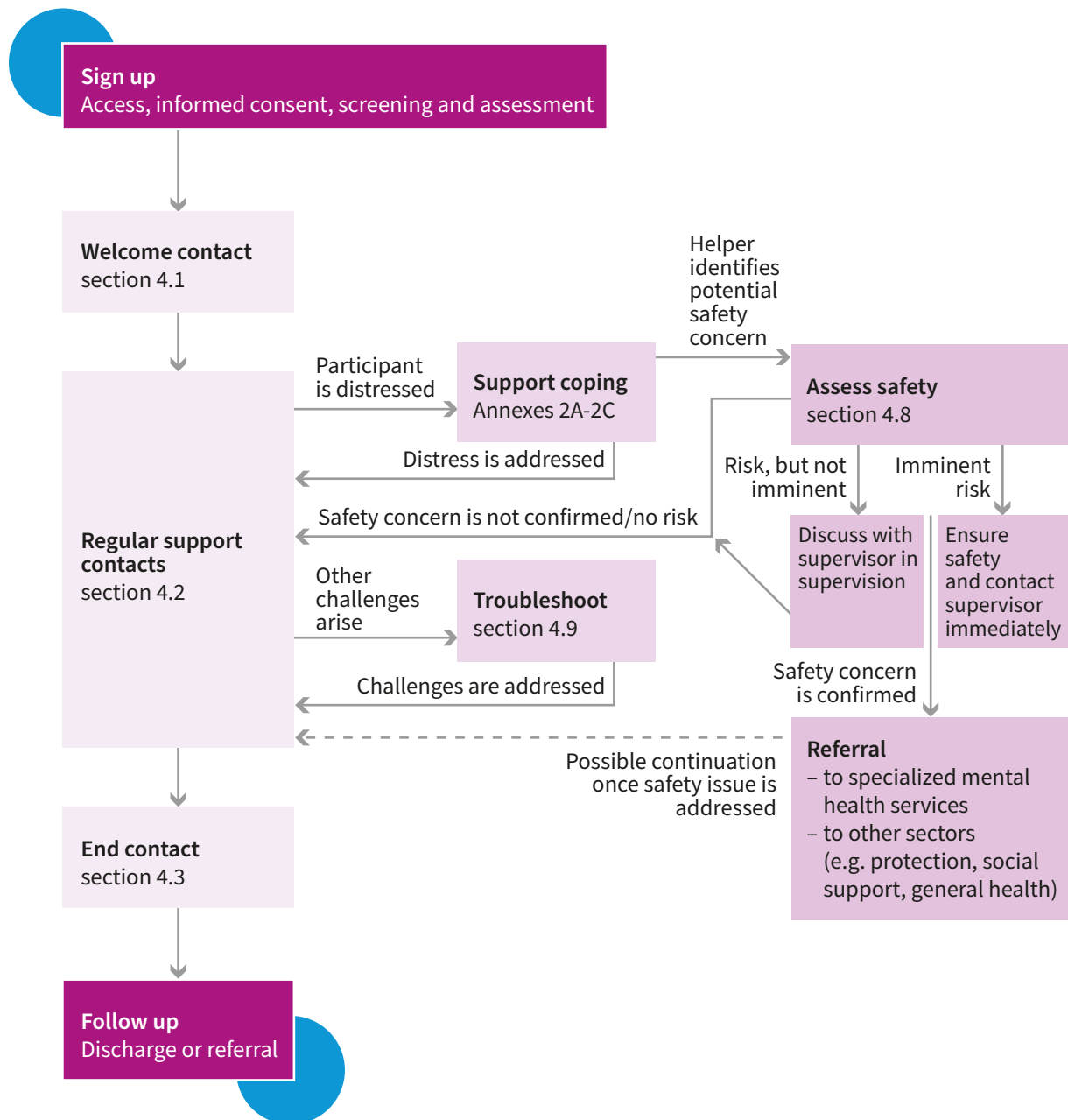
This section outlines the overall model for providing guided self-help ([Figure 4.1 Overall model for contacting and supporting service users](#)). Your organization may adapt this to your local context. It outlines the types of support contacts provided (which can be adapted for in-person delivery). If you are delivering Step-by-Step or Doing What Matters In Times of Stress, then these contacts are supplemented by the information in Chapters 5 and 6 respectively.

The aim of these contacts is to keep them to 15 minutes. Although this may seem brief for supporting someone in a psychological intervention, both research trials and real-world use of SbS and DWM have shown that effective and meaningful support can be delivered in this time. Keeping sessions short allows human resources to be used more efficiently, enabling more people to access help.

Once a person has signed up and been included in the intervention, they will be assigned to a helper. After this, you will make:

- **A welcome contact** to introduce the person to the intervention and to introduce yourself as their helper if they are receiving guidance (see section [4.1 Welcome contact](#)). You will provide the person with more details about the intervention and answer any questions or concerns they might have.
- **Regular support contacts** (typically once a week) over several weeks (for example, five calls for DWM and SbS corresponding to the five sections or chapters in each intervention). Each contact follows the same format (see section [4.2 Regular support contacts](#)). During these contacts, there are specific steps to follow if you identify a safety concern, if someone needs extra support with coping, or if other challenges arise (see sections [4.8 How do I manage imminent risk of suicide and other immediate safety concerns?](#) and [4.9 How do I deal with other challenges?](#)). If delivering Step-by-Step or Doing What Matters In Times of Stress, you will also refer to the specific information in chapters 5 and 6 during these calls.
- **End contact** when the person finishes the intervention to evaluate effects and outcomes and to identify and address any remaining support needs (see section [4.3 End contact](#)). This can be combined with the last regular support contact.

Figure 4.1 Overall model for contacting and supporting service users (see relevant sections for more detail)



4.1 Welcome contact

Your first contact with a service user is different to the others. It is a welcome call or message and happens after someone has signed up to the intervention. This contact is an opportunity to provide more information about the intervention, to introduce yourself and to establish how you will work together. This may help reduce drop-out later on. See [Annex 1A. Summary of tasks for welcome contact](#) for a summary of tasks for the welcome contact and [Annex 1B. Example script and text for welcome contact](#) for example scripts and text.

During the welcome contact, you will:

- welcome the person to the intervention and briefly introduce yourself;
- tell them about the intervention (including that it requires practice between modules);
- (for a guided intervention) explain what they can expect from you as a helper – tell them you are not a mental health specialist and explain what they can expect of you as their helper;
- (if there is an option of guidance, but they have not chosen it) explain what the guidance is like and that they can always switch to the guidance option;
- explain confidentiality and its limitations;
- give them the opportunity to ask any questions;
- (for a guided intervention) remind them of the day and time of your next contact; and
- say goodbye.

The aim of the welcome contact is to introduce the person to the intervention and yourself, to manage expectations and to help them feel more at ease. Remember to read through any information given to you (such as referral information) before you make the welcome contact.

4.2 Regular support contacts

After the welcome contact, the service user starts the intervention. At this point you will begin to have regular (for example weekly) support contacts with them (see [Annex 1C. Summary of tasks for regular support contact](#) for a summary of tasks for regular support contacts and [Annex 1D. Example script and text for regular support contact](#) for an example script and text). These typically last for as long as it takes for the person to complete the intervention, for example, five regular support contacts for SbS and DWM, which correspond to the five modules in the intervention (your organization should tell you the procedure for dealing with missed appointments and extending the time frame for providing support – see section [4.5 How to deal with unsuccessful contacts](#)).

The aim of regular support contacts is to offer support and to troubleshoot any problems people have with the intervention. Between the welcome contact and the first support contact, the person should have worked through the first module.

During your regular support contacts you will (see [Annex 1C. Summary of tasks for regular support contact](#) for a more detailed summary of tasks for regular support contacts):

- give an agenda for the support contact and remind them how long the call will last;
- ask the person how they are doing, and briefly discuss any issues they might mention;
- praise them for starting or finishing a module of the intervention;

- review the last module of the self-help intervention;*
- review the person's practice of the exercises;*
- discuss the benefits and purpose of the exercise;*
- ask them if they have any further questions or issues to raise; and
- remind them of the day and time for the next contact.

* See SbS and DWM specific information that can be added in Chapter 5 and Chapter 6.

It is important to follow the structure of the calls, but even more important to make sure that people feel validated, heard and listened to by using your basic helping skills. It can take some time to get used to doing this, especially if there is a person who is facing many problems. This will be covered in your training and can also be addressed in supervision. The aim is to complete most calls in 15 minutes, unless there are important issues to address such as a safety concern. Experience shows that with practice this can be achieved.

Extra support

During your regular support contacts, there may be times when you will be required to provide extra support. The service user may disclose information that makes you think they may be experiencing a mental disorder that might require specialized support (such as psychosis); a substance-related issue; an immediate safety concern, such as imminent risk of suicide; or an experience of sexual violence or intimate partner violence (see section [4.8 How do I manage imminent risk of suicide and other immediate safety concerns?](#)).

In all these situations it is important to contact your supervisor. They can advise you on the next steps. It may be that the person will benefit from extra support beyond the scope of the guided self-help intervention.

If there is no immediate health or safety concern but the person is distressed during your regular contacts or other challenges arise, you will need to support them to cope or troubleshoot in other ways (see section [4.9 How do I deal with other challenges?](#)).

4.3 End contact

After the service user has finished the intervention you will have an “end contact” with them. The end contact can be integrated into the last regular support contact, or it can be scheduled as a separate contact. During the end contact, you should congratulate the service user on completing the intervention and remind them that even though they have finished, they can continue to practise the skills they have learnt.

The main aim of the end contact is to evaluate the intervention's impact and see if people need more support. Repeating the baseline measures allows a comparison of symptoms before and after the intervention. It is also helpful to collect feedback on how people experienced the intervention by conducting a satisfaction questionnaire.

Questionnaires to evaluate the intervention's impact can be made available on digital platforms, if used. You only need to remind people to complete them online. If the questionnaire identifies a need for extra support consult your supervisor. Your organization will inform you if there are any more follow-up steps to take to discharge people from the service.

4.4 Ad hoc contact

Self-help interventions are not usually a crisis or on-demand service. Your organization should give you instructions on how to deal with contacts outside usual support contact times. You should inform service users of the procedure for this at the welcome contact but you may also need to remind them of it during the intervention if they contact you outside their appointment time.

If using messages instead of calls, it is important that the service user knows your process for responding to messages, for example, that you will not read messages until the appointment time and you may not check voicemails regularly. Your organization should have clear rules and guidelines that you can explain to people, including what they should do in an emergency (such as imminent risk of suicide).

4.5 How to deal with unsuccessful contacts

If the service user does not answer the telephone or reply to your message within the agreed time frame (usually one week), you can record an unsuccessful contact attempt.

The procedure for unsuccessful contacts or “missed appointments” may differ across organizations so always check your own organization's policy before proceeding (see [Box 4.1 Example policy for unsuccessful contacts](#) for an example policy based on the approach used in SbS trials in Lebanon). To manage caseloads, organizations may wish to reduce the threshold for unsuccessful contacts from three to a lower number. See [Annex 1E. Summary of tasks for end contact](#) for example text in case people stop responding.

Box 4.1 Example policy for unsuccessful contacts

If a service user cannot be reached for the appointment (for example, they do not answer their telephone or respond to a message by an agreed time point), the helper should:

- document the event as an unsuccessful contact in the case notes (see [Annex 5C. Example case note for missed contact](#));
- send the service user a message to acknowledge the missed appointment and explain next steps;
- contact them at the usual time next week (unless they get in touch to say they want to end the intervention).

After three unsuccessful contacts, the helper should inform the programme coordinator, who will discharge the service user from the helper's caseload.

If the helper is concerned about the service user's safety, they should always liaise with their supervisor before proceeding.

4.6 How to protect privacy during contacts

To protect privacy when contacting service users, only use the equipment, private space, platform, internet connection and log-in details that your organization gave you.

4.6.1 Protecting privacy in telephone or video contacts

Be aware that some people share phones, so it may not always be the service user who answers your call. It is important to ask about this in the welcome contact and to discuss what information you can share if someone else answers the phone. Your organization should have clear guidance on this issue (see Box 4.2 for an example protocol).

Never give out information about a service user or the intervention over the phone to anyone unless the service user has given you clear permission. See [Annex 2H. A friend or family member calls to ask about the intervention](#) for what to do when a service user's friend or family member is trying to get information from you.

Box 4.2 Example protocol for helpers for instances when someone other than the service user answers a contact call

- Say your name and that you are calling to speak to the service user.
- If the person on the phone asks who you are, do not give confidential information about the intervention or the service. Instead, say something generic – for example, you are calling from the outreach programme of your organization.
- If the person asks further questions, you could tell them that you are doing some health promotion outreach and that you need to speak to the service user directly.
- If the person does not let you speak to the service user, say you will call again another time, and explain this is due to the organization's confidentiality policy.
- Thank them and say goodbye.

Note. Protocols for dealing with other people answering a call will depend on your country and location, so check what your organization's policy is and agree what to do in advance with your supervisor.

4.6.2 Protecting privacy in written messages

When contacting people using written messages, use the messaging platform agreed by your organization. To ensure data protection and privacy, do not use your personal email or other messaging platform to contact people.

4.7 How do I write support messages?

If you are supporting service users through written messages, take care when drafting your text. For example, you will want to make sure your messages are clear and easy to digest.

If you are providing delayed (asynchronous) support, use short sentences and paragraphs for each topic you want to address.

If you are providing live (synchronous) support, remember that the person you are talking to will be waiting for a reply and reading long pieces of text can slow down the conversation. Break your text into smaller chunks so the person can start to read while you are typing the next section. Be sure to discuss in your welcome contact how you will indicate when you are still writing (for example, write “...” at the end of your sentence) so that the service user does not reply before you finish, as this can confuse the conversation and take it off course.

Make sure your feedback is neither too long nor too short. There are no specific guidelines on the right length, as it depends on the module being covered and the service user's responses. It is

important to stick to your allocated time where possible, and to ensure that the person can tell you paid attention to how they have engaged with the intervention that week. [Table 4.1 Helpful dos and don'ts for writing support messages](#) lists some other helpful dos and don'ts for writing support messages. Note that what is and is not appropriate will vary depending on local context and culture, so make sure you adapt the dos and don'ts accordingly.

Table 4.1 Helpful dos and don'ts for writing support messages

Do ...	Don't ...
• reread and spellcheck your message before you hit “send”; • be clear, concrete and specific – ensure what you write is not ambiguous and not likely to be misunderstood; • be positive and supportive and give compliments, but only if they are genuine; • read the message aloud. This can help you hear how the message may sound to the person receiving it. If in doubt, ask a colleague or supervisor for feedback; • write in a way that is professional, caring and easy to understand for the service user.	• use technical language or jargon; • use abbreviations; • USE ALL CAPS; • overuse exclamation or question marks (for example, !!! or ???); • use humour or sarcasm, as these can easily be misunderstood; • use emojis, as this can be misunderstood and might come across as unprofessional; • be overly positive. While it is great to give some positive feedback, adding things like “good job”, “well done” or “great work” after every sentence often comes across as patronizing or not genuine. Don't fake it, and don't overdo it.

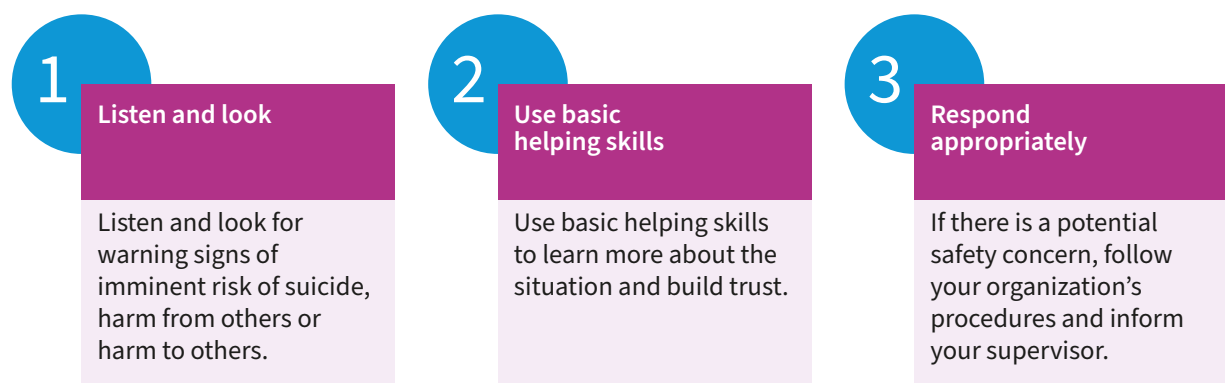
4.8 How do I manage imminent risk of suicide and other immediate safety concerns?

Immediate safety concerns include:

- imminent risk of suicide or self-harm;
- risk of harm to the service user or another person (for example, if the service user or someone else is experiencing physical or sexual violence, or if there is a child protection issue); and
- anything else your organization identifies as a potential safety concern.

This section presents a potential procedure for identifying and responding to safety concerns ([Figure 4.2 Overall procedure for dealing with safety concerns](#)). Please check with your organization, as they may have their own procedure that has been adapted for your context. For example, they may require you to get the service user's contact information and location in case emergency services need to be sent to support them. *In all cases you should follow your organization's procedures and guidelines for identifying and addressing safety concerns.*

Figure 4.2 Overall procedure for dealing with safety concerns



Note that the procedure that follows is written for live contacts with service users. If at any time you think there may be a potential safety concern through delayed messages, please contact your supervisor. You will probably be asked to speak to the service user over the telephone, in which case you would then follow the overall procedure outlined below.

4.8.1 Listen and look

The first step is to be alert during every contact and to listen and look for any indication of a safety concern, including imminent risk of suicide, harm from others or harm to others ([Table 4.2 Example warning signs that there may be an imminent safety concern](#)).

Table 4.2 Example warning signs that there may be an imminent safety concern

Indication of a safety concern	Example comments from the service user
Thoughts or plans of suicide, which may be either vague or explicit	• <i>"I just want to disappear."</i> • <i>"What's the point in living?"</i> • <i>"I'm just a burden to others."</i> • <i>"I have no reason to live."</i> • <i>"No one will miss me when I'm gone."</i> • <i>"I want to go to sleep and not wake up."</i> • <i>"I probably will not end my life, but if something was to happen to me, I would be okay with it."</i> • <i>"I just want to die."</i> • <i>"It would be better for everyone if I just killed myself."</i> • <i>"I have made a plan to die."</i> • <i>"I recently got some rope to kill myself."</i> • <i>"I have been looking for ways to kill myself."</i> (For example, any comment suggesting the person has sought access to pesticides or firearms, stockpiled medications, or researched suicide online) • <i>"I've been putting things in order in case something happens to me."</i> (For example, any comment suggesting the person has given away possessions or written a will)
Attempts at self-harm or suicide	• <i>"Earlier this year I tried to end my life."</i> • <i>"One time I took a lot of medication, hoping that I would not wake up."</i>
Severe emotional distress or hopelessness	• <i>"Things will never get better."</i> • <i>"Life is pointless."</i> • <i>"Nobody cares about me."</i>
Violent behaviour or extreme agitation	• <i>"I keep on getting into fights."</i> • <i>"I can't control my anger."</i> • <i>"My partner hit me."</i> • <i>"My partner humiliated me."</i>

Indication of a safety concern	Example comments from the service user
Withdrawal or unwillingness to communicate	• “I don’t want to talk to you or anyone.” • “No one can help me.”
Sudden and severe changes in mood or behaviour patterns	• “I’ve stopped eating.” • “I’ve not been sleeping.”
Reports of risk of harm from others or to others	• “I had to go to the hospital because they beat me so badly.” • “I’m scared for my life.” • “I can’t control my anger and am scared I’ll hurt them.” • “My husband/wife is beating our children.”

The list of warning signs in Table 4.2 is not exhaustive, and there may also be other, culturally specific warning signs. It is important that helpers and organizations delivering guided self-help are supported by relevant professionals (such as mental health professionals) who can oversee and give guidance on safety procedures.

4.8.2 Use basic helping skills

Step 2 in the procedure for dealing with safety concerns is to use your basic helping skills to remind the service user about confidentiality, learn more about their situation and build trust.

Listen empathetically and non-judgementally, and validate the person’s experience. For example:

“I’m glad you’re sharing these things with me. Before we speak more, I want you to understand that everything you tell me will be kept private, unless you or someone else is in immediate danger. That means that I will not talk with your family or other people outside the [add name] team about what you have said, without your permission. But if I am very concerned that you or someone else is in immediate danger, my supervisor and I may need to speak with people outside the [add name] team to get you the best kind of help and ensure the safety of the people involved. This is because my role is to care about your welfare and safety.”

Adapt this example text according to the procedures of your organization and any national guidelines.

To learn more about the person's situation and build trust, you could ask questions such as:

- *"Can you tell me a bit more about the situation?"*
- *"What has happened to make you feel this way?"*
- *"How is this situation affecting you?"*
- *"What do you mean by that?"*

Depending on the situation, you may need to assess imminent risk of suicide. Your organization should have a protocol for you to follow (see [Annex 3A. Example protocol for assessing imminent risk of suicide](#) for an example of what that might look like). If you have to ask about suicide and self-harm, make your questions clear and direct ([Box 4.3 How to ask about suicide and self-harm](#)).

Box 4.3 How to ask about suicide and self-harm

- When asking about suicide and self-harm, ask direct, clear questions. Avoid using vague or evasive words that could be misunderstood.
- Direct questions help the person feel that they are not being judged for having thoughts or plans of suicide or for having made suicide attempts or self-harmed in the past.
- Some people may feel uncomfortable talking with you about suicide, but you can tell them that it is very important for you to clearly understand their level of safety.
- Asking questions about suicide will not cause the person to take action to end their life, but rather often helps them to feel understood and less distressed.
- See [Annex 3A. Example protocol for assessing imminent risk of suicide](#) for more information on asking about suicide.

If you are providing support remotely, your organization should have clear processes for you to follow if, for example, someone hangs up on you or you lose the connection when you are talking with them.

Here are some things you might do to address the challenges of working remotely ([47](#)).

- If possible, try to find out where the person is located for the support contacts, so you can identify nearby places if the person you are speaking with needs emergency assistance or referral for additional support.
- Communicate the risk protocol clearly in the first contact.
- Create a safety plan in the first remote support contact.
- Establish a backup plan at the beginning of the intervention for what to do if you get disconnected.

- If you lose contact, try reconnecting with the person. Remind them that it is important for you to ensure their safety and check whether they wish to continue with the intervention. If they do not answer and you are concerned for their safety, you may need to speak with the safety contact agreed at the start of the programme (if your organization's safety protocol includes safety contacts).
- Tell the person about different support services available, so they have options.
- Contact your supervisor to let them know you lost connection with someone with a safety concern.

4.8.3 Respond appropriately

The last step in dealing with safety concerns is to respond appropriately. This means following your organization's procedures, so make sure you know what these are.

If there are indications of imminent risk of suicide or any other immediate safety concern you must inform your supervisor immediately ([Box 4.4 How to respond to someone at imminent risk of suicide](#)). This is essential to ensure that you get the support you need to deal with the situation and that appropriate actions are taken to support the person at risk.

Box 4.4 How to respond to someone at imminent risk of suicide

- *Contact your supervisor immediately.*
- Do not leave the person alone. Stay with them, or on the phone with them, while waiting for your supervisor or trusted person to arrive.
- Ask the person if you can contact someone they trust in the community to provide support and help to ensure that they are kept safe.
- Ask the person if they have access to any means of self-harm. Discuss ways to remove these items from their home environment.
- Attend to the person's mental state and emotional distress.
- Explore reasons and ways to stay alive.
- Focus on the person's strengths by encouraging them to talk about how earlier problems have been resolved.
- Be aware of your own response to this topic and try to follow the protocols.

If there is no indication of a safety concern, continue to support the person through your regular support contacts (see section [4.2 Regular support contacts](#)).

Your organization should provide information about how to report an imminent risk of suicide in contexts where it is illegal. For more information on ensuring confidentiality and understanding mandatory reporting see the Inter-Agency Standing Committee (IASC) Mental Health and Psychosocial Support Minimum Service Package (48).

Remember, one of your roles as a helper is to ensure that service users can access appropriate support and that they are kept safe.

4.9 How do I deal with other challenges?

There are many reasons why a support contact may become challenging. For example, the service user may:

- experience distress during a contact;
- talk about an overwhelming number of other problems in their life;
- need extensive support coping with external problems;
- continue talking after 15 minutes or send very long messages;
- report problems with completing the intervention's exercises or home practice;
- lack motivation;
- not respect boundaries;
- have a friend or family member who calls to ask about the intervention; or
- wishes to stop doing the intervention.

See [Annex 2. Troubleshooting templates](#) for a short troubleshooting guide to each of these challenges. This guide is not an exhaustive list but you can always consult your supervisor when you are experiencing challenges.

Tips and tools for specific interventions

5. Information for helpers delivering Step-by-Step

This chapter is for organizations delivering Step-by-Step (SbS). The content and illustrations for SbS can be found in the [Web Annex: Step-By-Step: A self-help intervention for depression](#).

This chapter contains specific information for helpers delivering SbS. It should be used alongside Chapter 4, which covers information on the support contacts and important information when providing the intervention.

Note for programme managers. Please adapt the information in this chapter to your context before sharing it with helpers.

5.1 Overview

SbS is a brief online self-help intervention for adults with depression (49). It was developed by WHO in collaboration with other organizations.⁷ Comprising WHO model, it has gone through extensive peer review, was tested in two RCTs in Lebanon and has been running as a national service in Lebanon since 2022 (see [Box 1.1 Step-by-Step in Lebanon](#)).

SbS:

- is proven to improve symptoms of depression, anxiety, functioning, post-traumatic stress, subjective well-being and self-identified personal problems;
- can be offered alongside other mental health interventions, or community programming;
- can be delivered remotely and at scale (to many people at the same time);
- provides psychoeducation and training in behavioural activation alongside other therapeutic techniques such as slow breathing, identifying strengths, positive self-talk, increasing social support and relapse prevention;
- is delivered using a fictional story narrated by people seeking help for depression, with a trusted character (such as a health worker) giving instructions on therapeutic techniques; and
- encourages people to practise techniques on their own, in between modules.

⁷ WHO collaborators were the National Mental Health Programme at the Ministry of Public Health in Lebanon, Freie Universität Berlin and University of Zurich.

SbS content can be added to any online platform (for example, an e-learning platform) and can be adapted for use in other media formats such as books or videos. SbS can also be delivered as an unguided intervention; this might make the intervention less effective, but it will be able to reach more people.

If delivered through a digital platform, people receiving SbS will need a compatible device (smartphone, tablet or computer) and a reliable internet connection to complete it. They may also need headphones to listen to the intervention's audio exercises. Using SbS on a platform that can be used offline (such as an app) is probably preferable in some settings. In all cases, implementation requires sufficient IT knowledge and resources to host and maintain the platform and ensure good security.

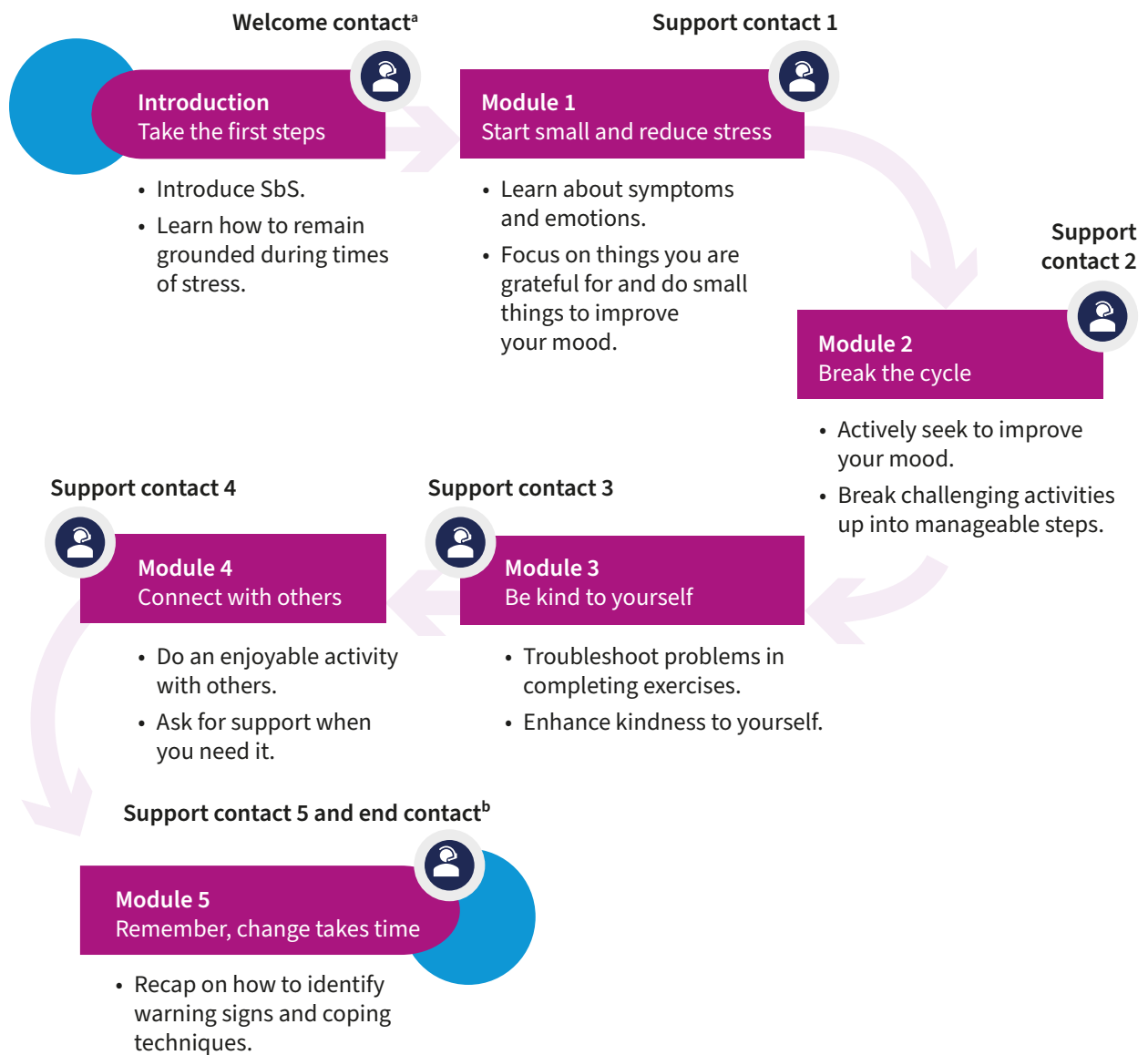
When offered with guidance, telephone, internet-based calls, video support or instant messaging will be required.

5.2 Step-by-Step skills, modules and support contacts

SbS seeks to boost a person's activity to create pleasant emotions, a sense of achievement and connection with others to improve mood (called behavioural activation). It comprises five weekly modules and an introductory module, each focused on a specific skill ([Figure 5.1 Overview of SbS modules](#) and [Table 5.1 Overview of SbS skills and exercises](#)). Each module is divided into parts with different sections and takes around 30 minutes to complete.

All modules include exercises that people are asked to practise in their own time. In the version of SbS that was evaluated in research there was one week between modules. In that time the person was expected to complete all the content and also have time to practise. There is some flexibility to this; for example, if someone does not complete a module one week or needs longer, then support contacts can be rearranged. In the research, one welcome call and five support contacts were delivered over a maximum eight-week period to account for any delays.

Ideally, service users will practise and use all skills, but this may not happen. Instead, people may be naturally inclined to use some skills more than others. Helpers should reinforce the importance of trying all the skills, particularly stress management and taking steps to improve mood by doing activities, including social activities.

Figure 5.1 Overview of SbS modules

^a The welcome contact is also provided when SbS is delivered unguided and for people requesting messaging support (see section 4.1 for more information). Implementation experience shows that having a welcome contact may help increase engagement and use and increases understanding of the intervention.

^b The end contact can be combined with support contact 5 or delivered separately.

Table 5.1 Overview of Sbs skills and exercises

Skill	Summary	How to practise	Benefits of practice
Grounding (Introduction)	Grounding brings us back to the present moment and unhooks us when we're caught up in emotional storms.	1. Notice how you are feeling and what you are thinking. 2. Slow down and connect with your body by stretching, pushing your feet down, and focusing on your breathing. 3. Refocus on the world around you and what you are doing.	• When we are caught up in emotional storms (causing stress) and hooked by difficult thoughts and feelings, it is hard to do anything useful. Grounding can help us to refocus on what we need to do in a stressful situation. • Practising regularly, even when we are not very stressed, can help us to then use grounding in more difficult situations.
Slow breathing (Module 1)	Like grounding, slow breathing helps us to reduce stress.	1. Relax your body. Shake out your arms and legs and let them go floppy and loose. 2. Breathe in through your nose and out through your mouth. 3. Slow down your breathing. Try to take around three seconds to breathe in and three seconds to breathe out.	• Breathing can alter how stressed people feel. • We can use this technique whenever we feel overwhelmed or stressed. • Practise regularly because we know from experience that many people find it helpful. • Users may naturally prefer and use either grounding or slow breathing exercise.

Skill	Summary	How to practise	Benefits of practice
Gratitude list (Module 1)	Making a gratitude list can help to improve mood.	1. Think of things you are grateful for. These can be anything, big or small. 2. Write down three things you are grateful for. 3. Look at this list whenever you need to and be reminded of the things you are grateful for in your life.	• Focusing on things we are grateful for helps improve how we are feeling. • Regular practice is important.
Small activities (Modules 1 and 2)	The small activities exercise shows us that what we do can have an impact on how we feel.	1. Write down three small things you would normally enjoy doing. 2. Try to do one thing every day that you enjoy or used to enjoy. 3. As you do this, take a moment to notice your mood. After doing the activity, do you feel even the smallest bit better?	• When we are very sad or stressed, doing even the smallest activity might seem hard. Regularly doing something that makes us feel good, even if it is something small, can help reduce stress and improve mood. • With practice, doing small, pleasant activities can lead to bigger improvements in mood.

Skill	Summary	How to practise	Benefits of practice
Breaking activities down (Module 2)	This exercise helps us break down more challenging activities into smaller more manageable steps.	1. Think of an activity you would like to do that seems overwhelming. 2. Divide and schedule the activity into small and manageable steps. 3. Write down possible difficulties you might find when trying to do the scheduled activity, and how to overcome these. 4. Start trying to do the activity.	• It is common to find it hard to do activities when we do not feel well. • Anticipating when things will be difficult is also an important part of managing our stress.
Mood tracking (Module 2)	Being aware of emotions and tracking them helps us notice patterns in how we feel, making it easier to understand what affects our mood.	1. Notice how you are feeling. 2. Make a note of this using the Mood Tracker in the programme or in another way. 3. Add anything that is making you feel this way.	• It can be hard to understand why we feel a certain way. • Noticing and recording our mood or emotions can help us understand patterns.

Skill	Summary	How to practise	Benefits of practice
Be kind to yourself (Module 3)	Being kind to ourselves means using kind words and actions with oneself.	1. Unhook from unkind thoughts and feelings by noticing and naming them. 2. Speak to yourself kindly. You could also imagine filling one of your hands with kindness and placing it somewhere on your body where you feel pain. 3. Speak to others kindly and engage in kind actions.	• Being kind to ourselves gives us more energy to cope with stressful situations. • Being an encouraging friend to oneself is important in moments of doubt and distress. • It can also help us stay on the right track and keep up our positive activities.
Social activities (Module 4)	Social activities help us to build social support and feel better.	1. Think about social activities you might like to try, even if they seem difficult right now. 2. Try a social activity and notice how it affects your mood.	• It is important to spend time with others, even if it feels hard. • This is another way of breaking the “sadness and withdrawal cycle”. • In general, seeing others helps us feel better.
Warning signs (Module 5)	Knowing our warning signs, and how to cope with them, helps us to feel good again.	1. Think about signs that show you are not coping well any more (these are your warning signs). 2. Write down a list of your warning signs. 3. Think of ways you can cope with your warning signs when they come up.	• It is common to feel worse again every once in a while. • Knowing our warning signs helps to do something to make ourselves feel better.

The ideal frequency for practising the different skills in SbS will differ for each service user, depending on the severity of their symptoms. The aim of the behavioural activation skills (small activities, breaking activities down and social activities) is to re-engage people in daily life. The goal is to gradually increase activity levels as the course progresses. For someone with severe depression, even doing one new activity a week, or a very small activity every day (such as a short walk), may be the most they can achieve. But for people with milder problems, it will be possible to encourage daily small activities or bigger activities a few times a week. It is important for people to pay attention to how they feel after each activity and to recognize any improvements in mood, even if they are brief.

The other skills are more aimed at reducing stress and should be practised every day after they are introduced. Some, like the grounding and breathing exercises, need repeating for people to notice their benefit. Individuals may prefer to use only grounding or breathing. Once people feel they have mastered the skill, they can use it any time they need to reduce stress.

The rest of this chapter summarizes the content of each module and gives tips for helpers to use in support calls. This should be used alongside the scripts and example texts in [Annex 1. Summary of tasks for different contacts, and example scripts and texts](#).

Whether or not you use the example scripts or text, always remember to personalize content for each service user. It is important to look at the person's progress, address any questions they may have, and tailor your message accordingly. It is also helpful to look at the content and guidance section for the module the person has just completed before you write your reply (see [Annex 1C. Summary of tasks for regular support contact](#) for more information).

5.2.1 Overview of support contacts for SbS

The following approach can be used when delivering SbS with support contacts (see [Figure 5.1 Overview of SbS modules](#)). Scripts are available in the annexes for live contacts and example text for messages for delayed contacts.

- 1. Welcome contact.** See section [4.1 Welcome contact](#) for general information on the welcome contact. In SbS, the welcome contact can be combined with additional information to support the module “Introduction: take the first steps” (see section [5.2.2 Introduction: take the first steps](#)). See [Annex 1A. Summary of tasks for welcome contact](#) for a summary of tasks for the welcome contact and [Annex 1B. Example script and text for welcome contact](#) for an example script and text.
- 2. Support contacts.** These are delivered weekly and are tailored to the specific SbS modules. Aim for 15 minutes per support contact.
 - For general information, see Chapter [4. How to provide support to service users](#).
 - See [Annex 1C. Summary of tasks for regular support contact](#) for a summary of tasks for regular support contacts and [Annex 1D. Example script and text for regular support contact](#) for an example script or message text.

- Before each contact, review the additional information about the specific module covered in the contact and adapt sections 3 and 4 of the example script or text ([Annex 1D. Example script and text for regular support contact](#)) (see sections below).
- 3. End contact.** This contact can be integrated in the last support contact (module 5: “Remember, change takes time”). See section [4.3 End contact](#) for general information, [Annex 1E. Summary of tasks for end contact](#) for a summary of tasks for the end contact and [Annex 1F. Example script and text messages for end contact](#) for an example script and text.

5.2.2 Introduction: take the first steps

The introductory module comprises three components.

- **Welcome to SbS.** SbS is introduced alongside an overview of what service users will learn, and how people can benefit best from the programme. The helpers are introduced, as are other parts of the intervention, such as the toolbox.
- **Introduction of characters.** The doctor and other characters in the story are introduced.
- **Grounding.** This core skill, which is practised throughout the intervention, is introduced. Service users practise three different grounding exercises: introductory, short and longer grounding (see [Annex 6. Script for grounding exercise](#) for a grounding exercise script for live contacts).

Tips for helpers (see also [Table 5.1 Overview of SbS skills and exercises](#))

People might not yet have gone through the introductory module when you contact them.

Prepare for the call by reviewing section 4.1 for information on what to cover in this contact and [Annex 1A. Summary of tasks for welcome contact](#) and [Annex 1B. Example script and text for welcome contact](#) for a summary and example script and text.

- If people have general questions about SbS, tell them about guided self-help, the exercises they will do, and how to practise them at home.
- If someone asks about helper support, refer to the information you gave them during the welcome call and explain it again.
- If someone asks about the grounding exercise, refer to the information provided in the introduction module, and explain that it is important to practise exercises to feel their effect.
- If someone complains of light-headedness during the grounding exercises, it may be because they are breathing too deeply or too fast. You might say: *“This can be a common problem. Most of the time if you feel dizzy or light-headed, it means you are breathing too fast or too deeply. Try breathing more slowly and gently.”*

5.2.3 Module 1. Start small and reduce stress

Module 1 has four content components.

- 1. Psychoeducation about stress.** The service user characters start sharing their story, talking about the symptoms they have experienced. The doctor character explains how experiencing adversity can lead to low mood or anxiety and physical symptoms.
- 2. Slow breathing exercise.** Service users learn the skills of slow breathing to help reduce stress (see [Annex 7. Script for slow breathing exercise](#) for a script for slow breathing that can be used in live contacts).
- 3. Gratitude list.** Service users are asked to think about and focus on things to be grateful for.
- 4. Small activities.** Service users learn that what we do can have an impact on how we feel and that starting to do small activities again can help us feel better.

Tips for helpers (see also [Table 5.1 Overview of SbS skills and exercises](#))

Psychoeducation stress

- People will probably ask about stress or might describe their symptoms to you. Use active listening and encouragement to explain that completing SbS may help them feel better.
- If someone uses medical terms such as “diagnosis”, explain that people have different names for their experience (for example, distress, low mood, depression). Ask the person what term they would like to use.

This information can be used for section 4, on reviewing practice of techniques and exercises, of the support contact scripts (see [Annex 1C. Summary of tasks for regular support contact](#) and [Annex 1D. Example script and text for regular support contact](#)).

- If someone asks about grounding or slow breathing, encourage them to practise these exercises regularly because we know from experience that many people find them helpful.
- If someone complains of light-headedness during breathing exercises, it may be because they are breathing too deeply or too fast. You might say: *“This can be a common problem. Most of the time if you feel dizzy or light-headed, it means you are breathing too fast or too deeply. Try breathing more slowly and gently.”*
- If someone asks how the gratitude list will help, refer to the explanation in the module, which says that focusing on things we are grateful for helps improve how we are feeling.
- Remind people that, like all exercises, regular practice is important.

Small activities tips

- If someone asks how the small positive activities will help, you can briefly explain that our activities can affect how we feel, and that they will learn more about this in the next module.

- Praise people's efforts (no matter how small) and reiterate that it is important to practise doing enjoyable activities (people may not be used to doing this).
- Starting small is a key point here, because if people try something too big and do not manage it this early in the programme, they are at particular risk of dropping out.
- You may also spend a few minutes brainstorming ideas of enjoyable activities they could do. Try to discourage unhealthy behaviours, such as drinking alcohol and smoking.
- Help people recognize the positive changes in their mood after doing an activity. Changes may be very small, so you may need to ask questions to prompt this recognition.
- If someone has found it difficult to think about or do activities over the past week, support them to identify small activities they can schedule for next week.

5.2.4 Module 2. Break the cycle

Module 2 has four content components.

1. **Story content.** Story characters reflect on their experience with the small activities exercise, saying how they stopped doing pleasant activities because of low mood, and how this made them feel even worse. The doctor character explains it is usual to struggle with activities.
2. **Sadness and withdrawal cycle.** In the story, the doctor character explains the sadness and withdrawal cycle, which is the main therapeutic concept used in SbS. This is a vicious cycle in which a person with low mood has little energy to do anything; they withdraw and so get even more low. This vicious cycle can be broken by starting to do enjoyable activities again (behavioural activation).
3. **Mood Tracker.** Service users get introduced to the Mood Tracker, a tool to track their mood and help identify what makes them feel better or worse.
4. **Breaking activities down.** Service users learn a technique for dealing with overwhelming activities by breaking them into smaller steps so that they become more manageable.

Tips for helpers (see also [Table 5.1 Overview of SbS skills and exercises](#))

Tips on sadness and withdrawal cycle

- If someone asks about the sadness and withdrawal cycle, consider explaining it once more, emphasizing how it keeps you feeling low or sad with no energy. Remind them that wanting to withdraw is common.
- Ask about their experiences with their activities, comparing how they feel after doing the activities with how they feel not doing them, that is, how they felt before starting SbS.
- If someone did not manage to do their activities, help them identify possible causes and encourage them to try again, as it can take some time to break the cycle of feeling down.

- If someone mentions a lot of negative thoughts about themselves, acknowledge this and say we often criticize ourselves when we feel down.
- If they say they are struggling, show empathy, tell them that it is okay to struggle – many people do – and remind them that it is important to keep trying to practise the exercises. If someone says the story is overly negative, praise them for not struggling that much.

This information can be used for section 4, on reviewing practice of techniques and exercises, of the support contact scripts (see [Annex 1C. Summary of tasks for regular support contact](#) and [Annex 1D. Example script and text for regular support contact](#)).

Mood Tracker

- If someone needs more information about the use of the Mood Tracker explain:
“the Mood Tracker allows you to enter how you feel whenever you want to. This is helpful because tracking your mood could help you spot the things that are making you feel good or bad, and in turn do more or less of them to feel better. Tracking your mood can make it easier for you to help yourself get out of the sadness and withdrawal cycle and become more aware of your emotional state, so I really encourage you to use it.”

Breaking activities down

- If someone says they feel worried about trying a bigger activity (now or during later modules), acknowledge their feelings, perhaps reflecting on how the sadness and withdrawal cycle can make us not believe in ourselves and feel very tired. Give hope and praise for taking on a larger activity.
- If someone is struggling with all the different exercises, give them any needed technical support and say that it is important to start small. Remind them that if they feel worried, it is okay to go back to the small activities exercise, or to only plan one or two steps of a challenging activity.

5.2.5 Module 3. Be kind to yourself

Module 3 has three content components.

- 1. Story content.** In the story, characters tell how they have struggled and did not feel SbS was helping them at first. The doctor explains that this often happens, and that it is important to keep practising. Then the characters describe how they struggled with meeting their friends and how the doctor helped them with a grounding exercise and a positive self-talk exercise.
- 2. Positive self-talk.** Service users learn how being kind to ourselves gives us more energy to cope with stressful situations.
- 3. Breaking activities down: problems and solutions.** Service users write down potential difficulties they might face when trying to implement their scheduled activities, and ideas for overcoming them.

Tips for helpers (see also [Table 5.1 Overview of SbS skills and exercises](#))

This information can be used for section 4, on reviewing practice of techniques and exercises, of the support contact scripts (see [Annex 1C. Summary of tasks for regular support contact](#) and [Annex 1D. Example script and text for regular support contact](#)).

Positive self-talk

- If someone asks about negative self-talk, you may have to explain that how we think about ourselves can affect our mood and vice versa.
- Ask the person how the positive self-talk went. If they have very low self-esteem they may struggle with this, and you may need to help them come up with ideas for kind things they can say to themselves.

Breaking activities down: problems and solutions

- By this time in the programme, people may be struggling with finding new things to do, so you may need to brainstorm more ideas with them by asking them a few questions about their community and what activities are possible there. You could also give examples of what other people might have tried.
- People may find it easy to list potential difficulties but harder to identify ways of overcoming them. Check people's responses to this exercise to make sure they have identified possible solutions and not only listed problems. If they have not identified solutions, tell them that you would like to help them.

5.2.6 Module 4. Connect with others

Module 4 has three components.

- 1. Story content.** In the story, the characters tell how they met their friends and how they felt hesitant at first but then felt relaxed and comfortable. They also share how they asked friends for support. The doctor character explains that seeing others generally helps us to feel better and that it is important to ask for help from others or organizations when you need it.
- 2. Social activities.** Service users plan a social activity.
- 3. Asking for support.** Service users are encouraged to ask for support.

Tips for helpers (see also [Table 5.1 Overview of SbS skills and exercises](#))

This information can be used for section 4, on reviewing practice of techniques and exercises, of the support contact scripts (see [Annex 1C. Summary of tasks for regular support contact](#) and [Annex 1D. Example script and text for regular support contact](#)).

Social activities

- If someone says they have not really stopped seeing people, explore whether they are enjoying the time they spend with others and whether it is quality time. If not, explain that planning an activity with someone you trust may still lift your mood.
- If someone didn't manage a social activity, help them identify possible causes and help them to plan one, even if it is something small.
- If someone did manage a social activity, ask them about how it made them feel (and focus on reflecting positive elements of their response). Was it fun or a challenge? What social activity might they do next time?
- If someone says they found it difficult, help them identify support and coping strategies, and explain that these can be added to their social activities exercise.
- If someone says they don't have anyone they can contact, try exploring whether they can think of someone they can reconnect with again, or someone they have just met they could get in contact with more.

Asking for help

- If someone asks you about local organizations that could help them, find out what area they live in and what they need help with. Remember to show empathy when someone is sharing their problems. Refer them to the list of services you were given by your organization.

5.2.7 Module 5. Remember, change takes time

Module 5 has two main components.

1. **Story content.** In the story, characters talk about how they are doing after SbS. They say how they felt better and how a difficult life event made them feel worse again. They say that they overcame the situation using the techniques they had learnt in SbS. The doctor character explains that it is common to have ups and downs, and that it is good to know our own warning signs.
2. **Warning signs and coping mechanisms.** Service users learn how to identify when their mood is getting worse (warning signs) and what to do if that happens (coping techniques) as a form of relapse prevention.

Tips for helpers (see also [Table 5.1 Overview of SbS skills and exercises](#))

- The support contact for module 5 of SbS has an “end contact” incorporated (see [Annex 1E. Summary of tasks for end contact](#) for a summary of tasks for the end contact and [Annex 1F. Example script and text messages for end contact](#) for an example script and text).
- If people feel sad or worried that this is the last module, encourage them to keep practising the skills they have learnt.

This information can be used for section 4, on reviewing practice of techniques and exercises, of the support contact scripts (see [Annex 1C. Summary of tasks for regular support contact](#) and [Annex 1D. Example script and text for regular support contact](#)).

- Ask the person if they know what their warning signs are. If they do, encourage them to continue using the SbS techniques, especially if they notice the warning signs returning.
- If the person has not identified their warning signs, refer them back to module 5 and ask them to complete the warning signs exercise, or complete this exercise with them.
- It is really important to make a plan for the future so you can continue to maintain all the great work you have done so far. You can look out for any warning signs and put in place strategies that you know can help you.

6. Information for helpers delivering Doing What Matters in Times of Stress (DWM)

This chapter is for organizations delivering Doing What Matters in Times of Stress (DWM). DWM is available in book form [here](#). It can be adapted to run on a digital platform (such as <https://dwmatters.eu>).

This chapter contains specific information for helpers delivering DWM. It should be used alongside Chapter 4, which covers the information on the different types of contact and general information important when providing support.

Note for programme managers. Please adapt the information in this chapter to your context before sharing it with helpers.

6.1 Overview

DWM is a brief self-help intervention that is one part of WHO's group stress management intervention, Self-Help Plus (9, 31). DWM can also be delivered as an individual self-help intervention. It has been used in research trials as a web application (50) and found to be effective using five support contacts. In addition, the book version of DWM has been found to be effective in a pilot RCT (16) using three phone support contacts, and in a study in Iraq using three face-to-face support contacts (24). Results from unpublished field tests with over 500 people in Syrian Arab Republic and Türkiye showed significant reductions in distress for people using DWM.

DWM is:

- based on acceptance and commitment therapy and adapted for many settings;
- intended as an intervention for managing stress and coping with adversity;
- evaluated for use with people aged over 18 years;
- designed to be completed over a short period of time;
- practical, so that people can practise skills regularly on their own; and
- illustrated, with accompanying audio recordings to support regular practice.

DWM content is publicly available as an illustrated book in a number of languages (9), and in a more limited number of languages as a web app (<https://dwmatters.eu>). WHO has a translation and adaptation package available on request that contains all DWM content (text and illustrations) and supports new translations or implementations.⁸

DWM can be provided in different ways, including by lay providers with weekly support contacts and by primary health care providers with follow-up as part of routine appointments. It can also be provided as unguided self-help, by making the book or web app accessible to people.

In the book version of DWM, each chapter introduces a skill, summarizes key tools and activities, and provides guidance and audio exercises to help people practise them.

What follows is a description of the approach used with the web application (<https://dwmatters.eu>), whereby a weekly support contact corresponding to each module was provided. This online version of DWM uses the same content as the book, including the same audio exercises and tool summaries. It can be accessed through a common web browser (such as Firefox, Google Chrome or Safari).

People who receive a web version of DWM need a compatible device (smartphone, tablet or computer) and a reliable internet connection (the intervention is not available offline). They may also need headphones to listen to the audio exercises.

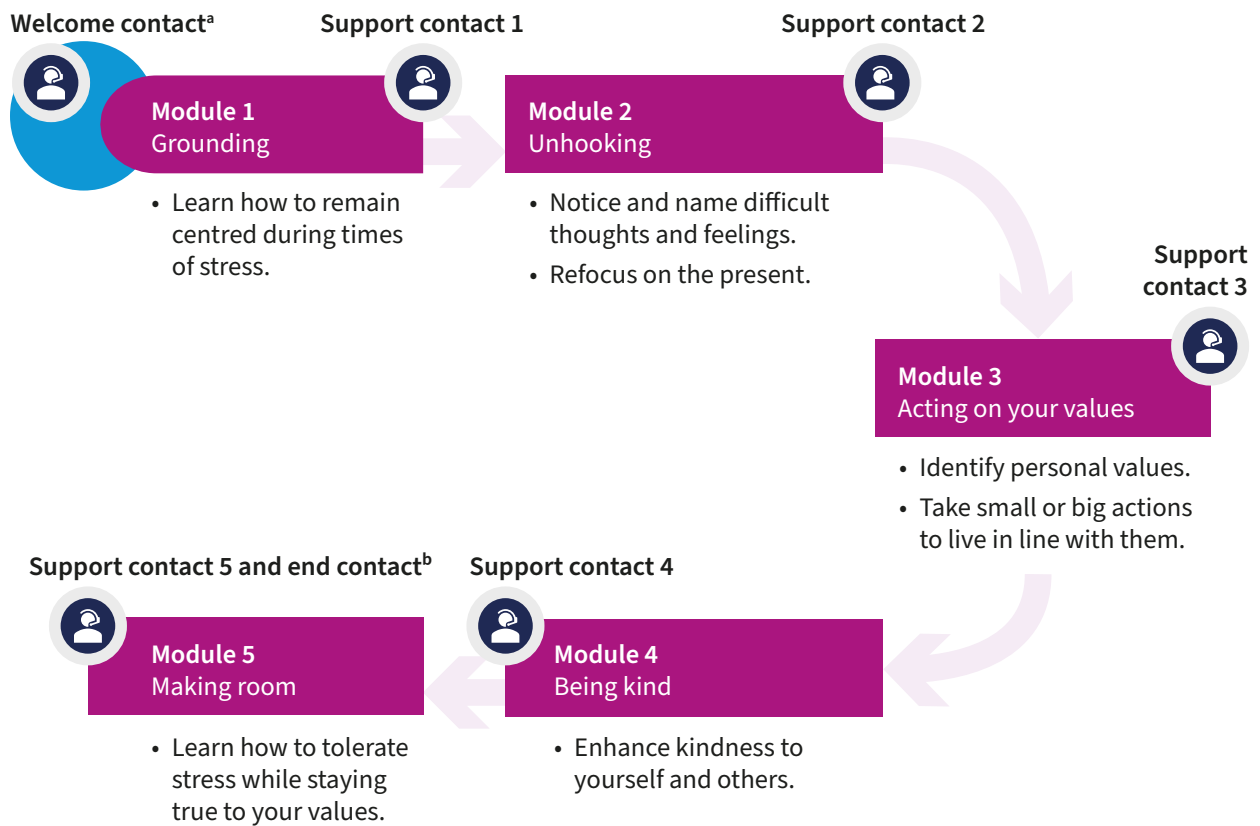
This model of support contacts can be used with the book version and can also be adapted to fewer contacts, which may be more suitable for delivery in primary care. An adapted shorter model, which has been tested in research studies, includes three support contacts: a first welcome contact, a follow-up contact a week or two later to check progress and provide motivation and support, and a final contact a few weeks later.

6.2 DWM skills, modules and support contacts

DWM aims to teach people skills to manage stress. It comprises five weekly modules, each focused on a specific skill ([Figure 6.1 Overview of DWM modules](#) and [Table 6.1 Overview of DWM skills and exercises](#)). In the online version, each module (which corresponds to a chapter of the book) is divided into three submodules and includes guided exercises to help service users learn key stress management skills.

⁸ Contact psych_interventions@who.int for more information.

Figure 6.1 Overview of DWM modules



^a The welcome contact is also provided when DWM is delivered unguided and for people requesting messaging support (see section 4.1 for more information). Implementation experience shows that having a welcome contact may help increase engagement and use and increases understanding of the intervention.

^b The end contact can be combined with support contact 5 or delivered separately.

Table 6.1 Overview of DWM skills and exercises

Skill	Summary	How to practise	Benefits of practice
Awareness exercise (Module 1)	The awareness exercise is sometimes called “paying attention with curiosity”, which means paying full attention to whatever is happening right in front of us, without judging it as good or bad.	1. Use your five senses to focus your full attention on an activity you are doing (such as eating, walking, cooking, cleaning, or talking or listening to someone). 2. Notice with curiosity what you can see, hear, touch, smell and taste.	• Paying attention with curiosity often helps us to focus on what we are doing and to do it better. • It can also help us to enjoy or appreciate what we are doing.
Grounding (Module 1)	Grounding brings us back to the present moment and unhooks us when we’re caught up in emotional storms (stress).	Steps for grounding: 1. Notice your thoughts and feelings. 2. Slow down and connect with your body by stretching, pushing your feet down and focusing on your breathing. 3. Refocus on the world around you and on what you are doing.	• When we are caught up in emotional storms (causing stress) and hooked by difficult thoughts and feelings, it is hard to do anything useful. Grounding can help us to refocus on what we need to do in a stressful situation. • Practising regularly, even when we are not very stressed, can help us to then use grounding in more difficult situations.

Skill	Summary	How to practise	Benefits of practice
Unhooking (Module 2)	Like grounding, unhooking helps us to unhook from difficult thoughts and feelings. It is faster than grounding and can be done in just a few seconds.	1. Notice a thought or feeling with curiosity. 2. Name the thought or feeling. 3. Refocus on the world around you and on what you are doing.	• Unhooking from difficult thoughts and feelings allows us to see them for what they are and reduces their control over our actions. • Like grounding, unhooking can help us to refocus on what we need to do and to act on our values.
Acting on your values (Module 3)	Acting on our values means acting in line with our deepest desires for how we want to behave as human beings, what is important to us, what kind of person we want to be, and how we want to treat others.	1. Choose the values that are most important to you (such as being kind, being supportive, being hard working). Only you can choose which values are most important to you; no one else can decide for you. 2. Pick one small action that would be in line with your values. This could be something you do, or something you say to someone.	• Acting on our values can give us a sense of purpose and meaning, even in stressful situations.

Skill	Summary	How to practise	Benefits of practice
Values-guided problem solving (Module 3)	There are three approaches to any difficult situation: 1. leave; 2. change what can be changed, accept the pain that cannot be changed, and live by our values; 3. give up, lose touch with our values, and do things that do not help or make the situation worse. Values-guided problem solving focuses on option 2.	1. Change what can be changed. Use your arms and legs and hands and feet and mouth to take action. Do whatever you can to improve your situation, no matter how small it might be. 2. Accept what cannot be changed. Use unhooking and grounding techniques to manage difficult thoughts and feelings. 3. Find other ways to live according to your values.	• Values-guided problem solving can empower us to take effective action in difficult and stressful situations.
Being kind (Module 4)	Being kind means using kind words and actions with ourselves and with other people.	1. Unhook from unkind thoughts and feelings by noticing and naming them. 2. Speak to yourself kindly. You could also imagine filling one of your hands with kindness and placing it somewhere on your body where you feel pain. 3. Speak to others kindly and engage in kind actions.	• Being kind to ourselves gives us more energy to cope with stressful situations and to help others. • Being kind to others can make a difference, even in very difficult situations. It can also give us a sense of connection, well-being and fulfilment.

Skill	Summary	How to practise	Benefits of practice
Making room (Module 5)	Making room means allowing painful thoughts and feelings to come and go like the weather, instead of spending energy fighting them.	1. Notice a difficult thought or feeling with curiosity. 2. Name the thought or feeling. 3. Allow painful thoughts or feelings to come and go, like the weather. Breathe into and around the feeling to make room for it.	• If we stop fighting with our thoughts and feelings and allow them to move through us, we will have more time and energy to act on our values.

The rest of this chapter provides an overview of the content in each DWM module and gives tips for helpers on how to support service users, through example answers to likely questions. In all cases, make sure you personalize the content to each service user.

6.2.1 Contacts overview for DWM

The following approach can be used when delivering DWM with support contacts (see [Figure 6.1 Overview of DWM modules](#)).

1. **Welcome contact.** See section 4.1 for general information on the welcome contact. See [Annex 1A. Summary of tasks for welcome contact](#) for a summary of tasks for the welcome contact and [Annex 1B. Example script and text for welcome contact](#) for an example script and text.
2. **Support contacts.** these are delivered weekly and are tailored to the specific DWM modules. Aim for 15 minutes per support contact.
 - For general information, see Chapter [4. How to provide support to service users](#).
 - See [Annex 1C. Summary of tasks for regular support contact](#) for a summary of tasks for regular support contacts and [Annex 1D. Example script and text for regular support contact](#) for an example script or message text.
 - Before each contact, review the additional information about the specific module covered in the contact and adapt sections 3 and 4 of the example script or text ([Annex 1D. Example script and text for regular support contact](#)) (see sections below).
3. **End contact.** This contact can be integrated in the last support contact (module 5, “Making room”). See section [4.3 End contact](#) for general information, [Annex 1E. Summary of tasks for end contact](#) for a summary of tasks for the end contact and [Annex 1F. Example script and text messages for end contact](#) for an example script and text.

6.2.2 Module 1. Grounding

Module 1 includes several components.

1. **Welcome to DWM and introduction.** DWM is introduced and service users are asked to write down why they are using DWM and what they hope to achieve by the end of it.
2. **Stress and becoming “hooked”.** Service users learn what stress is, and how stressful thoughts and feelings can “hook” us and create more stress.
3. **Awareness exercise.** Service users do an awareness exercise to learn how to pay close attention to an everyday experience, such as having a drink.
4. **Grounding.** Service users are introduced to the idea of grounding using the analogy of a storm to describe stress and emotions: it is best to be on the ground, where it is stable during a storm.

They practise three different grounding exercises: introductory, short and longer grounding (see [Annex 6. Script for grounding exercise](#) for a grounding script for live contacts).

5. Commitment to practice. Service users plan to practise skills learnt in the module at home.

Tips for helpers (see also [Table 6.1 Overview of DWM skills and exercises](#))

Getting hooked

If someone asks what getting “hooked” means, say:

“Getting ‘hooked’ means that a difficult thought or feeling has taken your attention away from what you are doing. For example, you are talking with your friends, then you start thinking about something stressful. This ‘hooks’ you: you get lost in your thoughts and cannot focus on talking with your friends any more.”

This information can be used for section 4, on reviewing practice of techniques and exercises, of the support contact scripts (see [Annex 1C. Summary of tasks for regular support contact](#) and [Annex 1D. Example script and text for regular support contact](#)).

Awareness exercise

If someone asks about the awareness of drinking exercise, say:

“The aim of this exercise is to experience giving all your attention to the activity you are doing, in this case drinking. This is very different from how we normally drink without paying much attention to it. Learning to give something our full attention is the first step towards learning to manage stress better.”

Grounding

If someone asks about grounding, explain:

“Grounding is a technique for managing emotional storms and intense emotional states. In such moments, we can get lost in what we are thinking and how we are feeling and lose connection with what we are doing. Grounding brings us back to the present moment by using our senses.”

“With grounding, we are not trying to get rid of our difficult thoughts and feelings. Rather, we are just trying to unhook ourselves from them, and to connect with the world around us while they pass.”

If people think they are not doing the grounding exercise right, explain:

“The purpose of grounding is to connect with whatever is happening in the present.

Notice your thoughts and feelings. Slow down and connect with your body by stretching, pushing your feet down, and focusing on your breathing. Refocus on the world around you and on what you are doing.

It is common to get distracted and it does not mean you are doing anything wrong.”

If the exercise makes someone feel uncomfortable, say:

“That is okay, you can skip any parts that are uncomfortable.”

If someone says they get dizzy or light-headed during grounding, it may be because they are breathing too deeply or too fast. Say:

“This can be a common problem. Most of the time if you feel dizzy or light-headed, it means you are breathing too fast or too deeply. Try breathing more slowly and gently.”

6.2.3 Module 2. Unhooking

Module 2 includes six components.

- 1. Introduction.** Service users are welcomed back and encouraged to continue practising the awareness and grounding exercises.
- 2. Summary of last week.** Service users review the concept of getting hooked by difficult thoughts and feelings.
- 3. Difficult thoughts and feelings.** Service users are given an overview of the different ways that people use to try to get rid of difficult thoughts and feelings, noting how these generally do not work. A new approach to unhooking from them is then introduced.
- 4. Notice and name.** Service users are encouraged to identify and write down or draw the types of thoughts and feelings they have in their body that hook them.
- 5. Refocus.** Service users are instructed how to refocus (the third and final step in unhooking) by fully engaging in an activity and giving their full attention to whoever they are with and whatever it is they are doing.
- 6. Recap and commitment to practice.** Key points are summarized and service users plan how to practise unhooking at home.

Tips for helpers (see also [Table 6.1 Overview of DWM skills and exercises](#))

This information can be used for section 4, on reviewing practice of techniques and exercises, of the support contact scripts (see [Annex 1C. Summary of tasks for regular support contact](#) and [Annex 1D. Example script and text for regular support contact](#)).

If someone complains that they can’t get rid of their thoughts and feelings, say:

“Many of us were taught from an early age that we should be able to control our thoughts and feelings. As children we might have been told to ‘stop crying’ or ‘cheer up’. Or we might think that because we can control things in other areas of our lives (such as what we do), we should be able to control our thoughts and feelings as well. The reality is that thoughts and feelings do not work that way. We cannot control what we think or how we feel at any given moment. In fact, trying to get rid of a particular thought or feeling usually makes it more powerful.”

If someone asks why it is important to learn how to notice and name difficult thoughts and feelings, explain:

“Noticing and naming difficult thoughts and feelings allows us to see them for what they are. By doing this, they have less impact and influence over us. As you become more practised at unhooking, you will find that difficult thoughts and feelings lose their ability to frighten, disturb, worry or depress you.”

6.2.4 Module 3. Acting on your values

Module 3 comprises seven components.

- 1. Introduction.** Service users briefly review concepts from module 2 and are encouraged to keep practising.
- 2. Values.** Service users are introduced to the concept of values and how these differ from goals. They discuss how we may not be able to achieve our goals but can always find ways to act on our values. We can either act in line with our values or move away from them.
- 3. Identifying your values.** Service users review a list of values and select those they think are most important.
- 4. Acting on your values.** Service users are asked to think of a role model in their community, and consider what actions this person takes that they admire, and what values this person demonstrates through these actions. Then they choose which of these values they would like to model.
- 5. Making your action plan.** Service users think of a relationship that is important to them, identify the values they want to live by in this relationship and then plan small steps that they can take to act on these values.
- 6. Values-guided problem solving.** Service users learn how to address problems in their lives by changing what can be changed, accepting the pain of what cannot be changed, and living by their values (that is, being the sort of person they want to be).
- 7. Recap and commitment to practice.** Key points are summarized and service users plan how to practise values-guided problem solving at home.

Tips for helpers (see also [Table 6.1 Overview of DWM skills and exercises](#))

This information can be used for section 4, on reviewing practice of techniques and exercises, of the support contact scripts (see [Annex 1C. Summary of tasks for regular support contact](#) and [Annex 1D. Example script and text for regular support contact](#)).

Values

If someone wants to understand more about values and actions, you could say:

“Our values guide our actions. A value is our deep desire for how we want to behave in life. For example, if my value is to be kind, an action might be to look after someone who is unwell. When stressed, we may act in ways that move us away from our values. But at any point, we can choose instead to say and do things that move us back towards our values.”

If someone seems confused or does not understand the values exercises, say:

“The idea of values can be difficult to understand at first, but this explanation may help. When you read the list of values, which ones described the kind of person you would like to be? For example, maybe one person’s values are being ‘hard working’ and ‘responsible’, while someone else’s values are being ‘caring’ and ‘loyal’. It’s okay if your values are different from other people’s, or if your values are different from what people say your values should be. You are the only person who can decide which values are important to you.”

If someone asks why values are important, say:

“Identifying and connecting with our values gives us a sense of meaning and fulfilment. That is because no matter what our circumstances are or how difficult our lives might be, all of us can live by our values right now. We might not be able to control many things in our lives, but we can always behave like the sort of person we want to be, even in difficult situations.”

“Values also give a sense of purpose to our lives. They can guide us and motivate us as we move through life. When we are guided by our values, we can unhook from our difficult thoughts and feelings more easily, and then focus on what is important to us.”

Action plans

If someone cannot or does not want to write an action plan to show how they will act on their values, support and encourage them to give it a go. Give some examples of action plans (Box 6.1). You might say:

“Imagine doing one thing that is in line with your values and who you want to be as a person. Try to make a realistic plan and then imagine doing it. If you are unsure about how to do this, or would rather not do this exercise, that is okay.”

Box 6.1 Example action plans for acting on your values

- If I value being hard working, but I do not have a job, I could still act on my value by spending time each day looking for work, or taking care of my responsibilities such as looking after children, or helping others, even if I feel discouraged sometimes.
- If I value being kind and caring, I could act on my value by visiting my neighbour who is sick, spending some time talking to her, and making her some food.

6.2.5 Module 4. Being kind

Module 4 comprises five components.

1. **Introduction.** Service users are introduced to the module and encouraged to keep practising.
2. **Importance of kindness.** Service users consider how kindness from others is important and how we often need to learn how to be kind to ourselves.
3. **Unhooking from unkind thoughts.** The idea of unhooking is reviewed and service users use an audio exercise to support their practice.
4. **Small acts of kindness.** Service users learn how small acts of kindness can make a difference in even very difficult situations, and are asked to identify things they can say or do to be kind to others.
5. **Recap and commitment to practice.** Key points are summarized and service users plan how to practise small acts of kindness over the coming week.

Tips for helpers (see also [Table 6.1 Overview of DWM skills and exercises](#))

This information can be used for section 4, on reviewing practice of techniques and exercises, of the support contact scripts (see [Annex 1C. Summary of tasks for regular support contact](#) and [Annex 1D. Example script and text for regular support contact](#)).

If someone says that they already do a great deal for others, say:

“Yes, it sounds as though you are already kind and caring and do lots for people. This exercise is asking you to pay attention to the kind things that you already do, think of any additional things that you could say and do, and then to notice the effect it has on you.”

If someone asks why they need to be kind to themselves, say:

“Sometimes when we really need a kind friend, no one is available. When you are kind to yourself, everyone in your life benefits. The kinder and more supportive you are to yourself, the more energy and motivation you will have to help or care for others. Similarly, when you are unkind or unfriendly to yourself, everyone in your life is worse off. Treating yourself

in this harsh way saps your energy and lowers your motivation, leaving you less able to help or care for others.”

If someone asks how they can practise being kind to themselves, suggest the following steps:

“First, unhook from unkind thoughts. When you catch yourself speaking to yourself in a very harsh or critical way, notice: ‘Here is an unkind thought.’

Next, silently say to yourself, in a kind and caring voice, ‘This hurts. This is really difficult. This is natural ... Everyone has difficult thoughts and feelings like these ... All people experience this when they are stressed.’

Next, try out a friendly physical gesture: put your hand over your heart, or on your belly or forehead – wherever feels soothing and comforting. Send a message to yourself with this hand: ‘I see you are in pain, and I care about you, and I am here for you.’ Feel the warmth and kindness flowing from your hand into your body.

Notice what it’s like to be kind to yourself.”

6.2.6 Module 5. Making room

Module 5 comprises five components.

- 1. Introduction.** Service users review the unhooking concept and are encouraged to keep practising.
- 2. Making room for pain.** Service users are introduced to the concept of making room using metaphors.
- 3. Notice it, name it, and make room for it.** Service users make room for difficult thoughts and feelings by noticing and naming them and allowing them to be present, instead of struggling to get rid of them. They practise this skill with an audio exercise.
- 4. Being kind to yourself.** Service users do a new audio exercise, imagining their hand filling with kindness and using that hand to pour kindness into any area of their body where they feel pain.
- 5. What comes next.** Service users are congratulated for completing DWM and are encouraged to continue practising the skills they have learnt at home. If relevant, remind people that they will be asked to fill out a questionnaire and share how they can access this.

Tips for helpers (see also [Table 6.1 Overview of DWM skills and exercises](#))

- The support contact for module 5 of DWM has an “end contact” incorporated. See [Annex 1E. Summary of tasks for end contact](#) for a summary of tasks for the end contact and [Annex 1F. Example script and text messages for end contact](#) for an example script and text.
- If people feel sad or worried that this is the last module, encourage them to keep practising the skills they have learnt.

This information can be used for section 4, on reviewing practice of techniques and exercises, of the support contact scripts (see [Annex 1C. Summary of tasks for regular support contact](#) and [Annex 1D. Example script and text for regular support contact](#)).

If someone asks how making room is different from trying not to think about things, say:

“This can be confusing at first, but accepting pain is different from just doing nothing or trying not to think about it. When accepting it, you acknowledge that the pain is there, you feel the pain, and then make room for it. You allow it to be there instead of pushing it away or ignoring it. An important idea here is that with time all feelings come and go if we let them just be there.”

If people confuse grounding, unhooking and making room, explain:

“You might notice that this skill is similar to others you have learnt in this course. Many of the skills in this course are based on the same initial step of noticing your thoughts and feelings. Each skill does something slightly different. Grounding is for when you are wrapped up in an emotional storm. Unhooking is when you notice a difficult thought or feeling, it can be quicker than grounding. Making room is more for painful feelings. It adds another step to noticing and naming by imagining the breath flowing in and around the painful feeling. You might find you prefer some of the exercises or that they help in different situations. Try to practise all of them, but it is okay if you don’t like or don’t get all of them. Use whatever you feel helps you the most.”

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Annexes: tools and templates

Note for programme managers.

All the tools and templates in this section need adapting to align with your organization's own context and existing procedures. Please check the manual web page for information on how to access editable versions of these.

Annex 1. Summary of tasks for different contacts, and example scripts and texts

Note for implementing organizations

Annexes 1A–1G provide a summary of key tasks and example scripts and text for the different types of support contacts that are used when providing guided self-help. They are based on versions used in trials of SbS and DWM. They provide an overview of the different support contacts and topics to be covered. They should be adapted for local contexts and the helper’s style. All information can be adapted but implementing organizations should pay particular attention to information [\[in square brackets\]](#). These can also be adapted for use in person (for example, for integration in existing primary health care consultations) or for more or fewer support contacts.

When implementing SbS or DWM, the information to add to these templates can be found in Chapters 5 and 6, respectively.

Note for helpers

It is important that before the welcome contact you read any information you have been given about the service user, for example from a referring agency or through an online platform. If there is information that suggests the person has a safety concern (for example, risk of imminent suicide) you must follow your organization’s risk protocol (see section [2.4.2 Addressing safety and protection concerns](#) and [Annex 3. Example protocols to assess and address safety and protection concerns](#)). After each contact, follow your organization’s procedures for recording key information about the contact, for example using case notes (see [Annex 5. Case note examples](#)).

Annex 1A. Summary of tasks for welcome contact

Before the session, make sure you review any key information and adapt this outline to the intervention you are using.

1. Welcome the service user and introduce yourself

- Give the person your name and welcome them to the intervention.
- Tell them you are contacting them to explain more about the intervention.
- Ask them how they are (in the customary way).
- Explain how many minutes you have with them [\[15 minutes for SbS and DWM\]](#).
- Ask them what name they would like to be called.
- If the person is using a shared telephone for contacts, confirm who you should ask for when calling and tell the person how you will explain the reason for your call to anyone else who answers (for example, a call concerning health promotion from a local health organization). [\[Confirm any other phone security measures for shared telephones in line with organizational procedures\]](#).
- [\[Confirm the person's physical location, or the contact details for a supportive person, if applicable\]](#) who can be contacted in an emergency.

2. Tell them about the intervention

Briefly explain the intervention. Key points to make include:

- The intervention comprises [\[X number of modules\]](#), which are completed [\[X time frame e.g. weekly/over 5/6 weeks\]](#).
- If using a digital intervention, ensure that the person managed to access the platform and can navigate it.
- People using the intervention follow the materials through [\[X format e.g. mobile app/online/book/video\]](#), which takes around [\[X minutes e.g. 30/40/50\]](#) minutes. It is good to review the content at least once a week.
- After they complete a module, people should take time to practise the skills they have learnt. This is important because the intervention works better when you practise what you have learnt between modules.
- New content will be available after [\[X time, e.g. all at once or unlocked on a time schedule\]](#) and the person can then move onto the next module.

3[A]. (If person has chosen guidance) Explain what the service user can expect of you as their helper

Key points to make include:

- You are there to guide the person through the intervention. You are not a professional therapist and will offer support with the intervention but can only give limited help with other issues (such as financial or other problems).
- You will contact them approximately [X frequency e.g. once a week/every two weeks] for a maximum of [X time e.g. 15 minutes] because you are supporting many people each week.
- When you contact them, you will try [X number of attempts e.g. two/three/four] times according to their preferred way of contact; and if these are all unsuccessful, you will stop contacting them.
- Everything they tell you will be kept confidential, unless they or somebody else is at risk of harm. Mention how you might discuss information in supervision, but emphasize that the intervention team is also bound by the same confidentiality rules that you are. [Add any other relevant information about this in line with organizational procedures.]
- [Only if applicable] They will be asked to complete a [periodic/end of intervention/follow-up] questionnaire, which is designed to help us know how the intervention helps people. They only have to fill it in if they want to. [Add any other relevant information about follow-up.]
- If they feel they need it, you can point them to sources of extra support, such as local support services. [Add any other relevant information about other support.]

Explain the format of future calls, telling the person that you will first cover their experiences with the modules and then with practising the activities. You will also discuss any question they have related to the modules or activities. Ask them if they have any other questions, or whether they want to add anything else before you finish. Check if they can access the intervention application.

If the person starts talking about other problems they have faced, remember this is okay. Allow them to talk and express themselves and show empathy. But try not to let this exceed five minutes before moving on.

If you are using live contacts, tell the person you will let them know when you have five minutes left for the support contact.

3[B]. (If person has not chosen guidance) Check whether they would like guidance and explain what it means

Key points to include:

- Ask the service user if they would like to know more about the difference between completing the intervention guided or unguided.
- If they want to know more, then cover the following points:
 - it is possible to complete it with or without guidance;
 - without guidance means they will be completing it on their own;
 - with guidance means having regular follow-up calls with a helper.
- Explain that completing it with guidance may help the person to feel more motivated and engaged. It also gives an opportunity to ask any questions or review any problems.
- If the service user agrees to switch to e-helper support, tell them they will be assigned an e-helper.

4. Remind the service user of the next time you will talk (if you are their helper) and say goodbye

(... in the customary way).

5. Record key points in your case notes

After you finish the contact, write up your case notes and remember to record any calling instructions, including security questions and responses, if you are using telephone contact.

Annex 1B. Example script and text for welcome contact

The aim is to complete contacts in 15 minutes.

Live welcome contact script

1. Greeting and introduction

- “My name is [your name] and I am contacting you to welcome you to [intervention name]. (If you are their helper) I have been allocated as your helper, and my role is to support you through the programme.
 - I have about [15/20/etc.] minutes to tell you more about [intervention name], and for you to ask any questions.
 - First, some practicalities. How would you like me to address you? And are you sharing a phone with someone else? If you do and someone else answers the phone, I will say [mention organization’s protocol (e.g. a call concerning health promotion from a local health organization)]. Is that okay?”
-

2. Give information about intervention [remember to adapt this for the specific intervention being used]

- “[Intervention name] is divided into [five (for SbS or DWM)] modules, which should each take about [20/30/40] minutes to complete. Each module is split into parts with ideas and skills for you to practise to reduce [depression and psychological distress (for SbS)/psychological distress (for DWM)]. You can either do all the parts together on one day or divide them over several days.
 - The techniques you will learn are simple and can be used for just a few minutes each day. [There are also recordings to help you practise the techniques at home, which you will be able to access through X.]
 - It is best if you do one module per week. A new module will unlock [every week/after completion of previous module/etc.]. You will be notified when a new module has been unlocked.
 - It is really important to try and practise what you learn. It can also help to reread a module after practising for a few days because each time you do you will probably notice new things about the idea. You can also listen to the exercise recordings as many times as you want [by going to X]. The time and place where you use [intervention name] is up to you.
 - At the end of each module, you will be encouraged to make a plan for practising the skills you learnt.”
-

3[A]. (If the person has opted for guidance) Explain what to expect of you as their helper

- *“My role as a helper is to guide you through [intervention name] and I will contact you [once a week/every two weeks/etc.]. I am here to offer support with [intervention name] but am limited in how much I can help with other issues (such as financial problems or general questions). I am not a professional counsellor but support people when they are using [intervention name].*
- *I have about [15/20/etc.] minutes a week to help you and I am looking forward to working with you.*
- *What we talk about is confidential. This means I will not talk to your family or friends about it or anyone else without your permission. I may at times talk about our calls with people in my [intervention name] team, to help me give you the best support. There is one exception to this – if I am concerned for your safety or the safety of someone else, I may have to let my supervisor know to help keep you or others safe. Is this clear and do you have any questions about it?*
- *I will contact you for a maximum of [15 minutes] per week. I only have this amount of time because I have to support many people each week. As it is a short amount of time, I will tell you when you have five minutes left for the [call/chat] so that we make best use of the time. If you want, I can send you a reminder that we have a meeting [the day before (or as agreed with the individual)].*
- *Every time I call, we will first talk about your experiences with the [intervention name] module and I will answer any general questions you have about the concepts it covered. Then we will talk about how you have practised or used the techniques covered in the module. We can also discuss any questions you have about the exercises.*
- *Because I have to support many different people, we can only talk during our agreed meeting times. But feel free to contact me if you need to rearrange a meeting or if you are having technical problems with the programme (e.g. lost password). I do not monitor my phone or messages outside work hours, so please don't expect a response during these times. If you have any urgent need for support, you should call or go to your nearest emergency health care facility.”*

3[B]. (If person has not chosen guidance) Check whether they would like guidance and explain what it means

- *“Before I continue, I want to confirm something. You requested to do the programme without support from a helper? Is that correct?*
- *Of course, you're free to choose what suits you best. I just want to explain the support option in case you ever want to change your mind.*
- *If you choose to do the programme alone, you'll be working through the module and exercises at your own pace without discussing it with anyone.*

-
- *If you choose to get support, an e-helper will contact you weekly and help you complete the programme in five to eight weeks. The benefit of having support is the encouragement to complete the programme and the chance to get clarification on anything unclear.*
 - *Would that interest you?*
 - *(If the person consents to guidance) You will be assigned to a helper who will contact you with more information.”*
-

4. Finish the contact

- *“We have five minutes left to talk/chat. Do you have any questions or is there anything else you would like to say before I go?*
 - *That’s the end of our call/chat today.*
 - *(If you are their helper) I will call/chat again next week on [date and time]. Would you like me to send you a reminder before our next contact?*
 - *(If you are not their helper) I wish you good luck with [intervention name].”*
-

Delayed welcome message example text

Dear [service user name],

My name is [your name] and I am writing to welcome you to [intervention name], explain more about the intervention and give you a chance to ask questions.

(If you are their helper) I have been allocated as your helper, and my role is to support you through the programme. I have about [15/20/etc.] minutes a week to help you and I am looking forward to working with you.

In [intervention name] there are [five] modules to complete. Each module should take you about [30/40/etc.] minutes a week. The modules are divided into smaller parts so you can do it all in one day or over several days – it's up to you!

In each module you'll learn a new skill to practise between the modules. When you finish a module, you will need to wait for [X number] of days before doing the next one to give you time to practise what you have learnt. Practice is really important for [intervention name] to work for you.

(If person opted for guidance) As your helper, I am here to guide you. We can exchange messages each week. I have about [15/20/etc.] minutes a week to help you and I am looking forward to working with you.

I will send you a message to ask how you have been doing with [intervention name]. You can reply and ask me any questions you have, and the following week I will send you another message. If I don't hear from you then I will send you a reminder, and if I don't hear from you after [2/3/4] weeks, I will assume that you do not want to continue and I won't contact you again.

Please remember that these messages are confidential. I will not tell anyone else about you or our contacts, apart from my [intervention name] colleagues in case I need some guidance. The only time I would need to speak to someone about you would be if I was very concerned for your safety.

Give the first module a go and let me know if you have any questions. I will be in touch next week on [insert day/time] to see how you got on.

(If person has not chosen guidance) Is it correct that you requested to do the programme without support from a helper? Of course, you're free to choose what suits you best. I just want to explain the support option in case you ever want to change your mind. If you choose to do the programme alone, you'll be working through the module and exercises at your own pace without discussing it with anyone. If you choose to get support, a helper will contact you weekly and help you complete the programme. The benefit of having support is the encouragement to complete the programme and the chance to get clarification on anything unclear. Let me know if you want to switch to receiving guidance.

I wish you all the best,
[your name]

Annex 1C. Summary of tasks for regular support contact

Below is a summary of tasks for regular support contacts. It covers a summary of everything you need to cover in every support contact.

Preparation and follow-up tasks (or key tasks) for regular support contacts

- Use this summary of tasks as a checklist of everything to cover.
- Adapt the regular support example script or text ([Annex 1D. Example script and text for regular support contact](#)) for every contact.
- Personalize the contact to each service user (that is, use their name and other previously used personal information).
- Adapt section 4 “Review activities and exercises” to the intervention and module you are covering (for example, if the contact is after module 1 of SbS, you will adapt this section to cover key aspects of SbS module 1, such as using the grounding exercise). You can find the specific information in Chapter 5 for SbS and Chapter 6 for DWM.
- Adapt the script or message to your own style.
- Before the contact, remember (if technically possible, depending on how the self-help content is accessed) to check how much of the module they completed.
- The aim is to complete contacts in 15 minutes.
- Write up a case note to document your contact (see [Annex 5. Case note examples](#)).

Summary of tasks for regular support contact

1. Greet the service user

- Share why you are contacting them and how long the support contact will be.

2. Ask the service user how they are this week

- If the person starts talking about other problems they have faced, remember this is okay. Allow them to talk and express themselves and show empathy, but try not to let this exceed five minutes before moving on.
- If the person seems overwhelmed by their problems and are asking for your help, consider taking a few minutes to help identify support and coping strategies (see [Annex 2B. Service user needs more support for other problems in their life than can be provided in a brief support contact](#) and [Annex 2C. Service user needs support coping](#)).
- If someone is low in mood, they may have negative thoughts about themselves (for example, “*I am weak*” or “*It is my fault*”). Acknowledge these thoughts and beliefs and

explain that these are common when people face difficulties in life, particularly when someone experiences depression or stress. You can explain that thoughts are just thoughts, and having these thoughts does not make them true. You can refer people to useful skills in the intervention (for example, “being kind to yourself” in DWM and SbS).

If the person mentions any possible safety issues, see section [5.2.5 Module 3. Be kind to yourself](#).

3. Review the last module

Add specific intervention and module information to this section. For SbS and DWM, see the module content overviews in Chapters 5 and 6.

- If needed, review content from the module.
- Talk to the person about how they got on with the latest module.
- Praise them for starting or finishing the module.
- Ask them what they learnt from the module and any thoughts they had about it – did they understand the concept? If the intervention uses a story approach, can they relate to the characters?

4. Review activities and exercises

Add specific intervention and module information to this section. For SbS and DWM, see the module overviews in Chapters 5 and 6.

- Ask the person how they found the module and the techniques covered in the module, if they were able to do the activities or exercises, or what was helpful or hard.
- If necessary, remind the person of the aim of the exercise they were asked to try (see Chapters 5 and 6 for information about specific modules for SbS and DWM so you can personalize your contact to the relevant module).

In some digital self-help interventions, you may be able to use the platform to check whether the person has completed them before you have your support contact. Always let the person know that you will do this so they are aware how you got the information.

- **If the person completed the activities**, review this with them and ask them how they found doing the exercise. Be positive and supportive of the effort.
- **If the person did not complete the activities**, give them an overview of what happens in that part of the module, and discuss this with them. Consider completing the exercise with them. Have a further discussion about completing the activities. This may naturally flow from, or happen in, the section above, so may not always be needed. Otherwise, open a discussion about practising exercises in the person’s own life.
- **If the person managed to practise the exercises and techniques**, praise them for doing so. Explore how it made them feel to do some enjoyable activities. Did it make them feel better, even if for a short while?

- **If the person tells you the exercises did not go well**, or they say they did not do any, normalize and try to identify barriers and how to overcome them.
- **If the person could not identify any ways to overcome barriers**, name some possible solutions, preferably two or more so the person has a choice.

5. Finish the contact

- Remind the person you have five minutes left (if live contact).
- Ask if they have any questions or anything they would like to discuss.
- Remind them of the day and time of the next appointment.
- Say goodbye.

Annex 1D. Example script and text for regular support contact

- See box in Annex 1C on how to use this script.
- Before every support contact, review the module-specific information in Chapters 5 and 6 and adapt the script of the message text.
- The aim is to complete contacts in 15 minutes.

Live regular support contact script

1. Greeting

- *"I am contacting you today to give you some support on using [intervention name]. It's a chance for us to discuss your progress and help with anything that has been difficult in using it. I have [number of minutes] to discuss this with you today."*

2. Ask the service user how they are doing

- *"How are you doing? How have you been feeling the last week?"*

If the person is in low mood or expresses negative thoughts about themselves, offer some brief support, for example by saying:

- *"It sounds as though you are [low in mood/criticizing yourself]. This sometimes happens when people experience depression or feel down. It is a natural reaction to adversity. Remember that thoughts are just thoughts: just because you have them it doesn't make them true. You are not weak to experience depression and you should never blame yourself for it."*

If the person is distressed or appears to need more support with coping, follow the advice in the troubleshooting guide in [Annex 2. Troubleshooting templates](#).

3. Review the module

- *(if needed). "In this week and module you were introduced to [add content from module]. Even if your story is different, remember you can still use this to help you out."*
- *How was it this week – have you been able to go through the module? Was there anything in particular you found helpful?*
- *Was there anything you didn't like about it or found confusing?*
- *Is there anything about any of the techniques or exercises that you'd like to clarify?"*

4. Review practice of techniques and exercises

If using a platform that allows you to see if module activities were completed, then adapt these questions as needed. For SbS and DWM, add module-specific information from Chapter 5 and Chapter 6.

- *“How did practising the exercises at home go for you this week?”*
- *Were you able to [name activity]?”*

If the person practised the exercises:

- *“It is really great that you managed to find time to use the techniques this week. How did you find the practice?”*

Ask the person to share what kind of activities they are planning.

- *“That’s really good, I’d like to hear how they go.”*

If harmful coping behaviours or activities (for example, drinking alcohol to control emotions) are suggested, try to suggest more helpful alternatives.

- *“Great to hear you have thought of something to do. How do you think [name activity] will help you?”*

If the person did not practise the exercises:

- *“It can be hard finding time to do the exercises or use the techniques. Don’t worry if you weren’t able to do everything you wanted to. Every little bit of practice is important. What were some of the problems that got in your way? What do you think you can do to make it more likely that you will practise the techniques more next week?”*
- *If you couldn’t think of any activities to do, maybe I could help you to think of some things.*
- *The idea would be to try something small that you might usually enjoy or something that would make you feel proud. Do you have any ideas of what this could be?”*

Try to help the person make a plan on when they can practice.

If the person does not have any ideas on how to overcome barriers:

- *“Some people I’ve spoken with have found that setting a time to do their activity in advance, linking it to a common activity (such as getting up or going to bed) or explaining to their family that they need some time to do the exercises helps. Would any of these be something you could try?”*
-

Finish the call

- “We have five minutes left to talk. Do you have any questions or is there anything else you would like to say before I go?”
 - “That’s the end of our call today. I will call again next week on the [date and time]. Would you like me to send you a reminder before our next call?”
-

Delayed regular support contact example text

Dear [name],

I hope you are well and enjoying [name intervention]. How have you been doing this week?

Have you been able to go through the module? This last module was about [add summary of last module]. Was there anything in particular you found helpful? Or something you didn’t like about it or found confusing? As always, you can ask me anything that you may not have understood from the story.

The invitation this time was to [add activity].

[Depending on the system being used for Step-by-Step, you may be able to see information they have added for the exercises (for example, adding what activities they will complete). You can include information on this as follows.]

Is there anything about any of the techniques or exercises that you’d like to clarify?

How did practising the exercises at home go for you this week? Great if you managed to find time to use the techniques this week. How did you find the practice? I’d like to hear how they went.

It can be hard finding time to do the exercises or use the techniques. Don’t worry if you weren’t able to do everything you wanted to. Every little bit of practice is important. Try to think of anything that can make it more likely that you will practise the techniques more next week. If you couldn’t think of any activities to do, maybe I could help you to think of some things.

I hope this helps. Feel free to reach out if you have any questions or want to comment on anything I’ve written here.

I look forward to connecting with you again next week.

Best wishes,
[your name]

Annex 1E. Summary of tasks for end contact

The end contact can be integrated in a final support contact for the intervention (as it is for SbS and DWM: see section [5.2.7 Module 5. Remember, change takes time](#) and section [6.2.6 Module 5. Making room](#)), but it could also be a stand-alone contact if that works better. See section [4.3 End contact](#) for more information on the end contact.

Summary of tasks for end contact

1. Congratulate the service user on completing the intervention, and ask them whether they notice any difference compared with how they felt when they started SbS.
2. Remind them that even though they have finished, they can continue to practise the skills they have learnt. Help motivate people by reflecting on their positive change. Be realistic and acknowledge that sometimes our mood fluctuates, but try to also convey a positive attitude.
3. Explain if and how they can still get access to the intervention materials.
4. Remind them that it is important to complete any final questionnaires and inform them about any further follow-ups or assessments protocols (to be adapted by the implementing organization).
5. If someone does not feel better, suggest that they get help from a local service (to be provided by the implementing organization).

Annex 1F. Example script and text messages for end contact

Live end contact script

"It is the last of our scheduled support contact today. But don't worry, because with all the skills you have learnt, I hope you will manage a little better and get more enjoyment out of life."

You made it to the end of [intervention name], that's great! I can see that you have shown real dedication and determination in finishing. I hope you enjoyed it and can see some benefits in your life compared with before you started."

[Add information about the last module here if the end contact is combined with the support contact for the last module.]

"Although you have reached the end of the intervention and our support contacts will end, try to keep these techniques a part of your daily life. It is common to feel worse every once in a while again. You can always go back to the techniques to help you feel better again."

[Add in any information related to keeping access to the intervention. For example:]

"You will still be able to access [intervention name] for add number] weeks in the same way you have accessed it in the last couple of weeks."

[Add in any information related to final questionnaires or assessments here. For example:]

"It would be great if you could complete the feedback questionnaire if you haven't already done so. This helps us to monitor how helpful people find Step-by-Step."

[If someone does not feel better, suggest that they get help from a local service:]

"If I don't hear from you again, then I would like to take this opportunity to wish you all the very best for the future."

Delayed end contact example text

Hi [name],

You made it to the end of [intervention name], that's great! I can see that you have shown real dedication and determination in finishing. I hope you enjoyed it and can see some benefits in your life compared with before you started.

This will be the last of our scheduled support contacts today. But don't worry, because with all the skills you have learnt, I hope you will manage a little better and get more enjoyment out of life.

[Add information about last module here if the end contact is combined with the support contact for the last module.]

Although you have reached the end of the intervention and our support contacts will end, try to keep these techniques a part of your daily life. It is common to feel worse every once in a while again. You can always go back to the techniques to help you feel better again.

[Add in any information related to keeping access to the intervention. For example:]

You will still be able to access [intervention name] for [add number] weeks in the same way you have accessed it in the last couple of weeks.

[Add in any information related to final questionnaires or assessments here. For example:]

It would be great if you could complete the feedback questionnaire if you haven't already done so. This helps us to monitor how helpful people find Step-by-Step.

[If someone does not feel better, suggest that they get help from a local service:]

If I don't hear from you again, then I would like to take this opportunity to wish you all the very best for the future.

Kind regards,

[your name]

Annex 1G. Example texts for missed contacts

The following are templates to send to service users when they miss regular support contacts.

Example message template for one missed contact

Hi [name],

I was sorry not to hear from you as planned. I hope that things are okay. Keep working on [intervention name] and I will be in touch at the same time next week.

Let me know if you are having problems attending, or if you no longer wish to have our contacts. Remember, if you need urgent support, you can contact [add in contact and adapt for context].

I hope to hear from you soon.

Kind regards,
[your name]

Example message after final contact attempt

Hi [name],

I was sorry not to hear from you as planned. I hope that things are okay.

As this was the [third/fourth] week in a row that we have not been able to talk over the phone, I am assuming you do not want to continue with the calls. As we agreed when you first started [intervention name], I will not continue to contact you. If you would still like to have contact, please call or message me to arrange a time.

If I do not hear from you then I wish you well. Remember you can continue to use [intervention name] even if we are not speaking.

Kind regards,
[your name]

Annex 2. Troubleshooting templates

The following annexes provide helpers with a summary of tasks and example scripts for responding to a selection of common problems that might arise when guiding people through a self-help intervention.

Annex 2A. Service user experiences distress during a contact

Anger, sadness or anxiety are normal human emotions. It is uncommon but it can happen that a service user becomes very distressed during a support contact. They may get angry, sad or worried. In some cases they may be very anxious (for example, experiencing an anxiety or panic attack where they find it hard to breathe or being anxious about a serious health problem).

1. Remain calm and acknowledge the distress

Be aware of your own stress and keep your voice and breathing calm. Acknowledge the person's distress by saying what you are hearing or seeing.

- *"I hear that you are feeling anxious/sad/angry at the moment ..."*

2. Give time and space

Allow time and space for the person to feel distressed. If they are crying you don't need to make them stop. Remain calm and supportive.

You can offer to talk to them about what is upsetting them, but do not rush into doing this. Allowing a few minutes to be distressed means they may start talking to you about it. If they do, then listen and use your helping skills.

3. Consider an exercise to reduce distress

If the user is distressed, for example panicking, feeling very angry or out of control, suggest that they do a grounding exercise (see [Annex 6. Script for grounding exercise](#)) or a slow breathing exercise (see [Annex 7. Script for slow breathing exercise](#)). Remind them that you are there to help them.

- *"Shall we try a breathing/grounding exercise together? I'm here to help you."*

4. Remind them that emotions like this are natural

- *"How you're feeling is really natural, many people feel like this but these emotions and thoughts will pass."*
- *Emotions are there to help protect us and get us to take a helpful action."*
- *Unpleasant emotions are hard to bear, just know they are not there to harm us and will pass."*

5. When calmer, offer to help them identify support and coping strategies.

- “Would you like me to help you to think about what you could do?” and “I have a list of contact details of some local organizations that may be able to help you.”

If appropriate, you may want to explore support options (see Annexes [2B. Service user needs more support for other problems in their life than can be provided in a brief support contact](#) and [2C. Service user needs support coping](#)).

6. End this part of the contact and move back to the intervention

Ask the service user whether they want to continue with the contact or end it at this point.

- “Shall we continue and move back to the content, or would you prefer to leave it here for today?”

7. Inform your supervisor if needed

If it is a particularly distressing situation or it has upset you, speak to your supervisor and if necessary take a bit of time before your next contact with the service user.

Be sure to inform your supervisor about what happened next time you meet if it was not an emergency.

Example message for delayed contact

Hi [\[name\]](#),

It sounds like you might be experiencing some distress. If you are, try not to worry, this is natural, especially when we go through difficult times. These feelings will pass but there might be more we can do to help you in the meantime.

I wondered if you have tried the breathing or grounding exercise from the [\[Step-by-Step programme/DWM\]](#) and if that helps you? Or talking to someone close to you about how you are feeling?

I also wondered if you would like to have a call to discuss how you're feeling a bit more, and I can tell you about some of your local support organizations.

Let me know if you'd like to do this, and as always remember that if you are in a crisis you can also contact the emergency services.

Kind regards,
[\[your name\]](#)

Annex 2B. Service user needs more support for other problems in their life than can be provided in a brief support contact

Some people may find it difficult to stop talking about problems in their life that cause them distress, making it hard to focus on the intervention. As a self-help intervention with only brief weekly support, the amount of time that can be provided to supporting service users with other problems in their life is limited. In a short call it is usually possible to offer some (limited) support with a problem, while also focusing on the intervention. It is important to try to do both, but this can be challenging for helpers, especially when new. This can be addressed with ongoing supervision.

Here it is important to respond using your basic helping skills, allowing the person time to talk, but also linking back to skills in the intervention that might help them or other sources of support. Try to make sure the person does not feel rushed or that the only thing they can talk about is the intervention. At the same time, try to move the conversation back to the intervention after a brief time. It may feel unusual, but signposting and support can be provided quite quickly. Remember not to become the person's therapist or social worker.

1. Use basic helping skills to provide support

Do not force the person to talk but explain that you are there to provide support if desired. Sometimes just acknowledging a person's distress and simply offering to listen to them can be enough.

- *"How can I help? Is there something you would like to speak to me about?"*
- *"Thank you for sharing your experiences with me. It sounds really difficult."*

If the person is very distressed, you could say something like this:

- *"I can see that you're very upset. I am here to support you. We can talk or just sit here quietly."*

2. Identify the service user's current coping and use of intervention skills, while reminding them of your role and the limited help you can provide

- *"I'm not able to offer much direct help with things outside [intervention name]. But we can spend a few minutes talking together to see if there is anything that can help."*
- *"What are you doing to cope or get support?"*
- *"Have you found any skills from the course helpful?"*

If needed, encourage the person to continue using any positive and safe coping strategies.

- *"This sounds like a difficult situation for you. What are you doing to cope or to get support?"*
- *"It sounds like this is helping you to cope. I would encourage you to continue with it."*

You can also encourage the person to use the skills they have learnt in the intervention. If someone is very clearly distressed, you can offer to lead them through one, for example a brief grounding exercise.

- *“Have you found any exercises in this course helpful?”*
- *Do you think it could be helpful to practise [name skill] now? Or, would you like me to talk you through a grounding exercise?”*

3. Signpost the service user to another service that may be able to help them

Ask questions to help the person think of other ways to support themselves (for example, calling on specific people or organizations, or using new intervention skills). It is important to ask open questions and to make suggestions rather than give advice. You can suggest local organizations or resources (such as a community leader or refugee organization) for some problems that the person tells you about (such as medical conditions, requests for help with food or employment). If the person does not already know about these services and supports, encourage them to connect with them. But remember that it is the person’s choice whether they follow through or not.

- *“Are there other people or organizations who might help you with this?”*
- *What have you done in the past when you have faced difficulties? Would doing that help you now?*
- *Some people may find that an organization or an important person, such as [the village leader], can help at times like this. Can you think of an organization or an important person who could help with this?*
- *Are there skills you have learnt in [intervention name] that could help with this problem?”*

Annex 2C. Service user needs support coping

If external problems are preventing someone from progressing with the intervention, you can support their coping strategies. In this situation, you can spend a few minutes helping the person to identify their own coping strategies and support, following the four steps below.

When discussing external problems remember to:

- ask open-ended questions;
- not rush into providing suggestions too quickly;
- give the person time to talk and think for themselves.

1. Summarize and paraphrase

Summarize your understanding of the situation and remind the person that you can only offer limited help in your role as helper.

- *“I see ... so it sounds as though you are having trouble with [X problem]. As you know, I can only offer limited help to you, but we could speak for a few minutes to see if we can identify things that might help you.”*

2. Identify current supports or coping strategies the person is using that may be helpful

Encourage the person to identify and use any positive and healthy coping strategies they have available. You can also encourage them to use the intervention’s skills, such as slow breathing or grounding, to help them feel calmer. For example:

- *“This sounds like a difficult situation for you. What are you doing to cope?”*
- *What have you done in the past to cope with similar difficulties?*
- *What might you say to a friend who was going through this?*
- *It sounds as though [X strategy] is helping you to cope with this situation. I would encourage you to continue with it. Remember that the slow breathing and grounding exercises can be used to help you feel calmer in the moment.”*

If the person is doing nothing to cope you can say:

- *“It sounds as though you’re finding things really tough and don’t know what to do to cope. Let’s see if we can think of anything together that might help.”*

3. If needed, help the person identify additional strategies and resources for coping with their situation

Ask the person if they can think of any strategies or resources they could use to help them cope – for example, people they could talk to or intervention skills they could use.

Ask if they are aware of any organizations that they can contact for support (for example, with a medical condition, financial problem or relationship issue).

If they are unsure, you could signpost them to trusted organizations (such as a community leader, refugee organizations, mental health charities, helplines), but remember to make this a suggestion rather than giving them advice.

You could say:

- *“Are there other people or organizations that might help you with this?”*
- *“What have you done in the past when you faced difficulties? Would doing that help you now?”*
- *“Are there skills you have learnt in the intervention that can help with this problem?”*

Encourage the person to connect with these other resources but remember that it is their decision whether they follow through with your suggestions.

4. Summarize

Repeat back what the person has said they will do to get help and gently bring the conversation back to the intervention support contact.

- *“It sounds as though you’re having a tough time, so to summarize, we’ve said that you’ll try talking to your friend and contacting [name organization] for help. Let’s review when we next speak and if it’s okay with you, we’ll continue with talking about [intervention name].”*

Example message for delayed contact

Hi [name],

It sounds as though there is a lot going on for you now. I understand that [X is happening, for example, you are concerned about your financial situation]. Unfortunately I'm only able to offer limited support with that because my role is limited to [intervention name] and it's not my expertise.

I wondered if there is anything you can try to help you – perhaps talking to a trusted loved one or a support organization? Do you know any local to you? You could try for example [X organization], who I know have helped people with this sort of thing in the past.

Even though you are worried about this, it sounds as though you have made good progress with [intervention name]. Do you have any questions about the modules so far? Let me know how you get on with the next module and I will be in touch again next week.

Kind regards,
[your name]

If the external problem relates to a risk issue, such as imminent risk of suicide, or a protection concern, always follow your organization's relevant protocol (see section [4.8 How do I manage imminent risk of suicide and other immediate safety concerns?](#) and [Annex 3. Example protocols to assess and address safety and protection concerns](#)).

Annex 2D. Service user continues to talk about the intervention beyond the agreed contact time, or sends very long messages

Service users may want to talk about the intervention or life difficulties for longer than the allocated contact time. This may be more common in earlier support contacts, when people may not be used to the idea of such a short time slot. To help to avoid this problem, remind the person about time frames:

- at the start of the contact:
 - “Remember we only have [15 minutes] together today.”
- towards the end of the contact:
 - “We only have five minutes left to talk”.

In some situations (for example, imminent risk of suicide) you may need to extend the call. If someone wants to talk beyond [15 minutes], continue being empathetic, kind and sensitive in your responses.

Follow the steps below to help end the call.

1. Stop the conversation

At the end of the [15 minutes], tell the person that you’re sorry but you’ll have to stop them there as you’re at the end of the support contact .

2. Look forward to next week

Tell them you look forward to picking things up with them at the same time next week.

In the case of delayed contacts, remind the person that you have [15 minutes] allocated to them each week, and ask them to prioritize the key things that they want to tell you that week. This will help you to spend your time on the most important elements.

Annex 2E. Service user reports problems with the exercises or home practice

Service users can report a range of problems with the exercises they were meant to do or with their home practice. Below are some suggestions as to how to respond to some of the most common.

The person finds the skills too simple, basic or repetitive

It can help to acknowledge that although the skills may be simple or basic, they are known to be very effective.

Explain the importance of repeated practice to ensure they can use the skills when distressed. It can help to use an analogy.

- *“Yes, you’re right, these skills in a way are simple. Even though they’re simple, they can be quite effective and we need to practise them many times so that we can use them automatically when we are facing a lot of stress.”*
- *“Even very simple skills can be very powerful, and they still require a lot of practice. This can help them to become more automatic. For example, in football it is important to practise the same move over and over again so it becomes easier and natural. Even very talented players need to practise. The same is true with the skills in [intervention name].”*

The person says the intervention is making them feel stressed or upset

Sometimes a breathing exercise can make a person feel dizzy or light-headed. Often, this is because they are breathing too deeply or too quickly. If this happens, remind the person that they should do the exercise in a way that makes them comfortable. Remind them to breathe slowly and gently, or just breathe normally and focus on the other parts of the exercise.

- *“This can be a common problem. If you feel dizzy or light-headed most of the time it means you are still learning how to do it well, you may be breathing too quickly or too deeply. Try to breathe more slowly, and focus on noticing the things around you. As you practice, the benefits will become clear.”*

Sometimes a person may become upset if the intervention causes them to think about difficult situations. You can reassure them that this is okay, and that these feelings do not stay forever. You can suggest that they continue trying the intervention’s exercises as they are aimed at reducing distress, and they tend to get easier with practice.

- *“When we talk or think about difficult situations, we can experience distressing thoughts and feelings. This is quite common, and it is important to remember that these feelings do not stay forever – rather, they come and go like the weather. It is okay if you do not want to do an exercise, but you might find that trying these exercises will help how you feel.”*
- *“I know facing feelings can be hard. Just know that facing difficult feelings today can help you feel better over time and help you reach your goals.”*

If the intervention is making them feel worse, or a difficulty keeps occurring, discuss this with your supervisor.

The person did not remember to practise

Make context-appropriate suggestions, such as setting an alarm or mobile phone reminder, asking someone to remind them, writing a reminder somewhere obvious, or linking it to a predictable daily task, such as washing or dressing in the morning or when arriving home.

- *“That’s a common problem. It can be hard to remember when we have so many other things going on, but it is really important because it’s through practice that we really see the difference. Do you have any ideas on how you could help yourself remember?”* [Prompts: set a reminder on your phone, or put a note somewhere in your home.]

The person says they struggle to do the skills in stressful situations

Encourage them to first practise during less stressful situations. You could use an analogy to explain this, for example, how a team practises before a football match, or how some people practise cooking a dish for themselves before making it for other people.

- *“It is harder to practise in stressful situations, so I’d always recommend practising a lot in less stressful situations to start with – like practising sport with your team before a match, or practising cooking a dish before making it for other people.”*

The person says they felt too bad to do the exercises

Remind them of the rationale for the exercise.

Explain that difficult feelings can occur, but practising the skill should help them with these feelings, so it can be useful to do the exercise even if they do not feel like it.

- *“It can be hard to motivate ourselves when we feel low or anxious, but these skills are to help you with these emotions, so they can be useful to do even when you don’t feel like it. It’s through doing these things, even when we feel bad, that we start to notice a difference in how we feel.”*

If they have practised the exercises, always praise their efforts, however small, and discuss ways to make practising easier to manage.

- *“It’s great that you’re taking part in [intervention name]. Sometimes people struggle to do the home practice, but you’re talking to me about it so that’s a really good start. There can be many reasons why practising is hard, let’s troubleshoot that together – tell me a bit more about why you are struggling to practise the exercises.”*

The person found the skill did not help so is reluctant to try again

Explain that it can take time before they start to feel better and encourage them to keep trying.

- *“It can take a while before people start feeling better using these techniques. I’d encourage you to keep trying as much as you can. Have you noticed any changes, even if very small?”*

The person says they are confused by the exercise

Use module-specific content to clarify concepts (see Chapters [5. Information for helpers delivering Step-by-Step](#) and [6. Information for helpers delivering Doing What Matters in Times of Stress \(DWM\)](#)) and encourage them to keep trying.

- *“It is okay if some of the course is unclear to you; these are new concepts. Please just continue to work through [\[intervention name\]](#) and try to practise and see what it is like.”*

The person indicates not having enough time for the exercises

See [Annex 2J. Service user indicates not having enough time for the intervention or exercises](#).

Annex 2F. Service user lacks motivation

If the service user tells you they are not motivated to work through a module or do an exercise, follow the steps below.

1. Reassure them and validate their experience

Tell the person that it is common to feel this way when in a low mood.

- *“I see you are having trouble finding the motivation for [intervention name]. It is very common for us to feel less motivated when we have a low mood. Struggling in life often gets us down and this is a natural reaction to adversity.”*

2. Encourage them

Remind the person of the strength they have shown in starting the intervention.

- *“Remember you’ve shown strength in starting [intervention name] and this is great!”*

3. Explore past coping strategies

Talk through the things that the person has done in the past that helped, for example talking to someone in their support network.

- *“Let’s try to explore together things you’ve done in the past to cope. Are there people you could talk to who could help you with this?”*

4. Explain the link between mood and motivation

Explain that as mood improves, it’s likely that the person’s motivation will also improve.

- *“Remember that as your mood improves, it’s likely that your motivation will too.”*

Annex 2G. Service user does not respect boundaries

There are many ways people can react to helpers. They may take out their distress or anger on you, or they may want to be your friend and meet you in real life. Your role as their helper is to support them for [15 minutes] a week. You should not meet outside this time. You are not the person's friend.

Below are some ways service users might not respect boundaries, alongside suggestions on how to deal with them.

The person is being aggressive or insulting or is asking personal questions

Stay kind and empathetic.

- *"It sounds like you are feeling quite [frustrated/angry/annoyed] today. I can try to help you as best as I can in the time we have, but to do that it will help if we can talk to each other a bit more calmly. Is it okay for us to continue like that?"*

Give a warning about their behaviour.

- *"I'm sorry, that type of language is unacceptable and we expect people not to speak to us in this way. I understand you might be distressed and we can only continue if you stop being [rude/insulting]. Would that be possible? If not, we can stop for this week and start again next time."*

Remind them of your role and boundaries.

- *"I notice that you want to learn a bit more about me. As we're limited on time, I'm keen to focus on [intervention name] and we have a policy not to answer personal questions about ourselves. My role is to help and support you so let's focus on how you did this week."*

If their behaviour continues speak to your supervisor.

The person contacts you excessively outside your allocated contact times

Tell them that you are not able to talk at the moment.

- *"Unfortunately I'm not able to talk at the moment."*

Remind them that you can only offer limited support about the intervention each week and that you will contact them soon to do a support module.

- *"Let's chat at our next support contact. Remember I have [15 minutes] a week and can offer you support about [intervention name]. I look forward to talking with you then."*

If it keeps happening, ask your supervisor for advice.

Annex 2H. A friend or family member calls to ask about the intervention

A service user's partner or family member might call you to ask you who you are and what the service user is doing or sharing with you, or to ask about the intervention. Follow the steps below if this happens.

Throughout these steps, it is important to maintain confidentiality. To prevent a service user's friends or family calling you, do not respond to any calls that come from an unrecognized number. [\[Add in any intervention-specific information, for example, all numbers for service users should be registered on your helper phones upon assignment.\]](#)

1. Stay concise and clear

2. Do not give out any information about the service user

Inform the person calling that this is a health programme and that its regulations and confidentiality rules do not allow you to give any information about the service user.

- “I work for a health programme, the confidentiality regulations mean that I cannot give you any information. I don't have the information you're looking for but I can refer you to my supervisor.”

Annex 2I. Service user (or a family member) wants to stop the intervention

People can be unhappy about or stop engaging with the intervention for many reasons, for example:

- they are not sure why they are completing the intervention;
- they do not understand the ideas presented;
- they do not like the intervention or are bored by the content; or
- they are experiencing other problems in their lives.

If the service user wants to stop

Tell them it is okay to want to stop and try to understand more while not putting pressure on them.

- *“I understand you want to stop and that’s okay. I just want to understand better – what is it that you’re unhappy about?”*

Choose your response depending on their reason for wanting to stop (see other subsections of [Annex 2. Troubleshooting templates](#)). If they still want to stop the modules after you have talked to them, then accept their decision.

If a family member wants the service user to stop

Try to get a little more information. If there is a risk issue, discuss it with your supervisor. The person can always stop the intervention and seek support from a local health service.

If the person is happy for you to talk to the family member, contact them to introduce yourself and explain what the intervention is. Always discuss this with your supervisor and never disclose any personal information about the service user.

- *“Hi, my name is [your name] and I work for [service name]. I am working with [service user name] and wanted to tell you a bit more about [intervention name] and how it can help.”*

If the person does not want you to talk to the family member, ask whether there is anything they could say themselves to explain to or show the family member what the intervention is about.

- *“Sometimes people might want you to stop doing [intervention name] because they don’t understand what it is about. Is there anything that you could tell them or show them that might help them understand what [intervention name] is and how it might help you?”*

Reinforce the intervention’s benefits and explore what they have found useful so far. Offer encouragement but also recognize they are free to stop if they wish.

- *“Has anything from [intervention name], no matter how small, been useful for you? Would you be willing to practise this skill and see if anything else becomes useful in later modules?”*
- *[Intervention name] can be difficult at first, but many people find it useful to stay with it for the full [X] modules. If you don’t want to, you’re free to stop any time.”*

Annex 2J. Service user indicates not having enough time for the intervention or exercises

- Acknowledge that it can be hard to find time and praise their efforts.
- Help them to identify a time (for example, five minutes) in their week where they can go through the intervention or practise regularly – for example in the evening before going to sleep.
- Remind them they can access the intervention whenever they want.
- Encourage them to start by practising a short exercise from the intervention (such as a breathing or grounding exercise – see Annexes [6. Script for grounding exercise](#) and [7. Script for slow breathing exercise](#)).
- Remind them they can practise these short exercises during other activities, such as waiting in line for something or before going to sleep.
 - *“It can be hard to get time to ourselves sometimes. The great thing about this intervention is that the modules will not take too long and can make quite a difference.*
 - *Are there times of day or places where interruptions are less likely, and you have some time to yourself? [Prompts: Could you practice when you’re doing another activity, such as waiting in line or just before you go to sleep?] Let’s try and identify a time each day where you can find five minutes or even just two minutes to practise one of the techniques.*
 - *Life can be really busy, and it is a struggle to get time to ourselves sometimes. Let’s discuss possible times or places where interruptions are less likely to happen.”*

Annex 3. Example protocols to assess and address safety and protection concerns

Annex 3A. Example protocol for assessing imminent risk of suicide

Source: [Early Adolescent Skills for Emotions \(EASE\)](#): group psychological help for young adolescents affected by distress in communities exposed to adversity. Geneva: World Health Organization and the United Nations Children's Fund (UNICEF); 2023. Licence: CC BY-NC-SA 3.0 IGO.

Helpers should follow their organization's procedures for assessing imminent risk of suicide. The example protocol below can also be adapted by organizations for their own contexts.

Ask both question 1 and question 2 and record the answers on the form. If the service user has already clearly provided enough information to answer a question, you do not need to ask the question again. First introduce the topic.

Example script for introducing the risk assessment questions:

"I'm glad that you're telling me about this. Sometimes when people are very upset or feel hopeless, they have thoughts about death or ending their own life. These thoughts are not uncommon, and you should not feel ashamed about having such thoughts if you do. I would like to ask you some more questions to better understand how things are for you. Is that okay? Can we continue?"

Question 1: In the past month, have you had thoughts of ending your life or a plan to end your life? Or have you taken any actions to end your life in the past year?	YES	NO	
	If the answer is “no”, circle “no”.		
	If the answer is “yes”, ask the person to describe their thoughts, plans or actions taken. Write details here:		
Question 2: Are you currently thinking of ending your life, do you have a plan of ending your life, or are you currently taking any steps to end your life?	YES	NO	UNSURE
	If the answer is “yes” or the person is “unsure”, ask the person to describe their thoughts, plan or actions to you. Write details here:		

- **If the person answers “yes” or “unsure” to Question 2, they may end their life in the near future. Follow the imminent risk steps below.**
- **If the person answers “yes” to question 1 and “no” or “unsure” to question 2 but is extremely agitated, violent, distressed or not communicating, they may end their life in the near future. Follow the imminent risk steps below.**
- If the person answers “yes” to question 1 and “no” to question 2 and they are NOT extremely agitated, violent, distressed or not communicating, they are unlikely to be at imminent risk of self-harm or suicide, but there might still be a risk. You do not need to follow the steps below, but tell the person you will inform your supervisor to discuss any additional support that may be helpful. For example, say this: “It’s very important that you have told me how you are feeling. These thoughts can happen when things are really difficult. If at any time you feel this feeling is getting worse, or you feel a risk of harming yourself or ending your life, then please make sure you speak to me.” When you talk to the service user, make sure to ask whether they still have thoughts about ending their life and ask question 2 accordingly.
- If the person answers “no” to both question 1 and question 2, thank them for answering your questions and continue with the remainder of the pre-intervention assessment.
- In case of any doubt about a person’s safety, talk to your supervisor.

Imminent risk: Steps to immediately take if the person may end their life in the near future:

- **You must always contact your supervisor immediately.** Explain this to the person. For example:
“From what you have described to me, I am concerned about your safety. As I mentioned before, if I believe that you are at risk of ending your life, I must contact my supervisor. This is very important, so we can get you the best kind of help for these problems as soon as possible. I am going to do this now, okay?”
- Ask the person to stay with you online or on the phone.
- Contact someone the person trusts. To find out this information, you could say:
“I would also like to contact someone in your community to ensure that you can be kept safe. Who would that be?”
- Ask the person where they could go that will be a place of safety. This could be a hospital, or perhaps to the home of a trusted friend or family member.
- Use your basic helping skills to give the person support while you wait for instructions from your supervisor.
- Ask the person if they have access to any means of self-harm. Discuss ways to remove these items from their home environment.
- Attend to the person’s mental state and emotional distress.

- Explore reasons and ways to stay alive.
- Focus on the person's strengths by encouraging them to talk about how earlier problems have been resolved.
- Give them the details of the local services and advise them that they can also contact emergency services if needed.

Guidance for asking about suicide or self-harm

Ask direct, clear questions

- When asking questions about suicide, avoid using indirect words that could be misunderstood.
- Direct questions help the person feel that they are not being judged for having thoughts or plans of suicide or for having made suicide attempts or self-harmed in the past.
- Some people may feel uncomfortable talking with you about suicide, but you can tell them that it is very important for you to clearly understand their level of safety.
- Asking questions about suicide will not cause the person to take action to end their life, but often helps them feel understood and less distressed.

If you are concerned about risk and lose connection (for example, the internet or phone signal goes down or the person hangs up or goes offline)

In the first support contact:

- Try reconnecting with them and remind them that it is important for you to ensure their safety. Check whether they want to continue with the support contact.
- If they do not answer and you are concerned for their safety, you may need to speak with the safety contact agreed at the start of the programme, if there is one.
- Tell the person about different support services available.
- Contact your supervisor to let them know so they can support you.

For information on how to ask questions about suicide and self-harm, and what to do if you are working online and get disconnected while concerned about a person's safety, see section [4.8 How do I manage imminent risk of suicide and other immediate safety concerns?](#)

Annex 3B. Steps to follow in cases of sexual violence, intimate partner violence or other protection concerns

Sources. Adapted from:

- Group Problem Management Plus (Group PM+): group psychological help for adults impaired by distress in communities exposed to adversity. Generic field-trial version 1.0. Geneva: World Health Organization; 2020 (<https://iris.who.int/handle/10665/334055>, accessed 18 September 2025).
- Self-Help Plus (SH+): a group-based stress management course for adults. Generic field-trial version 1.0. Geneva: World Health Organization; 2021 (<https://iris.who.int/handle/10665/345349>, accessed 18 September 2025).

Remember that people may find it hard to talk about sexual violence or intimate partner violence because of cultural taboos, shame or fear of stigma, or because they may be scared for their safety, or because thinking about their experiences causes them distress. They may face discrimination and rejection from their family, community or wider society. They may be pressured to stay quiet, may be blamed or may not be believed.

If someone chooses to talk about these experiences with you, they are showing great courage and trust. It is very important to show willingness and openness to hearing what they want to share with you.

If you know or suspect that a service user may have experienced or be at immediate risk of intimate partner violence or sexual violence, follow your organization's procedures for responding to safety issues (see section [4.8 How do I manage imminent risk of suicide and other immediate safety concerns?](#)), in addition to following the steps above.

Steps to follow are outlined below.

1. Restate confidentiality

If someone wants to disclose information related to sexual violence or intimate partner violence, it is important to remind them that you will not share what they are telling you with anyone outside the intervention team, unless there are any immediate safety issues that may need to be addressed (see section [3.2.2 Maintain confidentiality](#)).

2. Show empathy and validate the person's story and experience

You can say something like:

- *"That sounds like a very difficult and upsetting situation. You have shown a lot of strength in telling me this."*

3. Listen without judgement

Give the person space and time to speak. Sometimes people just want someone to listen to their story.

In other cases, they may want or need additional help.

4. If appropriate, point to support organizations

Consider telling the person about any organizations in the area that might be able to help them.

If the person declines help, you can tell them that they can ask for a referral to an organization later. You might say:

- *“It can be very difficult going through experiences such as the one you have described. There might be some organizations that can offer further support. Would you like more information about them?”*

For more information on addressing sexual violence and intimate partner violence see:

- Health care for women subjected to intimate partner violence or sexual violence: a clinical handbook. Geneva: World Health Organization; 2014 (<https://apps.who.int/iris/handle/10665/136101>, accessed 18 September 2025).
- World Health Organization, United Nations Population Fund and United Nations High Commissioner for Refugees. Clinical management of rape and intimate partner violence survivors: developing protocols for use in humanitarian settings. Geneva: World Health Organization; 2020 (<https://apps.who.int/iris/handle/10665/331535>, accessed 18 September 2025).
- How to support survivors of gender-based violence when a GBV actor is not available in your area: a step-by-step pocket guide from humanitarian practitioners (version 2.0). Geneva: Inter-Agency Standing Committee; 2015 (<https://gbvguidelines.org/en/documents/pocket-guide-supporting-survivors-for-non-gbv-specialists/>, accessed 18 September 2025).

Annex 4. Supervision templates

Templates to support supervision include supervision forms for helpers to fill out before their supervision meeting to ensure all important issues are covered (see [Annex 4A. Supervision forms for helpers](#)); and supervision checklists for supervisors, peers or coordinators to fill out during the supervision meeting to ensure the intervention is being implemented well (see [Annex 4B. Fidelity checklist](#)).

Annex 4A. Supervision forms for helpers

Your supervisor may have a template or example supervision form for you to use. Otherwise, you can use either of the following templates to identify and record discussion topics. Fill out one form per service user.

Example supervision form for helpers from DWM guide

Example from digital version of SbS

Date and time: 14 August 2025

Supervisor: Jane

Helper: Marc

Number of completed contacts so far: 3

General note about presenting symptoms and problems: Presented with depressive symptoms that started a couple of months ago.

General note about service user's progress: Had completed all 3 modules and all exercises.

What has gone well so far:

For example, did you feel particularly confident about how something went? Have you noticed the service user reporting any progress?

I think I explained the intervention well, they told me they are using it and are feeling a little better.

Questions or difficulties that they would like help with:

For example, about a particular problem the service user has, a technique or exercise, a practical issue or a personal issue the user mentions.

They told me they struggle to complete the exercises at home. How can I manage this?

Discussion notes:

Key points discussed within the supervision session and feedback from supervisor.

There might be many reasons why they are finding this hard – perhaps there are distractions, they don't have time, they don't remember. It would be helpful to explore this. They are making great progress so this may help them get even more from SbS.

Plan for next contact with service user:

For example, recommendations from supervisor on next steps.

Ask them why they find it hard – is it because there are distractions? They don't have time? Remind them of the benefits of regular practice and help them troubleshoot the barriers they identify.

Example supervision form for helpers from DWM guide

Domain of information	Details
Background information	
1. Service user's age and gender	
2. Brief description of background (e.g. family, living situation), if available	
3. Brief description of main problems or stressors identified	
4. Number of contacts with the service user so far. Did the service user complete the module and do the exercises?	
Supervision questions	
5. What questions or difficulties do you have with this service user that you would like help with? (e.g. about a particular problem the service user has, a DWM technique/exercise, a practical issue or a personal issue that the service user has mentioned)	– – – –
6. What has gone well so far? (e.g. did you feel particularly confident about how something went? Have you noticed the service user reporting any progress?)	– –
Planning for the next call	
7. Based on the received feedback, what is your plan for your next contact with the service user?	

Annex 4B. Fidelity checklist

The following is a fidelity checklist that can be used by supervisors to ensure that helpers are following the support protocol. With the permission of service users, supervisors can record or listen in to calls and ensure all steps of the support contact are being completed.

Step	Checklist	Date (if completed)	Details
Pre-service planning			
1. Read and review the manual for guided self-help	☐ Completed ☐ Not completed		
2. Read the intervention and practise the skills	☐ Completed ☐ Not completed		
Welcome contact			
1. Introduce the intervention	☐ Completed ☐ Not completed		
2. Describe the helper's role	☐ Completed ☐ Not completed		
3. Discuss confidentiality	☐ Completed ☐ Not completed		
4. Discuss the stress management online intervention	☐ Completed ☐ Not completed		
5. Schedule a follow-up	☐ Completed ☐ Not completed		
Regular contacts			
1. Greeting	☐ Completed ☐ Not completed		
2. Review service user's experience of the module	☐ Completed ☐ Not completed		
3. Discuss practising the exercises and techniques	☐ Completed ☐ Not completed		
4. Schedule next meeting	☐ Completed ☐ Not completed		

Annex 5. Case note examples

Every time you have contact with the service user, you should also make a written note to document your contact. In the case of written contacts, you will have a record of the contact but you may also wish to write a summary so it is easy for you to review quickly before your next supervision and support contact.

Annex 5A. Example case note for welcome contact

Date and time: 23 January 2025

Length of contact: 15 minutes

Purpose of contact: Welcome contact

Outcome of contact: Service user contacted, intervention explained, no questions, reported that they are looking forward to getting started. Contact details confirmed.

What to call service user upon answer: Alex

Who to say is calling: Primary care service

Other instructions: Phone is shared with a partner who is aware of the intervention, but Alex does not want to share details with them at this stage.

In case of emergency: We can use this number and ask to speak with Alex's partner. Address is within our area so local services will be useful if we need to signpost for further support.

Annex 5B. Example case note for support contact

Date and time: 28 March 2025

Length of contact: 15 minutes

Purpose of contact: Support contact

Service user status: Module 1. Service user completed the module and entered information before the appointment.

Main points discussed: Finding the programme easy to use. Found it useful to hear others' experiences. Has not practised the activity yet, discussed the importance of practising between modules and they have scheduled to try it later today, to discuss next time.

Additional support needed (if yes, provide details): No issues identified.

Potential risk situation (if yes, provide detail): None. Stated is low in mood but no thoughts of harm to self or others reported.

Clinical supervisor informed: No. Will be discussed in routine supervision.

Annex 5C. Example case note for missed contact

Date and time: 28 March 2025

Purpose of contact: Support contact

Outcome: Attempted call. No answer. Message sent. Will contact at the same time next week.

Annex 6. Script for grounding exercise

This script is for live contacts.

If a service user is highly distressed, helpers may consider using a grounding exercise to help calm them and bring their distress back to a manageable level.

Below you will find a script for supporting people through a grounding exercise. For highly distressed individuals, it is generally better for them to keep their eyes open during the exercise and to spend the most time on step 3, where they observe things in the external environment, rather than focusing on internal feelings.

Encourage the person to share out loud what they observe in the environment around them. At the end of the exercise, check how they are feeling, and offer to repeat the exercise if they want to.

1. Introduction

- *“Remember, there are three steps to grounding. You can keep your eyes open as we go through them. First, notice how you are feeling and what you are thinking right now. Are you feeling stressed or caught in an emotional storm?”*

2. Slow down and connect

- *“For step 2, slow down and connect with your body. Push your feet into the floor, stretch, and focus on your breathing.”*

3. Refocus

- *“Now for Step 3, we will refocus on the world around us.*
- *What are five things you can see? [Pause and wait for their response, prompting if needed.]*
- *What are three or four things you can hear? [Pause and wait for their response, prompting if needed.]*
- *Reach out and touch something – like whatever you are sitting on. What does it feel like under your fingers? [Pause and wait for their response, prompting if needed.]*
- *Notice where you are and what you are doing.” [Prompt them to say aloud where they are and what they are doing.]*

4. Check in

- *“We just did a brief grounding exercise together. How are you feeling?”*

5. Repeat if necessary

- *“Is this something that you feel able to do yourself, or would you like to repeat the exercise together now?”*

Annex 7. Script for slow breathing exercise

This script is for live contacts.

If a service user is highly distressed, helpers may consider using a slow breathing exercise to help calm them and bring their distress back to a manageable level.

Below you will find a script for supporting people through a slow breathing exercise. At the end of the exercise, check how they are feeling, and offer to repeat the exercise if they want to.

1. Introduction

- *"I'm going to guide you through a slow breathing exercise. I'll read the instructions and at times I'll stop talking so you have time to practise."*
- *The idea is that when we are stressed, we often take shorter, quick little breaths. This can make us feel even more tense. We can use this exercise to slow down our breathing to help us reduce the stress and tension.*
[pause 1 second]
- *Before we start, I want you to relax your body a bit.*
- *Shake out your arms and legs, and let them go floppy and loose. Take a few seconds to do that now."*
[pause 5 seconds]

2. Get comfortable

- *"Great, now try to sit in a comfortable position with your back straight. Roll your shoulders back and gently move your head from side to side."*
[pause 5 seconds]

3. Begin

- *"Okay, we're ready to begin."*
[pause 1 second]
- *We are going to start by breathing in through the nose and out through the mouth.*
[pause 1 second]
- *Take a few breaths now, in through your nose and out through your mouth. You don't have to make an effort, just breathe normally and focus on exhaling."*
[pause 10 seconds]
- *"That's great, now just take a few more."*
[pause 5 seconds]

4. Slow breathing down

- *“Okay good. Now the next step is to slow your breathing down. Try to take around three seconds to breathe in through your nose and three seconds to breathe out through your mouth. I will count for you.*
- *So breathe in, one [1 second], two [1 second], three [1 second].*
- *And breathe out, one [1 second], two [1 second], three [1 second].*
- *And again, breathe in, [1 second], two [1 second], three [1 second].*
- *And breathe out, one [1 second], two [1 second], three [1 second].”*

5. Repeat

- *“Do you notice how slowly I counted? That is about how long it can take for you to take a slow breath in and out.*
- *We will now repeat this exercise for about half a minute and I will count the breaths for us.”*
[repeat, counting breaths for about 30 seconds]

6. Check in

- *“How was that for you?*
[pause 2 seconds]
- *Take a few moments to just notice how your body feels. Has there been any change? Do you feel less tense?*
[pause 5 seconds]
- *If not, that is okay, sometimes it takes a little practice. I would encourage you to practise because over time, this technique could help.*
- *Remember that the aim is to help reduce your stress, so if you find yourself becoming more stressed from doing this, or worrying about doing it right, then just do whatever feels good and does not cause you stress.*
[pause 1 second]
- *Is this something that you feel able to do yourself, or would you like to repeat the exercise together now?”*

Web Annex: Step-By-Step: A self-help intervention for depression

Psychological self-help interventions

Delivering self-help for individuals, featuring Step-by-Step
and Doing What Matters in Times of Stress



Web annex
Step-by-Step

A self-help intervention
for depression



<https://doi.org/10.2471/B09738>

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